Written Arguments ke liye Advocate Material + Vakil File (Master Reference File)

Forum: District Consumer Disputes Redressal Commission, Chandigarh (orders mein: "CHANDIGARH DISTRICT COMMISSION") | **Agli tarikh:** 22.12.2026 — oral arguments (EV-005)

Round 2 of 6 complete · Tarikh: 29.10.2026 · Version: VR-03 (pichhla: VR-01, Round 1)

STATUS: WORKING FILE — CHECKS NOT RUN

(Round 2 ki early delivery: poora File Integrity Gate Round 6 mein chalega. Is version par structure aur traceability ki compact jaanch chali hai — Part C, "Compact Integrity Result".)

> **Cover note (ek baar):** Yeh file enrolled advocate ke liye **working material** hai. Advocate har tathya aur kanoon khud verify karke hi court ke kagaz (Written Arguments) taiyar aur file karenge. Yeh court mein jama karne wala document nahi hai. Real medical expert ki rai abhi record par nahi hai.

Case ek nazar mein

Cheez	Record kya dikhata hai
Paksh (roles)	Complainants C1–C6 = Late Sh. Manohar (P) ke "legal heirs" (memo of parties, EV-010). Opposite Party (OP) = PGIMER, Chandigarh (ek OP; vakalatnama mein "& ANR" — X-14).
Proceeding	PR-01: Consumer Complaint DC/44/CC/423/2026, DCDRC Chandigarh — **forum ESTABLISHED** (orders 22.04.2026 se 14.10.2026, EV-001–EV-005). OP ne limitation aur territorial jurisdiction **khud maane** (WS paras 22–23, EV-070).
Stage	S6 Arguments: OP ka WS dated 26.06.2026, filing order 01.07.2026 mein (EV-071, EV-003); OP ka evidence band (07.08.2026); hamara rejoinder band (14.10.2026); OP ke Written Arguments file (14.10.2026); hamare Written Arguments ke liye 3 din (14.10.2026) — EV-004, EV-005.
Deadline Summary	**DL-05** hamare Written Arguments: 14.10.2026 + 3 din = 17.10.2026 (Sat; exclusion) / 16.10.2026 (inclusion) — **POSSIBLY EXPIRED**: filing record mein nahi — NOT VERIFIED. **DL-06** Hearing 22.12.2026 (oral arguments) — 54 din baaki — WATCH. **DL-08** CCTV/records preservation (12–13.03.2026) — koi preservation order padhe gaye orders mein nahi; OP ne WS mein CCTV par kuch nahi kaha — DANGER / POSSIBLY EXPIRED.
Complainants kya maangte hain	Rs. 60,00,000 compensation + 12% p.a. pendente lite interest; Rs. 2,50,000 medical expenses; Rs. 1,00,000 litigation expenses; duty doctors ki details; records + CCTV preservation, sealed cover (prayer i–vi, EV-023). Total principal claimed Rs. 63,50,000 (code se). Documents se OP ko bhugtan: Rs. 2,150 (Rs. 2,000 advance, EV-083 + Rs. 150, EV-043).
OP kya maangta hai	Complaint dismiss + "exemplary costs against the Complainants" (WS prayer, EV-071).
Ab tak kya padha	ZIP_1: D-01 complaint bundle (62/62), D-02–D-06 orders (7/7), D-07 e-Jagriti (1/1). ZIP_2: D-08 OP ka Written Statement + Annexures 1–12 (25/25 pages). ZIP_3–ZIP_6 **khole nahi gaye** (Round 3–6).

Is file ko kaise padhein (legend)

1. **Source labels:** SROT-SAMARTHIT TATHYA (mool document padha, page diya) · SROT-SAAR (summary/kisi ka bayan, mool se nahi mila) · INFERENCE (hamara tark, aadhaar saath) · LEGAL PROPOSITION (kanoon ki baat, status saath) · UNRESOLVED (srot nahi ya do srot alag).
2. **VERIFIED / NOT VERIFIED:** is file mein **har kanoon aur citation NOT VERIFIED** hai — official sites is session mein khul nahi sakin (E-01).
3. **RECORD GAP** = padhe gaye pages mein us samay ki entries nahi mili (iska matlab "kaam nahi hua" nahi). **NOT READ** = page padha hi nahi.
4. **Page cite:** hamari complaint: "C-8, idx p.52 / PDF p.54" (Annexure C-8, index page 52, PDF page 54). OP ka WS: "WS p.5, para 8" (WS ka page aur para). OP ke annexures: "OP Ann-5 (D-08 p.17)". D-08 mein PDF page = haath ka page number.
5. **Har POINT ke 5 hisse:** 1 Hamari baat · 2 Virodhi ka jawab · 3 Hamara jawab · 4 Kanooni aadhar aur saboot · 5 Vakil ke liye antim dalil. EV-xxx = Evidence Register ki entry; CIT-xx = Citation Register (kanoon); C-1 se C-11 = hamare annexures; Ann-1 se Ann-12 = OP ke annexures.

Naam ki legend (sirf ek baar): P = Late Sh. Manohar (marij). C1 Noor Aalam (beta), C2 Muskan (beti), C3 Husan Bano (patni/widow), C4 Gulbahar, C5 Soni, C6 Jasmin (C4–C6 ka P se rishta memo mein nahi likha — Q-05). OP = PGIMER. GMSH-16 = Government Multi Specialty Hospital, Sector 16, Chandigarh. "Dr. A" = Junior Resident, Dept. of Internal Medicine, PGIMER (C-8 par stamp). OP ke doctor/staff sirf role se (Professor, Cardiology; Head, Transfusion Medicine; CCU duty doctor; on-duty SR). Aadhaar, mobile, UHID/CR number jaise identifiers is file mein nahi diye gaye (sirf aakhri 4 ank, jahan zaroori). OP ke blood register (Ann-7) par doosre marijon ke naam hain — unhe kahin dohraya nahi gaya.

CASE IMPACT — ROUND 2 (OP ka Written Statement + Annexures 1–12)

Line	Kya hua (ID aur page ke saath)
READ	D-08 reply.pdf: OP ka Written Statement (WS pp.1–12) + OP Annexures 1–12 (D-08 pp.13–25) — 25/25 pages READ (visual; haath ke charts native zoom par). Tier: TIER-A (OP ka apna filed record). WS dated aur verified 26.06.2026; court filing order 01.07.2026 mein (EV-071, EV-003).
STRENGTHENS	**POINT 1** (cardiac halat, duty): OP ka apna CCU proforma diagnosis "CAD – USA (Wellen's pattern)" aur "anaemia" likhta hai (EV-074, Ann-3); OP ne duty of care maani (WS p.10, 20-G) aur ACS ka ilaaj maana (WS p.8, para 17) — Documentary Support → Express Admission (ladder step 4). WS ka "merely suspected" (WS p.4, para 3) OP ke apne Ann-3 ke viparit (step 3). **POINT 2** (transfusion): OP ne maana ki blood "never released or issued" (WS p.5–6, para 8) — ab transfusion na hona client ka bayan nahi, OP ki Express Admission hai. OP ka apna plan "correct anaemia" (EV-074, Ann-3) aur apna Hb 7.5 g/dL (11.03.2026, EV-078, Ann-6) — WS ke "precautionary standby ... no medical indication" ke viparit (step 3). **POINT 5** (CCU se bahar, trolley): OP ne maana ki P ko CCU se shift kiya, ward bed mein "temporary delay" thi, aur P "on the trolley" tha (WS paras 10, 11, 20-b) — ALLEGED → Express Admission (step 4). X-13 (maut ki jagah) suljha. **POINT 14 (naya)**: OP ka apna nursing note: P "Received in CCU at 12:15 ... unresponsive & pulse was not palpable" (EV-077, Ann-5); aakhri vital entry 12/3 raat 9 pm — 3 h 15 min (code se) koi entry nahi. **POINT 10**: OP ke apne record mein "Adv. Rs. 2000.00 ... 11-Mar-2026" (EV-083, Ann-10); WS ne consumer status par koi aapatti nahi uthayi (No Reply). **POINT 11**: limitation aur territorial jurisdiction OP ne "admitted" likha (WS p.11, paras 22–23).
WEAKENS	**POINT 3 jaisa complaint mein likha** ("no monitoring after 8:30 AM"): OP ka CCU vital chart (Ann-5) 11/3 subah 9:30 se 12/3 raat 9 pm tak entries dikhata hai; C-9 OP ke mutabiq Emergency ka chart hai (WS p.5, para 7). POINT 3 ko gaps aur values par dobara bana diya gaya (W-27). **Maut ka samay**: OP ka WS (para 18) aur nursing note (Ann-5) 12:51 AM kehte hain; hamari complaint ka "1:53 AM" padhna galat tha — "incorrectly records" aur "manipulation" wali baat chhod deni chahiye (W-23). **"No attendant came to collect"**: OP ke Transfusion Medicine ka letter (01.06.2026, Ann-7) kehta hai blood "kept ready for issue around 12 p.m. However, no attendant came to collect the same" — OP iska istemal karega (W-24, X-21). **Heart failure wali baat**: OP ka BNP 37.4 pg/mL (Ann-1, print par tarikh/naam nahi) aur echo "(N) LVEF" (Ann-2) — "severe cardiac distress"/heart failure wali framing kamzor; ACS/Wellens (OP ka apna Ann-3) par tikein (W-25). **Aspiration**: OP kehta hai intubation ke waqt trachea mein "particulate matter and fluids" mile (WS para 19) — record annexed nahi, par expert ke saamne yeh dalil aayegi. **Costs**: OP ne complainants par "exemplary costs" maange (W-26).
CLOSES / OPENS	**Closes:** X-02 (maut ka samay: 12:51 AM, OP ke TIER-A record se), X-13 (jagah: CCU ke bahar collapse, CCU mein ghoshit), X-15 (C-9 ki doosri entry 8:30 AM — OP ne khud "8:30 AM on 11.03.2026" likha), W-05, W-13 (C-9 OP ka Emergency chart), G-04 (WS padha). **Opens:** X-18 se X-30; G-17 se G-24 (OP ke records jo annex nahi hue); Q-26 se Q-33; EQ-8 se EQ-13.
CHANGES	DL-01 (45 din): WS par tarikh 26.06.2026, filing 01.07.2026 — WS late tabhi jab notice 11.05.2026 (WS-tarikh aadhar) / 16.05.2026 (filing aadhar) ya pehle mila ho (code; exclusion rule) — abhi bhi CONDITIONAL. Supported bhugtan Rs. 150 → Rs. 2,150 (RH-03). POINT 3 ka framing badla; naya POINT 14; KUL POINTS 13 → 14. STALE: VR-01 ke RECORD GAP intervals jo C-9 ki doosri entry se maut tak gine gaye the (Ann-5 ne gap bhar diya).
NEXT	Advocate Written Arguments mein OP ke apne Ann-3, Ann-5, Ann-6, Ann-7, Ann-10 aur WS ki admissions (paras 8, 10, 11, 14, 15, 17, 18, 20-G, 22, 23) par tikein; OP ke jo records annex nahi hue (G-17–G-24) unki maang; parivar se Q-26/Q-27 (blood collect karne ki baat) ka jawab.

FILING SE PEHLE KYA JAANCHNA BAAKI HAI (Round 2 ke baad)

1 (sabse pehla sawal, advocate ke liye): 14.10.2026 ke order ke mutabiq hamare (complainants ke) Written Arguments "within 3 days in registry" file hone the (EV-005) — yaani 16.10.2026 (inclusion rule) ya 17.10.2026 (Saturday; exclusion rule; agle working day ka niyam NOT VERIFIED). **Kya ye file hue? Record mein nahi hai.** Agar file nahi hue, to 22.12.2026 se pehle kya kadam (permission/application) uthana hai, yeh advocate tay karenge — procedure NOT VERIFIED. [____ to be filled: filing ki tarikh aur diary number]

2. **OP ke Written Arguments** (14.10.2026 ko "filed", EV-005) ki copy abhi bhi kisi zip mein nahi (G-02). WS ke baad unmein naye points ho sakte hain — registry se copy lein.
3. **45-din wala point:** OP ka WS 26.06.2026 ko dated/verified hai (EV-071) aur order 01.07.2026 mein "filed" likha hai (EV-003). Notice ki service report (G-03) ke bina yeh CONDITIONAL hai (POINT 13) — lead point na banayein.
4. **Rejoinder band hai** (14.10.2026). Isliye OP ke WS ka har material para Written Arguments mein jawab maangta hai — Part B, section 6 (para-wise jaanch) aur Part A, A6a dekhein. Asar NOT VERIFIED (CIT-18).
5. **OP ka evidence band hai** (07.08.2026). WS ke medical daave (Hb 7 ki seema "global guidelines", "PGIMER protocol", Medical Grievances Committee ki rai, intubation ki finding) kisi evidence affidavit ya document se samarthit nahi — asar NOT VERIFIED (POINT 4).
6. **Maut ka samay galat na likhein:** OP ke record ke mutabiq P 13.03.2026 ko 12:15 AM par CCU mein "unresponsive" laaye gaye aur 12:51 AM par mrit ghoshit hue (EV-077, EV-062). Complaint ka "death certificate ... 1:53 AM" wala dava hamari padhai ki galti thi (W-23). "Manipulation" ka aarop na uthayein.
7. **Maut ki jagah ab saaf hai:** OP khud maanta hai ki P CCU se shift hue, trolley par the, aur raat ~9 pm par ek baar CCU laaye gaye (WS paras 10, 11, 14); 12:15 par CCU mein "unresponsive" laaye gaye. C-11 ka "ward No. CCU" = jahan maut ghoshit hui. Parivar se Q-30 (trolley theek kahan thi) ka jawab lein.
8. **"No attendant came to collect":** OP ka letter (Ann-7, 01.06.2026) kehta hai blood "kept ready for issue around 12 p.m." par koi lene nahi aaya. Parivar ka jawab chahiye (Q-26, Q-27) — complaint para 15 kehti hai parivar raat ~9 pm blood bank gaya to woh band mila.
9. **"Complainant No.2" ka galat ullekh:** complaint paras 16, 18, 21 maa/patni ko "Complainant No.2" kehte hain; memo aur vakalatnama mein C2 = Muskan (beti), C3 = Husan Bano (patni) (X-01). OP ne is par kuch nahi kaha, par Written Arguments mein sahi number likhein.
10. **Raat ke tathyon ka affidavit** sirf C1 ka hai, jo ghar chala gaya tha (W-06). OP ke apne nursing note (Ann-5) aur WS paras 14, 18 ab raat ki kai baatein khud maante hain — un par tikein.
11. **Autopsy:** C-11/Ann-9 par "NOT WILLING FOR AUTOPSY" aur C1 ke hastakshar (EV-044, EV-082) — W-04, Q-04.
12. **Heart failure ki baat na karein:** OP ka BNP 37.4 (Ann-1) aur echo "(N) LVEF" (Ann-2). Dalil ACS/Wellens + anaemia par rakhein (OP ka apna Ann-3) — W-25.
13. **Kanoon:** saari citations/provisions (CIT-01 se CIT-23) NOT VERIFIED (E-01). OP ne complaint ki teen judgments ko "settled law" maana (WS 20-H), par unka text phir bhi official source se padhna hoga.
14. **Legal heirs, rakam ke saboot, umar:** pehle jaisa (G-09–G-12; X-05, X-09). OP ka kehna: aay/dependency ka "documentary evidence" nahi (WS para 21).
15. **OP ke records jo annex nahi hue** (G-17–G-24): Medical Grievances Committee ke "Para-wise Comments", ward/trolley vital chart, shifting/transfer note, resuscitation record, PGIMER protocols, counselling notes, echo/radiology reports, ECGs ki tarikh/time. Request → application ka kram (POINT 12).
16. **Prayer (iv) aur (v)** (duty doctors ki details; CCTV) par WS chup hai (No Reply) aur orders mein koi aadesh nahi (G-16).
17. **Part A filing text nahi hai.** Cause title, prayer clause, verification aur signature block advocate khud likhenge (skill rule 11) — [OMITTED — rule 11].

PART A — WRITTEN ARGUMENTS KE LIYE ADVOCATE MATERIAL (English)

What this part is: material for the advocate's Written Arguments on behalf of the complainants — not a text for filing (skill rule 11). It gives the issues in the order a bench would take them, every fact with its document and page, the OP's written statement (WS) with its exact words and location, our answer, every legal point with its verification status, and what the Written Arguments must contain. EV-ids point to the Evidence Register (Part C). Unverified law is marked "(verify citation before filing)". Missing facts are "[____ to be filled]". Round 2 has added the OP's WS and Annexures 1–12 (D-08).

A1. Heading details the advocate will need (information, not a cause title)

Item	As the record shows
Forum	District Consumer Disputes Redressal Commission, Chandigarh ("CHANDIGARH DISTRICT COMMISSION" in every order; EV-001–EV-005).
Case number	Consumer Complaint No. DC/44/CC/423/2026 (EV-001; e-Jagriti EV-006: filing date 2026-04-07). The OP's WS is titled "COMPLAINT CASE NO. 423 OF 2026" (EV-046).
Parties as registered	"NOOR AALM & 5 ORS" v. "POST GRADUATE INSTITUTE OF MEDICAL EDUCATION & RESEARCH" (orders). Spelling differs: "Noor Aalam" (memo, EV-010; WS, EV-046), "NOOR ALAM" (vakalatnama, EV-045) — X-17.
Document	Written Arguments on behalf of the complainants — time granted: "within 3 days in registry" from 14.10.2026 (EV-005). Filing status: [____ to be filled] (first item of "Filing se pehle").
Next date	22.12.2026 "for oral arguments of the parties" (EV-005; EV-006).

A2. Brief synopsis (material)

1. The complainants sue as legal heirs of the patient (P), who died at PGIMER on 13.03.2026 (C-11; EV-044; memo of parties EV-010).
2. Before PGIMER, P was treated at GMSH-16 from 04.02.2026 for sputum-positive pulmonary TB on ATT (since 07.02.2026), newly diagnosed type 2 diabetes, hyponatraemia and hypoalbuminaemia (C-1, C-2, C-4; EV-025, EV-026, EV-030).
3. On 10.03.2026 GMSH-16 recorded Hb 8.2 g/dL (EV-033) and at 00:14 on 11.03.2026 referred P "to PGI Emergency regarding his recent ECG findings [CAD ?ACS?] (Wellens)" (EV-032). The OP accepts that P "was appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome" (WS p.4, para 3; EV-052).
4. PGIMER recorded "Imp - Wellens Syndrome" in its emergency OPD (C-8; EV-037) and admitted P to "Cardio C C U" at 10:10:36 on 11.03.2026, taking an advance of Rs. 2,000 (OP Ann-10; EV-083). Its own CCU proforma records "retrosternal chest pain × 1 day (11pm on 10/3/26)", ECG rate 123 bpm with new-onset T changes in V2–V4 (OP Ann-2; EV-073), the diagnosis "CAD – USA (Wellen's pattern)" and "anaemia", and the plan "correct anaemia" (OP Ann-3; EV-074). Its own haemogram of 11.03.2026 records Hb 7.5 g/dL (OP Ann-6; EV-078).
5. The OP admits it owed P a duty of care and treated him for Acute Coronary Syndrome (WS p.10, para 20-G; WS p.8, para 17; EV-067, EV-061).
6. One unit of PRBC was requisitioned for P (received at the ACC blood storage centre at 09:15 on 12.03.2026), cross-matched and "kept ready for issue" (OP Ann-7; EV-079; C-10, EV-041). The OP admits the blood "was never released or issued from the Blood Bank" (WS p.5–6, para 8; EV-055) and that its CCU duty doctor told the family there was "no immediate medical urgency for blood transfusion" (WS p.8, para 15; EV-060).
7. The OP admits P was shifted out of the CCU, that there was a "temporary delay in the availability of a ward bed", and that P was kept "on the trolley" (WS paras 10, 11, 20-b; EV-056, EV-057, EV-065). It says P was brought back to the CCU "only once at around 9pm on 12.03.26" (WS p.7, para 14; EV-059).
8. The OP's own nursing record states P was "Received in CCU at 12:15", "unresponsive & pulse was not palpable", and was declared dead after CPR at 12:51 AM on 13.03.2026 (OP Ann-5; EV-077; WS p.8, para 18; EV-062). The last vital-sign entry before that is at 9 pm on 12.03.2026 (EV-076).
9. Relief claimed: Rs. 60,00,000 with 12% p.a. interest, Rs. 2,50,000 medical expenses, Rs. 1,00,000 litigation expenses, details of duty doctors, preservation of records and CCTV (EV-023). The OP seeks dismissal with exemplary costs (EV-071).

A3. Procedural history (from the orders; all orders are procedural, none is on the merits)

Date	Exact words of the order / document (shortest that carry the point)	Effect (read at its own words)	EV
07.04.2026	Complaint filed (e-Jagriti "Filing Date 2026-04-07"; index dated 07.04.2026; affidavit attested 07.04.2026).	Filing	EV-006, EV-007, EV-024
22.04.2026	"Complaint is admitted for hearing. Notice to the OP(s) be issued for 29.5.2026 with the directions to OP(s) to file written statement within 30 days from the date of receipt of notice extendable for 15 days (total 45 days)."	Admission; time for WS fixed by the order	EV-001
29.05.2026	"Vakalatnama on behalf of OP has been filed. Let written version, alongwith evidence, on behalf of OP be filed within the prescribed period. Put up on 1.7.2026."	OP appeared through counsel	EV-002
26.06.2026	OP's WS "Dated: 26.06.2026"; verification "Verified at Chandigarh on the 26th day of June, 2026" by the "Authorised Representative, PGIMER" (a Professor of Cardiology).	Date on the WS (not the filing date)	EV-071
01.07.2026	"Written version on behalf of OP filed. Copy be supplied. Evidence has not been filed. Be filed by the next date. To come up on 07.08.2026 for filing rejoinder."	WS on record; OP evidence pending	EV-003
07.08.2026	"Evidence by OP not filed despite being provided sufficient opportunity. Hence the opportunity to file eviden[ce] by OP No.1 is closed by order of this Commission." "Rejoinder on behalf of complainant not filed. Time prayed. Granted being last chance. ... within 7 days ..." "Written arguments on behalf of both the parties be filed within 2 weeks in the registry." "For oral arguments of the parties, listed for 14.10.2026."	OP evidence closed; last chance for rejoinder; WA for both	EV-004

Date	Exact words of the order / document (shortest that carry the point)	Effect (read at its own words)	EV
14.10.2026	"Rejoinder on behalf of complainant not filed despite grant of sufficient opportunity ... opportunity to f[ile] rejoinder on behalf of complainant is closed by order of this Commission." "Written arguments on behalf of OP have been filed." "Written arguments by the complainant not filed. Time sought. Allowed. Same be filed within 3 days in registry." "To come up on 22.12.2026 for oral arguments of the parties."	Complainants' rejoinder closed; OP's WA on record; 3 days for complainants' WA	EV-005
22.10.2026	e-Jagriti: "Case Stage ADMIT", "Case Category POOR EMERGENCY CARE", "Next Hearing Date 2026-12-22".	Portal status (TIER-B)	EV-006

Words in [ ] are cut at the right margin in the original order PDFs (D-05, D-06). A procedural order is not an admission by the OP and not a finding on the merits (core rule 25); its legal effect is NOT VERIFIED (CIT-18).

A4. Facts with their sources (chronological)

Status: DOCUMENTED = shown by the record; AS RECORDED = the entry exists (truth/time not proved); ALLEGED = a party's pleading only; ADMITTED = stated by the OP in its WS. Page cite: our annexure "C-n, idx p. / PDF p."; OP's annexure "OP Ann-n (D-08 p.)"; WS "WS p., para".

F-id	Date / time	Fact	Source	Status
F-001	04.02.2026 11:37 – 15.02.2026 01:08	GMSH-16 admission; "Chronic Smoker / Newly Dxd DM"; "PTB Sputum +ve on ATT since 7/2/26"; "Hypoalbuminemia / Hyponatremia"; referred to PGI Pulmonology. Age 55; "Ayushman Bharat".	C-1, idx p.36 / PDF p.38 (EV-025)	DOCUMENTED
F-002	15.02.2026 05:06 – 17.02.2026 11:35	GMSH-16 re-admission; "c/o ↓ appetite"; discharged home. (Complaint para 4 says "evening" — X-06.)	C-2, idx p.37 / PDF p.39 (EV-026)	DOCUMENTED
F-003	20.02.2026	Private laboratory: Hb 10.0; Na 131.2; creatinine 1.7; albumin 2.8.	C-3, idx pp.38–41 (EV-027, EV-028)	AS RECORDED
F-004	24.02.2026 16:46	NT-proBNP 1117.48 pg/mL (ref 0–114.5; private WeCare laboratory; 9.76× upper limit, computed). The OP calls it a GMSH-16 report (X-29).	C-3, idx p.42 / PDF p.44 (EV-029)	AS RECORDED
F-005	24.02.2026 – 05.03.2026	GMSH-16 admission; "ATT induced hepatitis"; "Rifampicin reintroduced on 5/3/26"; "Cured". No cardiac diagnosis written (X-07).	C-4, idx p.43 (EV-030)	DOCUMENTED
F-006	02.03.2026; 05.03.2026	USG: "B/L moderate pleural effusion"; elastography 4.2 kPa.	C-5, idx pp.44–45 (EV-031)	DOCUMENTED
F-007	10.03.2026 12:52 – 11.03.2026 00:14	GMSH-16: "Nausea vomiting x 4 days", "Anorexia x 2 months"; "Trop T ... Negative"; referral "to PGI Emergency regarding his recent ECG findings [CAD ? ACS?] (Wellens)".	C-6, idx p.46 / PDF p.48 (EV-032)	DOCUMENTED
F-008	10.03.2026 13:07	"HGB 8.2 [g/dL]"; "Anemia"; B positive.	C-7, idx p.47 (EV-033)	DOCUMENTED
F-010	11.03.2026 02:04	PGIMER registration "SR CITIZEN (PRIORITY)", Internal Medicine (E M OPD).	C-8, idx p.51 (EV-036)	DOCUMENTED
F-011	11.03.2026	EMOPD: "BP - 130/100", "PR - 116/min", "SpO2 - 94% RA"; "Trop T Neg. (outside)"; "CBC - 8.2"; "BNP - 1117.48 (24/2)"; "Imp - Wellens Syndrome"; plan "Shift to CCU (ACC)".	C-8, idx pp.52–53 (EV-037, EV-038)	DOCUMENTED
F-024	11.03.2026 03:21	Chest X-ray acquired (image print only; no report annexed).	OP Ann-4 (D-08 p.16) (EV-075)	AS RECORDED
F-014	11.03.2026 05:00 and 08:30	Emergency B.P. chart: "5 Am 110/70, 80"; second entry 120/88, 80. The OP says this is "the vital chart of the Emergency Department" and dates the second entry "8:30 AM on 11.03.2026".	C-9, idx p.55 (EV-040); WS p.5, para 7 (EV-054)	DOCUMENTED
F-025	11.03.2026 10:10:36	Admitted to "Cardio C C U"; "Adv. Rs. 2000.00 Receipt No. [..]/1 Date: 11-Mar-2026". (CCU chart starts at 9:30 AM — X-19.)	OP Ann-10 (D-08 p.23) (EV-083); WS p.2, PO-VI (EV-050)	DOCUMENTED
F-026	11.03.2026	CCU proforma: "retrosternal chest pain × 1 day (11pm on 10/3/26)"; ECG "Rate: 123 bpm", T changes "V2 – V4 (new onset)"; echo "Tachycardia", "(N) LVEF"; admission vitals fields blank.	OP Ann-2 (D-08 p.14) (EV-073)	AS RECORDED
F-027	11.03.2026	Diagnosis "PTB on ATT", "CAD – USA (Wellen's pattern)", "anaemia"; plan "correct anaemia", "CXR"; signed by Cardiology SR.	OP Ann-3 (D-08 p.15) (EV-074)	DOCUMENTED
F-028	11.03.2026 (requisition date; time not printed)	PGIMER haemogram: "Haemoglobin, Hb (gm/dL) 7.5" (ref 13.5–17.5); "DUPLICATE COPY".	OP Ann-6 (D-08 p.18) (EV-078)	DOCUMENTED
F-029	11.03.2026 09:30 → 12.03.2026 21:00	CCU nursing vital chart: 11 entries (see C6/C7 in Part C); e.g. pulse 128 (9:30 AM 11/3), BP 90/49 (6 pm 11/3), BP 87/46 with pulse "45" [low-confidence] (9 AM 12/3), last entry 9 pm 12/3: 90/60, 82, 20.	OP Ann-5 (D-08 p.17) (EV-076)	AS RECORDED
F-030	11.03.2026 onwards	Drug chart: Brilinta 180 mg stat then 90 mg BD; Ecosprin 75 mg; Monit GTN 2.6 mg BD; Clexane 0.6 mL S/C BD; insulin; Lasix 40 mg stat then 20 mg IV BD; IVF 30 ml/hr; ATT drugs. No blood or transfusion order; no statin as read (X-22).	OP Ann-8 (D-08 p.21) (EV-081)	AS RECORDED
F-031	12.03.2026 09:15	Blood requisition (DTM No. ACC-0073148) received at the Blood Storage Centre, ACC.	OP Ann-7 (D-08 pp.19–20) (EV-079, EV-080)	DOCUMENTED
F-016	12.03.2026 12:55	Crossmatch for "Ward Cardio CCU", "Bed No. Trolly 66"; transfusion fields blank on the family's copy.	C-10, idx p.56 (EV-041)	DOCUMENTED
F-032	12.03.2026 "around 12 p.m."	"one unit of PRBC was crossmatched and kept ready for issue ... However, no attendant came to collect the same for the patient" (letter dated 01.06.2026). Register: issue columns blank. The OP admits the blood was "never released or issued".	OP Ann-7 (EV-079, EV-080); WS p.5–6, para 8 (EV-055)	ADMITTED (non-issue) / AS RECORDED (reason)

F-id	Date / time	Fact	Source	Status
F-017	12.03.2026 16:47:39	Receipt Rs. 150 POS (grouping and cross-matching).	C-10, idx p.58 (EV-043)	DOCUMENTED
F-033	12.03.2026 (time not stated; about 17:10 if "nearly 31 hours" from 10:10)	P shifted out of CCU; "temporary delay in the availability of a ward bed"; P "on the trolley".	WS paras 10, 11, 20-b (EV-056, EV-057, EV-065); PO-VI (EV-050)	ADMITTED
F-035	12.03.2026	The CCU duty doctor "correctly conveyed that there was no immediate medical urgency for blood transfusion".	WS p.8, para 15 (EV-060)	ADMITTED
F-034	12.03.2026 "around 9pm"	P "brought to the CCU only once"; "mild breathlessness"; pulse, saturation checked; auscultated. CCU chart 9 pm entry: 90/60, 82, 20.	WS p.7, para 14 (EV-059); OP Ann-5 (EV-076)	ADMITTED / AS RECORDED
F-018	12.03.2026 evening to night	Complainants' account: no CCU bed; stretcher on the 3rd floor near the staircase; chest pain; refusal to examine or re-admit; ACC blood bank closed about 9:00 PM; "no urgency"; C1 went home; the mother stayed.	Complaint paras 10–17 (EV-016–EV-018)	ALLEGED (partly ADMITTED: shifting, trolley, "no urgency")
F-036	13.03.2026 00:15	"pt Received in CCU at 12:15 ... Pt was unresponsive & pulse was not palpable. CPR started as per ACLS protocol ..." (nursing note; key words moderate confidence). The OP's WS: "received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered".	OP Ann-5 (EV-077); WS p.8, para 18 (EV-062)	DOCUMENTED / ADMITTED
F-037	13.03.2026 00:51	Death declared ("At 12:51 ... declared dead by on duty SR"; WS: "time of death as 12:51 AM"). The family's account of finding P unresponsive at about 12:10–12:15 AM (para 18) matches the time P was received unresponsive.	OP Ann-5 (EV-077); WS para 18 (EV-062); C-11 / OP Ann-9 (EV-044, EV-082)	DOCUMENTED
F-020	13.03.2026	Cause certified: I (a) Type I respiratory failure (b) aspiration pneumonia (c) coronary artery disease; II pulmonary tuberculosis; "Natural"; "NOT A MEDICOLEGAL CASE"; "NOT WILLING FOR AUTOPSY" (C1's signature).	C-11, PDF p.61 (EV-044); OP Ann-9 (EV-082)	DOCUMENTED
F-039	01.06.2026	Transfusion Medicine letter (Ann-7) written on Cardiology's letter dated 29.05.2026 — prepared after the complaint, for the defence.	OP Ann-7 (EV-079)	DOCUMENTED

A5. Issues for determination (proposed order)

Issue	Question	Element	POINTs (Part B)	Evidence sufficiency now
I-1	Are the complainants "consumers" and was the OP's service "service" for consideration?	Maintainability	POINT 10	Strong on facts: Rs. 2,000 advance + Rs. 150 (OP's records); no objection in WS. Law NOT VERIFIED.
I-2	Jurisdiction and limitation.	Maintainability	POINT 11	Strong: limitation and territorial jurisdiction expressly admitted (WS paras 22–23); pecuniary not objected.
I-3	What did the OP itself record about P's condition, and what duty did it accept?	Duty	POINT 1	Strong: OP's own Ann-2, Ann-3, C-8; duty and ACS treatment admitted.
I-4	Was the requisitioned and cross-matched blood withheld contrary to the OP's own plan?	Breach	POINT 2	Strong on documents (non-issue admitted; plan "correct anaemia"; Hb 7.5). Dispute on why it was not issued (X-21). Expert needed on indication.
I-5	Was it a deficiency to shift a CCU patient diagnosed with "CAD – USA (Wellen's pattern)" to a trolley for want of a ward bed?	Breach	POINT 5	Strong on facts (admitted); standard = expert.
I-6	Was P adequately observed, in the CCU and on the trolley, before he was found unresponsive at 12:15 AM?	Breach	POINT 3, POINT 14, POINT 6	Moderate–Strong: OP's own chart has 4–8 h gaps and no entry for 3 h 15 min before the arrest; values vs "stable throughout"; expert.
I-7	Did any deficiency cause or contribute to the death (causation, counterfactual)?	Causation	POINT 7, POINT 8, POINT 14	Weak–Moderate: sequence now documented; no expert opinion; no autopsy; co-morbidities.
I-8	Procedural: effect of the closure of the OP's evidence; unsupported medical assertions in the WS; 45 days (conditional); records not produced.	Procedural	POINT 4, POINT 12, POINT 13	Moderate; legal effects NOT VERIFIED; 45-day point CONDITIONAL.
I-9	What relief, and in what amount, is supported?	Damage	POINT 9	Harm documented; quantum largely unsupported (OP's para 21).

A6. Issue-wise material for the advocate

For each issue: facts with documents; the OP's words with WS location; our answer (from the record); law with status; red flags. These are argument lines for the advocate to use in their own words — not filing text.

I-1 Consumer status and consideration (POINT 10)

- **Facts:** "Adv. Rs. 2000.00 Receipt No. [..]/1 Date: 11-Mar-2026" on the OP's own inpatient record (OP Ann-10; EV-083); Rs. 150 by POS for cross-matching (C-10, idx p.58; EV-043); category "GENERAL" (EV-036, EV-043).
- **OP's WS:** no objection to consumer status, consideration or free service anywhere in PO-I to VII and paras 1–24 (WS pp.1–12 read; No Reply). Para 2 resists jurisdiction only on the ground that "a provable deficiency of service does not exist" (WS p.3, para 2; EV-051).
- **Our answer:** it can be submitted that the OP charged and collected consideration for the very admission and transfusion service in issue (EV-083, EV-043), and has not disputed consumer status. Effect of the absence of objection: NOT VERIFIED (CIT-20).
- **Law:** CPA 2019 ss.2(7), 2(42) (CIT-01, CIT-02); *V.P. Shantha* (CIT-09) (verify citation before filing).
- **Red flags:** do not call Rs. 2,50,000 "paid to PGIMER" without bills; GMSH-16 billing was "Ayushman Bharat" (relates to GMSH-16).

I-2 Jurisdiction and limitation (POINT 11)

- **OP's WS:** "The contents of Para 22 with regard to the date of the cause of action (13.03.2026) and the period of limitation are admitted." "The contents of Para 23 regarding territorial jurisdiction are admitted." (WS p.11, paras 22–23; EV-070). No pecuniary objection (No Reply).
- **Our answer:** admitted facts; the complaint was filed 25 days after the cause of action (computed).
- **Law:** CIT-03, CIT-04, CIT-08 (verify citation before filing).

I-3 Duty and the OP's own diagnosis (POINT 1)

- **Facts:** F-007, F-010, F-011, F-025, F-026, F-027, F-030 (EV-032, EV-036–EV-038, EV-073, EV-074, EV-081, EV-083).
- **OP's WS:** "the Complaint grossly exaggerates the severity of the patient's cardiac condition" (WS p.2, PO-III); "the ECG at GMSH-16 on 10.03.2026 showed only '?Wellens Syndrome' the '?' indicating that the diagnosis was merely suspected and not confirmed" (WS p.4, para 3); "The patient was appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome" (ibid.); "placed on guideline directed medical management for Acute Coronary Syndrome (ACS)" (WS p.8, para 17); "The Opposite Party owed a duty of care to the deceased" (WS p.10, para 20-G).
- **Our answer:** "OP's own CCU proforma records the diagnosis 'CAD – USA (Wellen's pattern)' and 'anaemia' (OP Ann-3), and its emergency note records 'Imp - Wellens Syndrome' (C-8, idx p.52)." It can be submitted that the WS's "merely suspected" is contrary to the OP's own Annexure 3 (record check: Contradiction, X-27).
- **Honest counterweight:** the OP's own tests show troponin negative, BNP 37.4 and normal LVEF (EV-072, EV-073; EV-037). Argue ACS/Wellens and anaemia, not heart failure (W-25).
- **Law:** CIT-10, CIT-11 (verify citation before filing).

I-4 Blood transfusion (POINT 2)

- **Facts:** F-008, F-027, F-028, F-030, F-031, F-016, F-032, F-017, F-035 (EV-033, EV-074, EV-078, EV-081, EV-079, EV-080, EV-041, EV-042, EV-043, EV-055, EV-060).
- **OP's WS:** "blood was arranged at PGIMER only as a precautionary standby measure ... to be used only in the event of emergency bleeding such as haemoptysis. The blood was never released from the Blood Bank. The patient's haemoglobin at PGIMER was above 7 g/dL ... No medical indication for routine transfusion existed." (WS p.2, PO-V); "admitted that a blood cross match procedure was completed by the Blood Bank on 12.03.2026"; "the blood was never released or issued from the Blood Bank in the first instance. The six hour window ... would only commence from the time of issue which never occurred." (WS pp.5–6, para 8); "The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion" (WS p.8, para 15).
- **Our answer:** (1) the OP's own CCU plan was "correct anaemia" (Ann-3) and its own Hb on 11.03.2026 was 7.5 g/dL (Ann-6), lower than the 8.2 the WS discusses (para 3) — Omission; (2) the OP's own Transfusion Medicine says the unit was "crossmatched and kept ready for issue" for the patient (Ann-7), not reserved for haemoptysis — Possible Inconsistency (X-28); (3) no transfusion order appears in the OP's drug chart (Ann-8); (4) the "Hb < 7" threshold "per established/global guidelines" is asserted without any guideline or evidence (Unsupported Assertion; OP's evidence closed, POINT 4). Whether transfusion was indicated in a patient being treated for ACS, and whether it would have changed the outcome, is for the expert (EQ-3).
- **Red flags:** the OP's letter "no attendant came to collect the same" (Ann-7; W-24; X-21) — answer from the record only: the letter is dated 01.06.2026 and was written on the Cardiology department's request of 29.05.2026; the OP's own case is that its doctor said there was no urgency (WS para 15); the slip says "Kindly bring Blood Transport Box from the ward at the time of issue" (EV-042). Family's account: Q-26, Q-27.
- **Law:** no verified benchmark (CIT-21: transfusion guideline, NOT VERIFIED).

I-5 Shifting a CCU patient to a trolley (POINT 5)

- **OP's WS:** "Any delay in ward bed availability is a well known systemic constraint at PGIMER ... and cannot constitute negligence per se." (WS p.6, para 10); "It is admitted that there was a temporary delay in the availability of a ward bed due to PGIMER's extremely heavy patient load"; "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff." (WS pp.6–7, para 11); "shifted from CCU to the ward only after adequate monitoring and clinical assessment" (WS p.9, para 20-b).
- **Our answer:** the shifting and the trolley are admitted (Express Admission). The annexed vital chart (Ann-5) has no entry after 9 pm on 12.03.2026 until P was received unresponsive at 12:15 AM (EV-076, EV-077) — the WS's "conclusively evidenced by the vital chart" is not borne out by the chart annexed (Possible Inconsistency, X-24). No transfer or shifting assessment note is annexed (Omission). It can be submitted that a shortage of beds is the hospital's own arrangement and does not lessen its duty to a patient it had itself diagnosed with "CAD – USA (Wellen's pattern)" (CIT-22, NOT VERIFIED).
- **Expert:** EQ-4.

I-6 Observation before the arrest (POINT 3, POINT 14, POINT 6)

- **OP's WS:** "His vitals BP, pulse rate, and oxygen saturation were monitored constantly and recorded in the vital chart." (WS p.2, PO-VI); "The Annexure C-9 referred to by the Complainants is in fact the vital chart of the Emergency Department of PGIMER and

not the CCU. In CCU, a new vital chart was made." (WS p.5, para 7); "hemodynamic stability throughout" (WS p.8, para 15; p.10, 20-E).

- **Our answer:** the OP's own CCU chart (Ann-5) has 11 entries over about 35 hours, with intervals of 4 h (9 pm → 1 am), 8 h (1 am → 9 am), 4 h and 6 h (computed), and records pulse 125–128 and BP 90/49, 87/46 and 90/60 (EV-076). Whether these values showed instability needing escalation is for the expert (EQ-8); the label is Possible Inconsistency (X-25), not a finding. The last entry is 9 pm on 12.03.2026; P was received in the CCU unresponsive at 12:15 AM (EV-077) — RECORD GAP of 3 h 15 min while P was on the trolley (POINT 14).
- **Correction to our own pleading:** complaint para 7 / ground A ("no monitoring after 8:30 AM") is contradicted by Ann-5 (W-27) — do not repeat it; argue the gaps and the values.

I-7 Causation and counterfactual (POINT 7, POINT 8, POINT 14)

- **OP's WS:** death "attributable entirely to the cumulative and progressive deterioration of these underlying conditions" (WS p.2, PO-IV); aspiration "is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation, where particulate matter and fluids were visually identified in the trachea" (WS pp.8–9, para 19).
- **Our answer:** the OP's own certificate lists "CORONARY ARTERY DISEASE" in the chain (EV-044); the OP's WS says "secondary to Aspiration" while the certificate says "ASPIRATION PNEUMONIA" (X-26); no resuscitation or intubation record is annexed (Unsupported Assertion). The arrest occurred before P reached the CCU at 12:15 AM (EV-077). Whether closer observation or the planned correction of anaemia could have changed the outcome is for the expert (EQ-3, EQ-6, EQ-13).
- **Red flags:** no autopsy (W-04); co-morbidities (W-08); C-6 vomiting (W-09).

I-8 Procedural points (POINT 4, POINT 12, POINT 13)

- **OP's evidence closed** (EV-002–EV-004). The WS's medical assertions rest on the WS alone, verified by the Professor of Cardiology as authorised representative (EV-071): the "Medical Grievances Committee" para-wise comments (PO-II) are not annexed; the "existing protocol at PGIMER" (paras 6, 10, 12) is not annexed; the guideline for "Hb < 7" is not annexed. Effect: NOT VERIFIED (CIT-18, CIT-23).
- **Records:** WS is silent on prayers (iv) and (v) (No Reply); records not annexed listed at G-17–G-24.
- **45 days (CONDITIONAL):** WS dated 26.06.2026 (EV-071), filing recorded 01.07.2026 (EV-003). Late only if notice was received on or before 11.05.2026 (WS-date basis, exclusion rule) or 16.05.2026 (filing-date basis, exclusion rule) — computed; date of receipt: [____ to be filled] (G-03). Law: CIT-06, CIT-07.

I-9 Relief and quantum (POINT 9)

- **OP's WS:** "The claim of Rs. 60,00,000/- (Rupees Sixty Lakhs) is wholly excessive, speculative, and unsupported by any documentary evidence of income or financial dependency." (WS p.11, para 21); "The sympathy and loss felt by the Complainants are acknowledged" (ibid.); prayer for "exemplary costs against the Complainants" (WS p.12).
- **Our answer:** the death and the heirs are documented (EV-044, EV-010); quantum needs documents (G-09–G-12). The WS does not answer prayers (ii) Rs. 2,50,000 and (iii) Rs. 1,00,000 specifically (No Reply). Supported payments to the OP: Rs. 2,150 (EV-083, EV-043).
- **Law:** CIT-12, CIT-13, CIT-14 (the OP "admitted as settled law" these three, WS 20-H; still verify), CIT-16.

A6a. OP's preliminary objections and the material for each answer

PO	OP's words (shortest) and WS location	Material for the answer	Record-check label
PO-I	"No deficiency in service ... received timely, appropriate, and guideline directed medical management at PGIMER from 11.03.2026 to 13.03.2026" (WS p.1)	OP's own records: plan "correct anaemia" not carried out (Ann-3, Ann-6, Ann-7, Ann-8); shifted to trolley (paras 10–11); no observation 9 pm → 12:15 AM (Ann-5). No guideline annexed.	Unsupported Assertion ("guideline directed")
PO-II	Complainants "deliberately suppressed and misrepresented material medical facts"; Department of Cardiology's "Para-wise Comments submitted to the Medical Grievances Committee ... conclusively demonstrated" conformity (WS p.1)	The committee's comments are not annexed (G-17); they are the treating department's own view (not independent); no expert affidavit (evidence closed).	Unsupported Assertion
PO-III	"grossly exaggerates the severity"; NT-proBNP "recorded at GMSH-16"; "repeat BNP ... only 37.4 pg/mL"; normal LVEF (WS p.2)	OP's own Ann-3 "CAD – USA (Wellen's pattern)" (X-27); NT-proBNP report is from a private lab (X-29); BNP print (Ann-1) carries no name or date and shows MYO 176 and D-dimer 3280 flagged abnormal (Omission); BNP and NT-proBNP are different tests (EQ-10).	Contradiction / Omission / Possible Inconsistency
PO-IV	co-morbidities incl. "co-existing Coronary Artery Disease (CAD)"; "The patient's relatives were repeatedly informed of his very poor overall prognosis" (WS p.2)	CAD admitted (helps POINT 1, 7). No counselling note annexed (G-22).	Express Admission (CAD) / Unsupported Assertion (counselling)
PO-V	blood "only as a precautionary standby measure ... never released"; "No medical indication for routine transfusion existed" (WS p.2)	Ann-3 "correct anaemia"; Ann-6 Hb 7.5; Ann-7 "kept ready for issue"; no guideline annexed.	Contradiction (indication) / Express Admission (never released)
PO-VI	"admitted to the CCU ... at 10:10 AM on 11.03.2026 ... observed and monitored for nearly 31 hours ... monitored constantly ... ECG was repeated ... showed NO ST elevations" (WS p.2)	Ann-10 confirms 10:10:36; Ann-5 shows 4–8 h gaps (X-25); Ann-11/12 ECGs carry no legible date, time or name (EQ-9); "31 hours" puts the shifting at about 17:10 on 12.03.2026 (computed).	Consistent Record (time) / Possible Inconsistency (constantly)
PO-VII	Rs. 60,00,000 "emphatically denied ... arbitrary, baseless, excessive" (WS p.3)	Claim ledger (Part C, C12); OP does not address prayers (ii)–(v).	Express Denial

A7. What the Written Arguments must contain (checklist for the advocate)

1. Brief synopsis and procedural history (A2, A3) with the exact words of the orders and the WS date.
2. Facts in date order, each with its document and page (A4) — separating what the OP admits from what only the complainants say.
3. Issues I-1 to I-9 with the material above, and a para-wise answer to the OP's WS (A6a; Part B section 6), since the rejoinder opportunity is closed.
4. The OP's own admissions and records first (Ann-3, Ann-5, Ann-6, Ann-7, Ann-10; WS paras 8, 10, 11, 14, 15, 17, 18, 20-G, 22, 23).
5. Corrections the advocate should make openly: time of death (12:51 AM, not "1:53 AM"); "Complainant No.2"; "no monitoring after 8:30 AM"; heart-failure language (W-23, W-07, W-27, W-25).
6. Our weak points answered honestly (Part B, section 5).
7. Relief tied to the Damages Ledger (Part C) — claimed separate from supported. Answer to the OP's prayer for exemplary costs.
8. List of documents relied upon (A8), including the OP's own annexures.
9. [OMITTED — skill rule 11: the cause title, prayer clause, verification and the place/date/signature block are prepared by the advocate.] Answers to the OP's Written Arguments of 14.10.2026: [____ to be filled after reading them, G-02].

A8. Documents relied upon

Mark	Document	Page	Note
C-1	GMSH-16 discharge summary 04/15.02.2026	idx 36 / PDF 38	age 55; Ayushman Bharat
C-2	GMSH-16 discharge summary 15/17.02.2026	idx 37 / PDF 39	
C-3	Medical reports (Labogram 20.02.2026; WeCare NT-proBNP 24.02.2026)	idx 38–42	Labogram pages "2 of 5", "4 of 5" not in bundle
C-4	GMSH-16 discharge summary 24.02.2026/05.03.2026	idx 43	
C-5	GMSH-16 ultrasound and elastography	idx 44–45	
C-6	GMSH-16 discharge summary 10/11.03.2026 (referral)	idx 46	
C-7	GMSH-16 reports: haematology, investigations, two ECGs (10.03.2026)	idx 47–50	black box on ECG copy at idx p.50
C-8	PGIMER emergency OPD record and treatment sheet 11.03.2026	idx 51–54	OP's own record
C-9	PGIMER Emergency B.P. chart (identified by the OP, WS para 7)	idx 55	
C-10	PGIMER blood reaction form, identification slip, test receipt (12.03.2026)	idx 56–58	system printouts (CIT-17)
C-11	PGIMER medical certificate of cause of death	PDF 61 (handwritten "60")	same document as OP Ann-9
OP Ann-1	Point-of-care cardiac panel print (BNP 37.4; MYO 176 and D-dimer 3280 flagged)	D-08 p.13	no name/date on the print
OP Ann-2, Ann-3	CCU admission proforma (chest pain, ECG, echo; diagnosis "CAD – USA (Wellen's pattern)", "anaemia"; plan "correct anaemia")	D-08 pp.14–15	OP's own CCU record
OP Ann-4	Chest X-ray image 11.03.2026 03:21	D-08 p.16	no report
OP Ann-5	CCU nursing vital chart + night note (received 12:15 unresponsive; death 12:51)	D-08 p.17	key document
OP Ann-6	PGIMER haemogram 11.03.2026 (Hb 7.5)	D-08 p.18	"DUPLICATE COPY"
OP Ann-7	Transfusion Medicine letter 01.06.2026 + register page	D-08 pp.19–20	post-complaint; other patients' data on the register page (not reproduced)
OP Ann-8	Drug and treatment chart	D-08 p.21	no blood order
OP Ann-9	MCCD (OP's copy)	D-08 p.22	same as C-11
OP Ann-10	Inpatient record cover (CCU admission 10:10:36; advance Rs. 2,000)	D-08 p.23	cover only
OP Ann-11, Ann-12	ECG copies	D-08 pp.24–25	no legible date/time/name

PART B — MASTER REFERENCE FILE (VAKIL FILE) — Hinglish

STATUS: WORKING FILE — CHECKS NOT RUN | VR-03 (Round 2 of 6)

KUL POINTS KI SANKHYA: Is file mein kul 14 alag-alag kanooni aur medical points hain.

FILE KA MAQSAD: Yeh Master Reference File isliye banayi gayi hai taaki hamare Vakil Sahab isme diye gaye saboot, virodhi ke daavon aur record ke beech ke virodhabhas (contradictions) aur kanooni points padh kar apne hisaab se Court File / Written Arguments khud taiyar kar sakein. Saath hi koi aam vyakti bhi ise padh kar samajh sake ki vivaad kya hai, virodhi ne kya kaha aur hamara jawab kya hai.

Round 2 mein OP (PGIMER) ka Written Statement aur uske Annexures 1–12 padhe gaye. Har POINT ka "Virodhi ka jawab" ab mool WS ke shabdon aur WS Location ke saath bhara gaya hai. Ek naya POINT 14 juda (raat 9 pm se 12:15 tak koi entry nahi; P CCU mein "unresponsive" laaye gaye). Saare findings **PROVISIONAL UNTIL PART 4** hain (red-team/disconfirmation Round 6 tak poora hoga).

Case ek nazar mein, Deadline Summary aur legend: Cover page; Round 2 ka asar: CASE IMPACT page (dohraya nahi gaya).

JEETNE WALE BADE POINTS (taakat ke kram mein — Round 2 ke baad)

Rank	POINT	Ek line mein	Element	Taakat (kaaran)	Sabse majboot saboot	Virodhi ka jawab (WS) / sambhavit	Kya aur majboot karega
1	POINT 1	OP ke apne CCU record mein "CAD – USA (Wellen's pattern)" aur "anaemia"; OP ne duty of care aur ACS ka ilaaj maana	Duty	Strong: OP ka TIER-A record + Express Admission	EV-074 (Ann-3); EV-067 (WS 20-G); EV-061 (WS 17)	"merely suspected", "grossly exaggerates" (WS p.4 para 3; p.2 PO-III)	Cardiologist (EQ-1, EQ-9)
2	POINT 2	Blood requisition (09:15), crossmatch, "kept ready for issue"; OP ka plan "correct anaemia", Hb 7.5; OP maanta hai blood "never released or issued"	Breach	Strong (documents) / Moderate (kyun nahi diya — X-21)	EV-055 (WS 8); EV-074; EV-078; EV-079	"precautionary standby ... haemoptysis"; "Hb above 7"; "no attendant came to collect" (Ann-7)	Q-26/Q-27; EQ-3; OP ka transfusion order/register
3	POINT 5	OP maanta hai: CCU se shift kiya, ward bed nahi mila, P trolley par rahe	Breach	Strong: Express Admission; standard = expert	EV-056, EV-057, EV-065 (WS 10, 11, 20-b)	"systemic constraint ... cannot constitute negligence per se" (WS 10)	EQ-4; shifting note (G-19)
4	POINT 14	12/3 raat 9 pm ke baad koi vital entry nahi; 12:15 AM par P CCU mein "unresponsive & pulse was not palpable" laaye gaye; 12:51 par mrit	Breach, Causation	Strong–Moderate: OP ka apna nursing note; note ke kuch shabd low-confidence	EV-077 (Ann-5); EV-062 (WS 18)	"Even on the trolley ... monitoring ... evidenced by the vital chart" (WS 11)	Trolley/ward chart (G-18); EQ-13
5	POINT 3	OP ke CCU chart mein 4–8 ghante ke gap; pulse 125–128, BP 90/49, 87/46, 90/60 — WS kehta hai "monitored constantly", "stable throughout"	Breach	Moderate: values ka arth expert ka; monitor logs nahi	EV-076 (Ann-5)	"monitored constantly"; C-9 Emergency chart hai (WS 7)	Monitor logs; EQ-8
6	POINT 11	Limitation aur territorial jurisdiction OP ne "admitted" likha	Maintainability	Strong: Express Admission	EV-070 (WS 22–23)	Pecuniary par kuch nahi	CIT-03, CIT-04 official text
7	POINT 10	OP ke record mein Rs. 2,000 advance + Rs. 150; consumer status par koi aapatti nahi	Maintainability	Strong (facts); kanoon NOT VERIFIED	EV-083 (Ann-10); EV-043	No Reply	CIT-01, CIT-02, CIT-09, CIT-20
8	POINT 4	OP ka evidence band; WS ke medical daave (Hb 7 "global guidelines", protocol, committee) bina document/affidavit	Procedural	Moderate: orders TIER-A; asar NOT VERIFIED	EV-004; EV-046, EV-049, EV-071	(OP: annexures kaafi hain — forecast)	CIT-18, CIT-23
9	POINT 7	MCCD mein OP ne CAD likha; aspiration ka aadhar (intubation finding) annexed nahi; "secondary to Aspiration" vs "Aspiration Pneumonia"	Causation	Moderate	EV-044/EV-082; EV-063	intubation par "particulate matter and fluids" (WS 19)	Resuscitation record (G-20); EQ-5
10	POINT 12	OP ne kai records annex nahi kiye (committee comments, ward chart, shifting note, protocols, counselling, resuscitation); CCTV par No Reply	Procedural	Moderate (tactical)	EV-046, EV-054, EV-056, EV-064; EV-023	No Reply (prayers iv, v)	Request → application (missing_record_strategy)
11	POINT 6	Shaam/raat ko OP ne sirf pulse, SpO2, auscultation likha; 12.03 ki shaam ka koi dated ECG nahi	Breach	Weak–Moderate: chest pain par DISPUTED; 9 pm BP recorded	EV-058, EV-059; EV-084	chest pain nahi tha (WS 17, 20-a); "mild breathlessness" (WS 14)	Dated ECGs (G-24); EQ-9
12	POINT 9	Maut 13.03.2026; C1–C6 legal heirs; rakam ka daava	Damage	Strong (maut) / Weak (rakam)	EV-044, EV-010	"unsupported by any documentary evidence of income" (WS 21)	G-09–G-12

Rank	POINT	Ek line mein	Element	Taakat (kaaran)	Sabse majboot saboot	Virodhi ka jawab (WS) / sambhavit	Kya aur majboot karega
13	POINT 8	Causation ki kadi: ab sequence documented, par expert zaroori	Causation	Weak–Moderate	EV-074, EV-078, EV-055, EV-057, EV-077	"attributable entirely to ... underlying conditions" (PO-IV)	Expert opinion (EQ-6, EQ-13)
14	POINT 13	45-din WS point — CONDITIONAL	Procedural	Weak: service report nahi	EV-001, EV-003, EV-071	(OP: samay par — forecast)	Service report (G-03)

POINT ka number us mudde ki pehchaan hai; rank taakat ka kram hai. Sab POINT 1–14 neeche paanch hisson mein hain. Jahan WS ne jawab diya, uske mool shabd WS Location ke saath hain; "forecast" = hamara anumaan, OP ke shabd nahi.

POINT 1: OP ke apne record mein Wellens / "CAD – USA" aur CCU ka faisla; OP ne duty of care aur ACS ka ilaaj maana

ELEMENT TAG: Duty (Breach ke liye decision context) | **Taakat:** Strong — OP ka apna TIER-A record + Express Admission; kanoon NOT VERIFIED

1. HAMARI BAAT / AAROP: GMSH-16 ne 11.03.2026 ko 00:14 par refer kiya: "We are referring our patient to PGI Emergency regarding his recent ECG findings [CAD ?ACS?] (Wellens)" (C-6, idx p.46; EV-032). PGIMER ke emergency note mein: "BP - 130/100 mm Hg", "PR - 116/min", "SpO2 - 94% RA", "CBC - 8.2/13k/336k", "BNP - 1117.48 (24/2)", "ECG → Wellens", "Imp - Wellens Syndrome"; plan "Shift to CCU (ACC)" (C-8, idx pp.52–53; EV-037, EV-038) — SROT-SAMARTHIT TATHYA. CCU mein admission 11.03.2026 10:10:36, "Ward: Cardio C C U" (OP Ann-10, D-08 p.23; EV-083). OP ke CCU proforma mein: "retrosternal chest pain × 1 day (11pm on 10/3/26)", ECG "Rate: 123 bpm", T changes "V2 – V4 (new onset)" (OP Ann-2, D-08 p.14; EV-073) aur diagnosis "CAD – USA (Wellen's pattern)", "anaemia" (OP Ann-3, D-08 p.15; EV-074) — SROT-SAMARTHIT TATHYA. ("USA" yahan unstable angina ka chhota roop maana ja raha hai — INFERENCE; cardiologist confirm kare.)

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]**

[WS Location: WS p.2 PO-III, PO-IV; p.4 para 3; p.8 para 17; p.10 para 20-G — poore para padhe]: "the Complaint grossly exaggerates the severity of the patient's cardiac condition" (PO-III); "co-existing Coronary Artery Disease (CAD)" (PO-IV); "the ECG at GMSH-16 on 10.03.2026 showed only '?Wellens Syndrome' the '?' indicating that the diagnosis was merely suspected and not confirmed"; "The patient was appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome." (para 3); "The patient was placed on guideline directed medical management for Acute Coronary Syndrome (ACS)" (para 17); "The Opposite Party owed a duty of care to the deceased and discharged that duty at every stage" (para 20-G).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — duty aur ACS ilaaj RECORD SE SAMARTHIT; "merely suspected" RECORD KE VIPARIT.
- Express Admission: duty of care (WS 20-G; EV-067); ACS ka ilaaj (WS 17; EV-061); CAD "co-existing" (PO-IV; EV-048); referral "ECG changes suggestive of Wellens Syndrome" ke liye (WS 3; EV-052).
- Express Denial: severity "grossly exaggerates" (PO-III; EV-047); "critical" ya "decompensated heart failure" kehna "a clinical falsehood" (WS p.5, para 6; EV-053).
- No Reply: LAGU NAHI — is point par WS ne jawab diya.
- Hamare record ka samarthan: EV-032, EV-036–EV-038 (C-6, C-8); OP ke apne EV-073, EV-074, EV-081 (Ann-2, Ann-3, Ann-8).
- Virodhabhas: X-27 — WS para 3 ka "merely suspected and not confirmed" OP ke apne Ann-3 ("CAD – USA (Wellen's pattern)", bina "?") aur C-8 ("Imp - Wellens Syndrome") ke viparit — Contradiction (ladder step 3). X-10 — treatment sheet par "Date 10/3/26" (Possible Inconsistency; maasoom galti sambhav).
- Imaandari se: OP ke apne tests mein Trop T/TNI negative (EV-037, EV-072), BNP 37.4 (EV-072) aur echo "(N) LVEF" (EV-073) — heart failure ki baat kamzor; ACS/Wellens par tikein (W-25).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-032, EV-036–EV-039, EV-073, EV-074, EV-081, EV-083; OP ki admissions EV-048, EV-052, EV-061, EV-067. Kanoon: duty of care aur standard — CIT-10 (*Jacob Mathew*, 2005), CIT-11 (*Kusum Sharma*, 2010) — LEGAL PROPOSITION, NOT VERIFIED (citation to be confirmed by counsel); "service" — CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "OP's own CCU proforma records the diagnosis 'CAD – USA (Wellen's pattern)' and 'anaemia' (EV-074; OP Ann-3), its emergency note records 'Imp - Wellens Syndrome' (EV-037; C-8, idx p.52), and the OP admits that it owed the patient a duty of care and treated him for Acute Coronary Syndrome (WS p.10, para 20-G; WS p.8, para 17)." Yeh dalil di ja sakti hai ki OP ka "merely suspected" (WS para 3) uske apne Annexure 3 ke viparit hai (ladder step 3), aur 11.03.2026 se hi OP ne P ki cardiac halat jaan kar CCU-level care ka zimma liya tha. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (RECORD-SUPPORTED) "Troponin negative, BNP 37.4, LVEF normal — heart failure nahi tha." → hum heart failure nahi keh rahe; OP ne khud "CAD – USA (Wellen's pattern)" likha aur ACS ka ilaaj kiya; standard ka sawal expert se (EQ-1, EQ-7, EQ-9). | **RELIEF + SEEMA:** duty aur OP ki jaankari ka nishkarsh; akele yeh point breach ya causation saabit nahi karta.

POINT 2: Blood transfusion — OP ka apna plan "correct anaemia", Hb 7.5; blood requisition, crossmatch, "kept ready for issue" — par OP maanta hai blood "never released or issued"

ELEMENT TAG: Breach (Causation ka sawal expert ko) | **Taakat:** Strong (documents aur admission) / Moderate (blood kyun nahi diya — X-21; zaroorat ka sawal expert ka)

1. HAMARI BAAT / AAROP: Hb 8.2 g/dL (GMSH-16, 10.03.2026 13:07; C-7, idx p.47; EV-033). PGIMER ka apna haemogram (requisition date 11/03/2026): "Haemoglobin, Hb (gm/dL) 7.5" (OP Ann-6, D-08 p.18; EV-078). OP ke CCU proforma ka plan: "correct anaemia" (OP Ann-3; EV-074; 3× zoom par MODERATE confidence). OP ke Transfusion Medicine ka letter (01.06.2026): requisition "received in the Blood Storage Centre, ACC on 12th March, 2026 at 9.15 a.m. vide DTM No. ACC-0073148. Subsequently one unit of PRBC was crossmatched and kept ready for issue around 12 p.m. However, no attendant came to collect the same for the patient." (OP Ann-7, D-08 p.19; EV-079); register mein issue ke saare khaane khaali (D-08 p.20; EV-080). Crossmatch 12:55, "Ward Cardio CCU / Trolly 66", transfusion ke fields khaali (C-10, idx p.56; EV-041); slip: "Kindly ensure transfusion of blood units within six hours of issue ... Kindly bring Blood Transport Box from the ward at the time of issue of blood." (C-10, idx p.57; EV-042); Rs. 150 POS 16:47:39 (EV-043). OP ke drug chart mein koi blood/transfusion order nahi (OP Ann-8; EV-081) — NOT RECORDED IN THE PAGES READ. Complaint paras 15–16 (EV-018): ACC blood bank raat ~9 PM band mila; Nehru Hospital blood bank ne naya crossmatch maanga; CCU doctor: "no urgency" — CLIENT-REPORTED.

2. VIRODHI (PGIMER) KA JAWAB [WS-MOOL-TEXT]

[WS Location: WS p.2 PO-V; pp.5–6 para 8; p.8 para 15; p.10 para 20-D — poore para padhe]: "blood was arranged at PGIMER only as a precautionary standby measure ... to be used only in the event of emergency bleeding such as haemoptysis. The blood was never released from the Blood Bank. The patient's haemoglobin at PGIMER was above 7 g/dL, which is the established clinical threshold below which transfusion is indicated. No medical indication for routine transfusion existed." (PO-V); "It is admitted that a blood cross match procedure was completed by the Blood Bank on 12.03.2026"; "Critically, the blood was never released or issued from the Blood Bank in the first instance. The six hour window specified in the Blood Reaction Form would only commence from the time of issue which never occurred." (para 8); "The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion" (para 15).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — "never released" RECORD SE SAMARTHIT (register khaali); "no medical indication" aur "solely ... standby" RECORD KE VIPARIT / JAANCH AVASHYAK.
- Express Admission: crossmatch 12.03.2026 ko hua (WS 8; EV-055); blood kabhi issue nahi hua (PO-V, WS 8, WS 15, 20-D; EV-049, EV-055, EV-060, EV-066); CCU duty doctor ne parivar ko "no immediate medical urgency" kaha (WS 15; EV-060).
- Express Denial: blood "procured" by complainant at doctors' advice (WS 8); "false assurance" (20-E).
- No Reply: OP ka apna Hb 7.5 (Ann-6) WS para 3 mein nahi aata — WS sirf GMSH ka 8.2 discuss karta hai (Omission).
- Hamare record ka samarthan: EV-074 (plan "correct anaemia"), EV-078 (Hb 7.5), EV-079 ("kept ready for issue"), EV-080, EV-041, EV-042, EV-043, EV-081.
- Virodhabhas: X-28 — "No medical indication for routine transfusion" (PO-V) vs OP ka apna plan "correct anaemia" (Ann-3) — Contradiction (step 3; maasoom vyakhya: anaemia ko doosre tareeke se theek karna — Ambiguity, expert); "standby for haemoptysis" vs Ann-7 "kept ready for issue ... for the patient" — Possible Inconsistency. "Hb < 7 ... per established/global guidelines" (paras 3, 8, 15, 20-D) — koi guideline annex nahi: Unsupported Assertion. X-12 (slip "six hours" vs form "two hours"); X-20 ("around 12 p.m." vs crossmatch 12:55).
- OP ka bada counter (hamari kamzori, W-24): "no attendant came to collect the same" (Ann-7) — parivar ke bayan (para 15) se alag: X-21 UNRESOLVED. Dhyan dein: yeh letter 01.06.2026 ka hai, Cardiology ke 29.05.2026 ke letter par likha gaya (baad mein, defence ke liye) — contemporaneous clinical record nahi.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-033, EV-074, EV-078, EV-079, EV-080, EV-041, EV-042, EV-043, EV-081; OP ki admissions EV-049, EV-055, EV-060. Standard (ACS ke marij mein Hb 7.5 par transfusion ki zaroorat aur urgency): "no verified benchmark for this date" → EQ-3; transfusion guideline CIT-21 NOT VERIFIED. Deficiency — CIT-02 NOT VERIFIED. System printouts — CIT-17 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "PGIMER's own CCU plan records 'correct anaemia' (EV-074; OP Ann-3) and its own haemogram of 11.03.2026 records Hb 7.5 g/dL (EV-078; OP Ann-6). One unit of PRBC was requisitioned (received at 09:15 on 12.03.2026), cross-matched and 'kept ready for issue' (EV-079; OP Ann-7), yet the OP admits that the blood 'was never released or issued' (WS pp.5–6, para 8). No transfusion order appears in the OP's drug chart (EV-081; OP Ann-8)." Yeh dalil di ja sakti hai ki OP ka "precautionary standby ... no medical indication" uske apne "correct anaemia" plan se mel nahi khaata (ladder step 3), aur transfusion na hone ka faisla OP ke apne CCU doctor ka tha ("no immediate medical urgency", WS para 15). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (RECORD-SUPPORTED) "Hb 7.5 seema se upar tha; blood taiyar tha; parivar lene nahi aaya." → (1) issue ke liye ward ko request bhejni hoti hai (slip: "from the ward at the time of issue"); (2) OP ke apne doctor ne kaha urgency nahi — to parivar ka na jaana OP ki salah ke mutabiq tha; (3) "no attendant came" baad ka letter hai, issue request ka koi record nahi; (4) zaroorat par expert (EQ-3). | **RELIEF + SEEMA:** deficiency in service ka nishkarsh (RH-01 se juda); causation expert ke bina nahi.

POINT 3: Monitoring — OP ke apne CCU chart mein 4–8 ghante ke gap aur pulse 125–128, BP 90/49, 87/46, 90/60; WS kehta hai "monitored constantly", "stable throughout"

ELEMENT TAG: Breach | **Taakat:** Moderate — OP ka apna chart; values ka arth aur monitor logs ka sawal expert/record ka (Round 1 ka "8:30 ke baad koi monitoring nahi" ab is roop mein nahi rehta — W-27)

1. HAMARI BAAT / AAROP: Emergency B.P. chart (C-9, idx p.55; EV-040): "11/3, 5 Am, 110/70, 80" aur 8:30 AM "120/88, 80" — OP ise "the vital chart of the Emergency Department of PGIMER" maanta hai (WS p.5, para 7; EV-054). OP ka CCU "NURSING SERVICE" vital chart (OP Ann-5, D-08 p.17; EV-076) — entries (date | time | BP | pulse): 11/3: 9:30 AM 113/73, 128; 10:30 "—"; 12 pm 11?/79, 125 (temp 100°F, "Tab PCM given"); 3 [pm] 124/103 [LOW], 126; 6 pm 90/49, 106; 9 pm 130/79, 110. 12/3: 1 am 110/74, 100; 9 AM 87/46, "45" [LOW: 45 ya 95]; 1 pm 110/73, 82; 3 p[m] 9?/60, 114; 9 pm 90/60, 82 — AS RECORDED (haath ki likhai; native zoom). Gaps (code se): 9 pm (11/3) → 1 am = 4 h 00 min; 1 am → 9 am = 8 h 00 min; 9 am → 1 pm = 4 h; 3 pm → 9 pm = 6 h. OP ki treatment sheet: "I/O, vitals, RBS" (EV-039); drug chart: "vitals q [?]" (EV-081).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]**

[WS Location: WS p.2 PO-VI; p.5 para 7; p.8 para 15; p.9 para 20-a; p.10 para 20-E — poore para padhe]: "His vitals BP, pulse rate, and oxygen saturation were monitored constantly and recorded in the vital chart." (PO-VI); "The Annexure C-9 referred to by the Complainants is in fact the vital chart of the Emergency Department of PGIMER and not the CCU. In CCU, a new vital chart was made. The vital chart of the CCU as well as the Ward at PGIMER was duly maintained throughout the patient's admission"; "The allegation that 'no monitoring was conducted after 8:30 AM on 11.03.2026, is factually false" (para 7); "his hemodynamic stability throughout" (para 15); "The vital chart conclusively establishes hemodynamic stability throughout the admission." (20-E).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — "8:30 ke baad bhi entries" RECORD SE SAMARTHIT (Ann-5); "constantly" aur "stable throughout" JAANCH AVASHYAK.
- Express Admission: C-9 OP ka apna Emergency chart hai aur doosri entry "8:30 AM on 11.03.2026" (WS 7; EV-054) — X-15 aur W-13 suljhe.
- Express Denial: "no monitoring after 8:30 AM" (WS 7).
- No Reply / Omission: WS kehta hai ward ka vital chart bhi "duly maintained" (WS 7), par sirf CCU chart annex hai — ward/trolley chart nahi (G-18).
- Hamare record ka samarthan: EV-076 (OP ka apna chart), EV-039, EV-081.
- Virodhabhas: X-25 — "stable throughout" vs chart ke values (BP 90/49, 87/46, 90/60; pulse 125–128): Possible Inconsistency (clinical arth expert ka; maasoom vyakhya: marij ke liye yeh values sweekarya ho sakti hain). "Monitored constantly" vs 4–8 ghante ke gap: Possible Inconsistency (maasoom vyakhya: CCU monitor continuous ho, chart periodic — monitor logs chahiye). 9 AM 12/3 (BP 87/46) ke baad agli entry 1 pm par — beech mein koi kaarvai ka record padhe gaye pages mein nahi.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-076, EV-040, EV-054, EV-039, EV-081. Standard (ACS ke marij mein CCU mein monitoring ki frequency; in values par escalation): Benchmark NOT VERIFIED → EQ-1, EQ-8. Kanoon: CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "OP's own CCU vital chart records eleven entries over about 35 hours, with intervals of 4 h, 8 h, 4 h and 6 h between them, and records pulse 125–128 on 11.03.2026 and blood pressure 90/49 (6 pm, 11.03), 87/46 (9 am, 12.03) and 90/60 (9 pm, 12.03) (EV-076; OP Ann-5)." Yeh dalil di ja sakti hai ki WS ka "monitored constantly" aur "hemodynamic stability throughout" OP ke apne chart se samarthit nahi (Possible Inconsistency) — values ka arth expert batayega. "Monitoring nahi hui" na kahein; complaint ka "8:30 ke baad koi monitoring nahi" na dohraayein (W-27). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (PLAUSIBLE) "CCU mein continuous cardiac monitor tha; chart sirf periodic entries hain." → OP monitor logs/central station records pesh kare (missing_record_strategy); tab tak record jitna dikhata hai utna hi. | **RELIEF + SEEMA:** breach ka nishkarsh expert aur OP ke poore record par; akela yeh point negligence saabit nahi karta.

POINT 4: OP ka evidence ka mauka 07.08.2026 ko band — WS ke medical daave bina document ya affidavit ke

ELEMENT TAG: Procedural | **Taakat:** Moderate — orders TIER-A; kanooni asar NOT VERIFIED; hamara rejoinder bhi band hai

1. HAMARI BAAT / AAROP: Orders: 29.05.2026 "Let written version, alongwith evidence, on behalf of OP be filed within the prescribed period." (EV-002); 01.07.2026 "Evidence has not been filed. Be filed by the next date." (EV-003); 07.08.2026 "Evidence by OP not filed despite being provided sufficient opportunity. Hence the opportunity to file eviden[ce] by OP No.1 is closed" (EV-004) — SROT-SAMARTHIT TATHYA; procedural orders. WS ko "Authorised Representative, PGIMER" (Cardiology ke Professor, yaani ilaaj karne wala vibhag) ne 26.06.2026 ko verify kiya (WS p.12; EV-071).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]**

[WS Location: WS p.1 PO-II; p.4 para 3; pp.5–6 paras 6, 8; p.6 para 10; p.7 para 12; p.12 verification — poore para padhe]: "The Opposite Party's Department of Cardiology has, in its Para-wise Comments submitted to the Medical Grievances Committee (in CC No. 423 of 2026), conclusively demonstrated that the clinical management ... was in full conformity with accepted medical practice" (PO-II); "(which is Hb < 7 g/dL per established guidelines)" (para 3); "As per the existing protocol at PGIMER, all cardiac patients are shifted to CCU and triaged according to clinical condition which was duly followed" (para 6); "As per the routine protocol at PGIMER, all patients are clinically assessed prior to shifting from CCU to the ward this was duly done." (para 10).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: RECORD SE SAMARTHIT NAHI — committee ke comments, PGIMER protocol, "established guidelines" — inmein se koi annex nahi (D-08 pp.13–25 padhe).
- Express Admission: LAGU NAHI — is point par.
- Express Denial / No Reply: LAGU NAHI — WS ne evidence band hone par kuch nahi kaha (WS order se pehle ka hai).
- Hamare record ka samarthan: EV-002, EV-003, EV-004; EV-005 ("Written arguments on behalf of OP have been filed").
- Virodhabhas: Unsupported Assertions (PO-II, paras 3, 6, 8, 10, 12, 15, 20-D) — Part B section 6 ki list dekho. Verifier khud ilaaj karne wale vibhag ka hai — independence ka sawal (expert_reports_on_record).
- Seema: order "OP No.1" kehta hai jabki memo mein ek hi OP (X-14).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-002–EV-005, EV-046, EV-052, EV-053, EV-056, EV-071. Kanoon: evidence band hone ka asar — CIT-18; bina evidence affidavit ke WS ke tathyon ka wazan — CIT-23 — LEGAL PROPOSITION, NOT VERIFIED [provision: advocate to confirm]. Procedural order OP ki admission nahi hai (core rule 25).

5. VAKIL KE LIYE ANTIM DALIL: "By order dated 07.08.2026 the Commission closed the OP's opportunity to file evidence, after directions on 29.05.2026 and 01.07.2026 (EV-002–EV-004). The OP's written statement relies on a Medical Grievances Committee's comments, an 'existing protocol at PGIMER' and 'established guidelines', none of which is annexed (D-08 pp.13–25)." Yeh dalil di ja sakti hai ki in daavon ke samarthan mein record par na document hai na affidavit — iska kanooni asar NOT VERIFIED (advocate check). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (PLAUSIBLE, forecast) "WS ke saath medical record ke annexures hain; complainant ne bhi rejoinder nahi diya." → annexures (Ann-3, Ann-5, Ann-6, Ann-7) kai jagah WS ke hi viparit hain (POINT 1, 2, 3, 14); dono paksh ke procedural tathya imaandari se saath rakhein (W-01). | **RELIEF + SEEMA:** OP ke bina-saboot daavon par kam wazan ki maang — kanoon verify hone tak sirf argument.

POINT 5: OP maanta hai — P ko CCU se shift kiya, ward bed nahi mila, P trolley par rahe

ELEMENT TAG: Breach | **Taakat:** Strong — Express Admission (WS paras 10, 11, 20-b); standard of care ka sawal expert ka (EQ-4)

1. HAMARI BAAT / AAROP: Complaint paras 10–13 (EV-016, EV-017): 12.03.2026 ki shaam "no bed was available in the CCU"; ward mein bed nahi; "placed on a stretcher on the 3rd floor near the staircase"; CCU duty doctor ne re-admit karne se mana kiya — CLIENT-REPORTED. OP ke apne kagaz: 12.03.2026 ko "Ward Cardio CCU", "Bed No. Trolly 66" (C-10, idx pp.56–57; EV-041, EV-042) — SROT-SAMARTHIT TATHYA. CCU admission 10:10:36 (EV-083) + "nearly 31 hours" (WS PO-VI) = 12.03.2026 ~17:10 (code se) — INFERENCE (shifting ka andaazan samay).

2. VIRODHI (PGIMER) KA JAWAB [WS-MOOL-TEXT]

[WS Location: WS p.6 para 10; pp.6–7 para 11; p.7 paras 12, 14; p.9 paras 20-b, 20-c — poore para padhe]: "The patient was under constant monitoring by both nursing staff and doctors in the CCU prior to being shifted. As per the routine protocol at PGIMER, all patients are clinically assessed prior to shifting from CCU to the ward this was duly done." "Any delay in ward bed availability is a well known systemic constraint at PGIMER ... and cannot constitute negligence per se." (para 10); "It is admitted that there was a temporary delay in the availability of a ward bed due to PGIMER's extremely heavy patient load"; "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff." (para 11); "The allegation that a Ward Doctor specifically advised the immediate re-shifting of the patient to the CCU is a completely false allegation." (para 12); "the patient was brought to the CCU only once at around 9pm on 12.03.26. We deny the fact that the patient was repeatedly sent back from the CCU." (para 14); "The patient was shifted from CCU to the ward only after adequate monitoring and clinical assessment. The temporary non-availability of a bed in the ward ... does not constitute 'abandonment.' The patient continued to be monitored even on the trolley." (20-b).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — shifting, trolley, ward bed ki deri RECORD SE SAMARTHIT (OP ki apni admission); "assessed prior to shifting" aur "monitored even on the trolley" RECORD SE SAMARTHIT NAHI.
- Express Admission: CCU se shifting (WS 10, 20-b); ward bed mein "temporary delay" (WS 11); P "on the trolley" (WS 11, 20-b); raat ~9 pm par P ko CCU "brought ... once" (WS 14).
- Express Denial: "abandoned"/"abandonment" (WS 11, 20-b); ward doctor ki CCU wapas bhejne ki salah (WS 12); "repeatedly sent back from the CCU" (WS 14); bed na milne ki soochna na dena (20-c).
- No Reply / Omission: shifting/transfer assessment ka koi note annex nahi (G-19); ward/trolley ka vital chart annex nahi (G-18).
- Hamare record ka samarthan: EV-041, EV-042 (trolley); OP ka apna Ann-3 diagnosis "CAD – USA (Wellen's pattern)" (EV-074).
- Virodhabhas: X-24 — "monitoring ... even on the trolley ... as conclusively evidenced by the vital chart" vs Ann-5 mein 12/3 raat 9 pm ke baad 12:15 tak koi entry nahi (Possible Inconsistency; maasoom vyakhya: alag trolley chart jo annex nahi). X-13 SULJHA: OP ke WS (paras 10, 11, 14, 18) aur Ann-5 ke mutabiq P CCU ke bahar collapse hue aur CCU mein mrit ghoshit hue — C-11 ka "ward No. CCU" ghoshna ki jagah hai.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-056, EV-057, EV-058, EV-059, EV-065 (WS), EV-041, EV-042, EV-074, EV-076, EV-077, EV-083. Standard (Wellens/ACS ke marij ko CCU se trolley par shift karna; handover; observation): EQ-4. Bed ki kami ("systemic constraint") deficiency se bachav hai ya nahi: CIT-22, LEGAL PROPOSITION, NOT VERIFIED. Deficiency: CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "The OP admits that the patient, whom its own CCU had diagnosed with 'CAD – USA (Wellen's pattern)' (EV-074), was shifted out of the CCU and kept on a trolley because no ward bed was available (WS p.6, para 10; WS pp.6–7, para 11; WS p.9, para 20-b)." Yeh dalil di ja sakti hai ki bed ki kami hospital ki apni vyavastha hai, aur usse marij ke prati duty kam nahi hoti (CIT-22 NOT VERIFIED); OP ka "assessed prior to shifting" aur "monitored even on the trolley" uske annexed record se samarthit nahi (ladder step 2–3). Corridor/3rd floor ki sateek jagah abhi parivar ka bayan hai (Q-30). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (WS se) "Systemic constraint; negligence per se nahi; trolley par bhi monitoring hui." → OP ka apna chart trolley wale samay ki koi entry nahi dikhata (POINT 14); shifting note pesh nahi; standard expert batayega. | **RELIEF + SEEMA:** bed/handover/observation mein deficiency ka nishkarsh — RH-01 se juda.

POINT 6: Shaam/raat ki jaanch — OP ne CCU duty doctor ki jaanch mein sirf pulse, SpO2 aur auscultation likha; 12.03.2026 ki shaam ka koi dated ECG nahi

ELEMENT TAG: Breach | **Taakat:** Weak–Moderate — chest pain DISPUTED; 9 pm par BP recorded hai (hamara "no BP" sahi nahi, W-28); ECG ka sawal khula

1. HAMARI BAAT / AAROP: Complaint paras 12–14 (EV-017): stretcher par "acute and severe chest pain"; CCU doctor ka jawab "solely based on pulse oximeter readings"; "failed to conduct any further assessments such as blood pressure monitoring or ECG" — CLIENT-REPORTED. OP ke CCU proforma mein chest pain shuruati shikayat thi: "retrosternal chest pain × 1 day (11pm on 10/3/26)" (EV-073). OP ke annexed ECG (Ann-11, Ann-12) par tarikh, samay ya naam padhe nahi jaate (EV-084); C-7 ke ECG GMSH-16 ke hain (EV-035). 12/3 raat 9 pm ki CCU entry: 90/60, 82, 20 (EV-076).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.7 paras 12, 14; p.8 para 17; p.9 para 20-a — poore para padhe]**: "The CCU Duty Doctor thoroughly examined the patient, pulse and oxygen saturation were checked and found to be within normal limits, with findings similar to those at initial admission to PGIMER, no tachycardia or tachypnoea" (para 12); "His primary complaint when he was brought back to CCU was of mild breathlessness ... His pulse, saturation was checked which were within normal limits. The patient was auscultated" (para 14); "after initiation of cardiac management, the patient did not complain of chest pain." (para 17); "The patient denied chest pain on multiple occasions during his CCU stay"; "ECG was repeated during his CCU stay and showed no ST elevations or dynamic changes." (20-a).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: JAANCH AVASHYAK — dated ECG nahi; "no tachycardia ... similar to ... initial admission" RECORD KE VIPARIT ho sakta hai (neeche X-23).
- Express Admission: CCU duty doctor ne pulse aur saturation dekhe, auscultation kiya (WS 12, 14).
- Express Denial: cardiac ilaaj ke baad chest pain (WS 17, 20-a).
- Omission: WS paras 12, 14 mein in encounters par ECG ka zikr nahi; 12.03 ki shaam ka koi dated ECG annex nahi (G-24).
- Hamare record ka samarthan: EV-073, EV-076, EV-084.
- Virodhabhas: X-23 — WS 12 "no tachycardia ... findings similar to those at initial admission" vs OP ka apna Ann-2 "Tachycardia", ECG "Rate: 123 bpm" aur C-8 "PR - 116/min" (Possible Inconsistency; WS ka bayan shaam ki jaanch ka hai, shuruat ka nahi — dhyan se). X-30 — shaam ko chest pain: client (para 12) vs OP (paras 14, 17) — DISPUTED.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-017, EV-058, EV-059, EV-061, EV-064, EV-073, EV-076, EV-084. Standard: EQ-4, EQ-9. Kanoon: CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: Yeh dalil di ja sakti hai ki OP ke apne bayan ke mutabiq raat ko CCU duty doctor ki jaanch pulse, saturation aur auscultation tak simit thi (WS paras 12, 14), aur us shaam ka koi dated ECG record par nahi (ladder step 2). Complaint ka "no BP" 9 pm ke liye na kahein (Ann-5 mein 9 pm ka BP hai — W-28). Chest pain DISPUTED hai — ise tathya ki tarah na rakhein. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (WS se) "Shikayat mild breathlessness thi; pulse/SpO2 normal; chest pain nahi." → OP ke apne Ann-2 mein shuruati shikayat chest pain thi; dated ECG aur doctor ke notes maangna; expert. | **RELIEF + SEEMA:** RH-01 se juda; akele yeh point breach saabit nahi karta.

POINT 7: Death certificate mein OP ne khud "Coronary Artery Disease" likha; aspiration ka aadhar (intubation finding) record par nahi; WS "secondary to Aspiration" kehta hai, certificate "Aspiration Pneumonia"

ELEMENT TAG: Causation | **Taakat:** Moderate (CAD ko apne paksh mein rakhne aur aspiration ke aadhar par sawal ke liye); maut ka samay ab vivaad nahi (12:51 AM)

1. HAMARI BAAT / AAROP: MCCD (C-11, PDF p.61; OP Ann-9, D-08 p.22; EV-044, EV-082 — ek hi document, ek hi lineage): "I (a) TYPE I RESPIRATORY FAILURE ... (b) ASPIRATION PNEUMONIA ... (c) CORONARY ARTERY DISEASE"; "II PULMONARY TUBERCULOSIS"; "Manner of Death: Natural"; "NOT WILLING FOR AUTOPSY" — SROT-SAMARTHIT TATHYA. Complaint para 19 aur ground F (EV-020, EV-021) poore cause ko nakaarti hai aur "deliberately recorded an incorrect cause of death to conceal" kehti hai — ALLEGED. Complaint para 18: maut "around 12:10–12:15 AM", certificate "incorrectly records ... 1:53 AM" (EV-019).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.8 para 18; pp.8–9 para 19; p.10 para 20-F — poore para padhe]**: "The Medical Certificate of Cause of Death (MCCD form) clearly and correctly states the time of death as 12:51 AM. The nursing vital chart records that the patient was received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered placing the time of death at approximately 12:51 AM" (para 18); "'Type I Respiratory Failure, secondary to Aspiration' is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation, where particulate matter and fluids were visually identified in the trachea"; "Aspiration is not limited to the aspiration of solid food ... The patient was consuming water and prescribed medication" (para 19); "There was no manipulation of the death record." (20-F).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — samay 12:51 AM RECORD SE SAMARTHIT (Ann-5 nursing note "At 12:51"); intubation finding RECORD SE SAMARTHIT NAHI (koi resuscitation/intubation record annex nahi).
- Express Admission: P 12:15 AM par "received" hue aur 35 min CPR ke baad 12:51 par mrit (WS 18); certificate mein CAD (Documentary Support).
- Express Denial: death record mein manipulation (20-F).
- Omission: intubation/resuscitation ka record (G-20).
- Virodhabhas: X-26 — WS "secondary to Aspiration" (intubation ke waqt ki ghatna) vs MCCD "(b) ASPIRATION PNEUMONIA" (pneumonia = infection) — Possible Inconsistency; expert EQ-5. X-02 SULJHA: certificate ka samay 12:51 AM (OP ke TIER-A record se); parivar ka 00:10–00:15 = jab P unresponsive mile/laaye gaye (Ann-5 "Received in CCU at 12:15") — dono saath sahi ho sakte hain.
- Hamari kamzori: complaint ka "1:53 AM" padhna galat tha (W-23); C-6 mein "Nausea vomiting x 4 days" (W-09) — OP isi ka istemal karta hai (WS 19).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-044, EV-082, EV-062, EV-063, EV-066, EV-077, EV-032. Expert: EQ-5, EQ-6. "Concealment"/"manipulation" ka aarop language ladder ke step 5 ki teeno shartein poori nahi karta — na uthayein. Kanoon: CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "OP's own Medical Certificate of Cause of Death lists 'Coronary Artery Disease' in the chain of causes (EV-044; C-11 / OP Ann-9), consistent with its CCU diagnosis 'CAD – USA (Wellen's pattern)' (EV-074)." Yeh dalil di ja sakti hai ki OP ka "aspiration" ka aadhar (intubation par mili cheez) record par nahi, aur WS ka "secondary to Aspiration" certificate ke "Aspiration Pneumonia" se alag hai (Possible Inconsistency) — expert ke saamne rakhein. Samay ka vivaad na uthayein (12:51 AM; W-23). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (WS se) "Intubation par aspiration dikha; parivar ne autopsy nahi karwayi; maut natural." → intubation/resuscitation record maangna (G-20); CAD wali entry apne paksh mein; expert. | **RELIEF + SEEMA:** causation ka nishkarsh expert ke bina nahi; yeh point dil ki bimari aur aspiration ke aadhar par sawal tak simit hai.

POINT 8: Causation — kami (deviation) aur maut ke beech ki kadi: ab sequence OP ke record se documented, par expert opinion zaroori

ELEMENT TAG: Causation | **Taakat:** Weak–Moderate — expert opinion nahi, autopsy nahi, kai purani bimariyan; par OP ke apne record se ghatnakram saaf hai

1. HAMARI BAAT / AAROP: Complaint para 21 (EV-022): OP ke "gross negligent acts and omissions ... directly resulted in the death" — ALLEGED. Round 2 ke baad OP ke apne record se ghatnakram: CCU diagnosis "CAD – USA (Wellen's pattern)" + "anaemia", plan "correct anaemia" (EV-074); Hb 7.5 (EV-078); blood "kept ready" par "never released or issued" (EV-079, EV-055); P CCU se trolley par (EV-056, EV-057); 9 pm ke baad koi vital entry nahi, 12:15 par "unresponsive & pulse was not palpable" (EV-076, EV-077); 12:51 mrit (EV-062) — har kadi apne label ke saath neeche table mein.

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.2 PO-IV; p.3 para 1; p.11 para 21 — poore para padhe]**: "the death of the patient is attributable entirely to the cumulative and progressive deterioration of these underlying conditions, involving multiple organ systems lungs (TB), liver (DILI), heart (CAD)" (PO-IV); "resulting in multi-organ dysfunction over several months" (para 1); "foreseeable consequence of ... multi-organ dysfunction" (para 21).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: JAANCH AVASHYAK — "entirely" ka koi expert ya autopsy aadhar annex nahi.
- Express Admission: CAD maut ke kaaranon mein (PO-IV; MCCD EV-044).
- Express Denial: negligence se maut (para 1).
- No Reply: LAGU NAHI.
- Virodhi ke counter-causes (hamare aur OP ke record se): PTB on ATT, T2DM, smoker, hyponatraemia, hypoalbuminaemia, ATT hepatitis/DILI, B/L pleural effusion, anaemia, troponin negative, aspiration (EV-025, EV-026, EV-030–EV-033, EV-037, EV-044, EV-048, EV-072).

4. KANOONI AADHAR AUR SABOOT: Causation chain (har kadi alag):

Kadi	Record kya dikhata hai	Status	OP ka counter-cause is kadi par	Kya tay karega
1	OP ne "CAD – USA (Wellen's pattern)" + anaemia maana, CCU mein rakha (EV-074, EV-083)	DOCUMENTED	Trop/TNI negative, BNP 37.4 (EV-072)	Cardiologist (EQ-1, EQ-9)
2	Plan "correct anaemia"; Hb 7.5; blood taiyar par issue nahi (EV-074, EV-078, EV-055, EV-079)	DOCUMENTED (non-issue ADMITTED)	Hb > 7; "standby"; parivar lene nahi aaya (EV-049, EV-079)	EQ-3; X-21
3	CCU se trolley par; 9 pm ke baad entry nahi (EV-057, EV-076)	ADMITTED (shifting) / RECORD GAP (observation)	"monitored even on the trolley" (EV-057)	Trolley chart (G-18); EQ-4, EQ-13
4	12:15 par unresponsive, pulse nahi; CPR; 12:51 mrit (EV-077, EV-062)	DOCUMENTED	Aspiration, purani bimariyan (EV-063, EV-048)	EQ-5, EQ-6
5	Kadi 2–3 ki kami se maut hui ya jaldi hui	UNKNOWN	Natural progression (PO-IV)	Expert (EQ-6, EQ-13); autopsy nahi hua

Sabse kamzor kadi: 5. Kanoon: causation ka standard — CIT-10, CIT-11 leads, NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: Yeh dalil di ja sakti hai ki OP ke apne record ka ghatnakram — cardiac diagnosis, "correct anaemia" ka plan jo poora nahi hua, CCU se trolley par shifting, aur 9 pm se 12:15 tak koi recorded observation nahi — maut se pehle ke ghanton ka hai (kadi 1–4, DOCUMENTED/ADMITTED). In kamiyon ne maut mein kitna yogdaan diya (kadi 5), yeh real expert batayega ("expert opinion required"); bura parinaam ya adhoora record dekh kar final negligence ka nishkarsh nahi maanga ja sakta (core rule 6). | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (WS se) "Maut entirely purani bimariyon se." → "entirely" ka koi expert aadhar record par nahi (OP ka evidence band, POINT 4); apportionment ka sawal expert se (EQ-6). | **RELIEF + SEEMA:** bina expert ke causation par nishkarsh nahi; RH-01 isi par tika hai.

POINT 9: Maut aur legal heirs; rakam ka daava

ELEMENT TAG: Damage | **Taakat:** Strong (maut documented) / Weak (rakam ke saboot)

1. HAMARI BAAT / AAROP: MCCD (EV-044, EV-082): "admitted in this hospital ... on 11-3-26 and expired on 13-3-26" — SROT-SAMARTHIT TATHYA. Memo of parties (EV-010): C1–C6 "All being the legal heirs of Sh. Manohar (since deceased)"; vakalatnama par chhahon ke naam aur number (EV-045). Para 21 (EV-022): P "small clothing shop" se kamata tha; "a sum of Rs. 60,00,000/- is claimed" — ALLEGED. Prayer (EV-023): Rs. 60,00,000 + 12% p.a.; Rs. 2,50,000; Rs. 1,00,000.

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.3 PO-VII; p.11 para 21; pp.11–12 prayer — poore para padhe]**: "The claim of compensation amounting to Rs. 60,00,000/- (Rupees Sixty Lakhs only) is emphatically denied" (PO-VII); "The sympathy and loss felt by the Complainants are acknowledged"; "The claim of Rs. 60,00,000/- (Rupees Sixty Lakhs) is wholly excessive, speculative, and unsupported by any documentary evidence of income or financial dependency." (para 21); prayer: "Award exemplary costs against the Complainants, in view of the frivolous, vexatious, and mala fide nature of the present complaint" (prayer ii).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — "income ka documentary evidence nahi" RECORD SE SAMARTHIT (hamare bundle mein aay ka koi kagaz nahi, G-09); "frivolous, vexatious" RECORD SE SAMARTHIT NAHI (OP ke apne kai documents hamare points ka samarthan karte hain).
- Express Admission: complainants ka nuksaan aur sympathy "acknowledged" (para 21) — dhyan: yeh liability ki admission nahi.
- Express Denial: Rs. 60,00,000 ka daava (PO-VII, para 21).
- No Reply: prayer (ii) Rs. 2,50,000 aur (iii) Rs. 1,00,000 par alag se jawab nahi; heirs/rishton par koi aapatti nahi.
- Seema: C4–C6 ka rishta (X-05), legal heir certificate (G-11), umar 55/60 (X-09; OP ke sab records 60 kehte hain — EV-036, EV-083).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-044, EV-082, EV-010, EV-045, EV-022, EV-023, EV-043, EV-083, EV-069, EV-071. Kanoon: CIT-12, CIT-13, CIT-14 (OP ne inhe "admitted as settled law" kaha, WS 20-H; phir bhi official text se verify), CIT-16 — NOT VERIFIED. Rakam: Damages Ledger RH-01 se RH-05 (Part C).

5. VAKIL KE LIYE ANTIM DALIL: "The patient died at PGIMER on 13.03.2026 (EV-044); the complainants sue as his legal heirs (EV-010), and the OP does not dispute their standing." Harm DOCUMENTED. Rakam: claimed principal Rs. 63,50,000 + 12% p.a. byaj (EV-023; code se jod); documents se OP ko bhugtan Rs. 2,150 (EV-083, EV-043); aay aur dependency ke kagaz abhi nahi — SUPPORT NAHI. Koi rakam "milegi" ki tarah na likhein; yeh Commission ka vivek hai. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (WS se) "Excessive; income ka saboot nahi; exemplary costs do." → G-09–G-12 ke documents; claim aur supported alag dikhayein; unsupported aaropon ko chhod kar exemplary costs ka khatra kam karein (W-26). | **RELIEF + SEEMA:** RH-01–RH-05 (Part C); supported Rs. 2,150.

POINT 10: Consumer aur "service" — OP ke apne record mein Rs. 2,000 advance aur Rs. 150 POS; WS mein consumer status par koi aapatti nahi

ELEMENT TAG: Maintainability | **Taakat:** Strong (facts aur No Reply); kanoon NOT VERIFIED

1. HAMARI BAAT / AAROP: OP ka inpatient record cover: "Adv. Rs. 2000.00 Receipt No. [..]/1 Date: 11-Mar-2026" (OP Ann-10, D-08 p.23; EV-083) — SROT-SAMARTHIT TATHYA. PGIMER Test Receipt: "TOTAL AMOUNT 150.00", "POS/150 RUPEES", "CATEGORY : GENERAL", "12/03/2026 16:47:39" (C-10, idx p.58; EV-043) — SROT-SAMARTHIT TATHYA. Proforma: "Total Consideration Paid Rs. 2,50,000" (EV-008) — ALLEGED (bills nahi).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]**

[WS Location: WS p.1 PO-I; p.3 para 2 — poore para padhe; WS pp.1–12 mein consumer status par koi aur para nahi]: "No deficiency in service, as defined under Section 2(11) of the Consumer Protection Act, 2019" (PO-I); "The invocation of jurisdiction ... is resisted, as the foundational condition for such jurisdiction i.e., a provable deficiency of service does not exist" (para 2).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: LAGU NAHI — OP ne consumer status ko chunauti hi nahi di.
- Express Admission: LAGU NAHI — seedhe shabdon mein nahi.
- Express Denial: sirf deficiency ka (PO-I, para 2).
- No Reply: consumer status, consideration, free service, pecuniary jurisdiction — kahin nahi (WS pp.1–12 padhe).
- Hamare record ka samarthan: EV-083 (Rs. 2,000), EV-043 (Rs. 150) — kul Rs. 2,150 OP ko (code se).
- Seema: GMSH-16 ka "Ayushman Bharat" billing (EV-025, EV-026, EV-030) GMSH-16 ka hai, OP ka nahi.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-083, EV-043, EV-036, EV-008, EV-046, EV-051. Kanoon: CPA 2019 s.2(7), s.2(42) (CIT-01, CIT-02); *Indian Medical Association v. V.P. Shantha*, (1995) 6 SCC 651 (CIT-09); aapatti na uthane ka asar (CIT-20) — sab NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "The OP's own inpatient record shows an advance of Rs. 2,000 taken on admission on 11.03.2026 (EV-083; OP Ann-10), and its receipt shows Rs. 150 charged for cross-matching on 12.03.2026 (EV-043; C-10, idx p.58). The written statement raises no objection to the complainants' status as consumers." Yeh dalil di ja sakti hai ki seva bhugtan ke badle thi aur "service" ki paribhasha mein aati hai — kanoon NOT VERIFIED. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (forecast, PLAUSIBLE) arguments mein naya objection "sarkari sanstha, nominal charges". → OP ke apne Ann-10 aur C-10 receipts; WS mein yeh objection nahi (CIT-20). | **RELIEF + SEEMA:** maintainability bachaane ka point; jeet ka point nahi.

POINT 11: Jurisdiction aur limitation — OP ne limitation aur territorial jurisdiction "admitted" likha

ELEMENT TAG: Maintainability | **Taakat:** Strong — Express Admission; pecuniary par aapatti nahi; kanoon NOT VERIFIED

1. HAMARI BAAT / AAROP: Para 22 (EV-022): "the cause of action arose on 13.03.2026"; filing 07.04.2026 (EV-006, EV-007) — 25 din baad (code se). Para 23 (EV-022): C1–C3 Dhanas, Chandigarh; OP Sector 12, Chandigarh. Commission ne complaint "admitted for hearing" ki (EV-001).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.11 paras 22, 23, 24 — poore para padhe]**: "The contents of Para 22 with regard to the date of the cause of action (13.03.2026) and the period of limitation are admitted." "The contents of Para 23 regarding territorial jurisdiction are admitted." "The contents of Para 24 are admitted to the knowledge of the Opposite Party."

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: RECORD SE SAMARTHIT (Consistent Record).
- Express Admission: cause of action ki tarikh, limitation, territorial jurisdiction (EV-070).
- Express Denial: LAGU NAHI.
- No Reply: pecuniary jurisdiction par koi aapatti nahi.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-001, EV-006, EV-007, EV-022, EV-070. Kanoon: CPA 2019 s.34 (CIT-03), 2021 Jurisdiction Rules (CIT-04), s.69 (CIT-08) — LEGAL PROPOSITION, NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "The OP expressly admits the date of the cause of action (13.03.2026), limitation and territorial jurisdiction (WS p.11, paras 22–23)." Limitation: 25 din — SAFE (DL-07). Pecuniary: agar aadhar "consideration paid" hai (CIT-03, verify), to documented Rs. 2,150 ya stated Rs. 2,50,000 District Commission ki seema ke andar; OP ne yeh mudda uthaya hi nahi. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (forecast, SPECULATIVE) arguments mein "claim Rs. 50 lakh se zyada" ka naya mudda. → WS ki admissions aur s.34 ka official text. | **RELIEF + SEEMA:** maintainability ka bachaav; merits par koi nishkarsh nahi.

POINT 12: Records aur CCTV — prayer (iv) aur (v) par WS chup; OP ne kai records ka hawala diya par annex nahi kiye

ELEMENT TAG: Procedural | **Taakat:** Moderate (tactical) — records ki maang ka kram ab saaf list ke saath

1. HAMARI BAAT / AAROP: Prayer (iv): "the details of the doctors on duty in the CCU and the ward on 12.03.2026 and 13.03.2026 may be summoned"; prayer (v): "preserve all medical records ... along with the CCTV footage ... CCU unit, Ward and the 3rd Floor corridor ... 12.03.2026 to 13.03.2026 ... in a sealed cover" (EV-023) — SROT-SAMARTHIT TATHYA (jo maanga gaya). Orders D-02 se D-06 mein in par koi aadesh nahi (EV-001–EV-005).

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]**

[WS Location: WS pp.1–12 poora padha; prayers (iv)/(v) par koi para nahi; hawale: p.1 PO-II; p.5 para 7; p.6 para 10; p.2 PO-IV; p.9 para 20-a]: OP ke WS mein records ke hawale jo annex nahi: "Para-wise Comments submitted to the Medical Grievances Committee" (PO-II); "The vital chart of the CCU as well as the Ward ... was duly maintained" (para 7); "all patients are clinically assessed prior to shifting ... this was duly done" (para 10); "The patient's relatives were repeatedly informed of his very poor overall prognosis" (PO-IV); angiography deferral "was communicated to the family" (20-a).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: RECORD SE SAMARTHIT NAHI — in hawalon ke documents D-08 mein nahi.
- Express Admission: LAGU NAHI.
- Express Denial: LAGU NAHI.
- No Reply: prayer (iv) (duty doctors ki details) aur (v) (records aur CCTV ka preservation/deposit) par koi jawab nahi.
- OP ne kya diya: Ann-1 se Ann-12 (selected pages). Kya nahi diya: G-17 (committee comments), G-18 (ward/trolley chart), G-19 (shifting/transfer note), G-20 (resuscitation/intubation record), G-21 (protocols), G-22 (counselling/consent notes), G-23 (echo aur radiology reports), G-24 (dated ECGs); inpatient file ka sirf cover (Ann-10); CCTV.
- Dhyan: C-10 slip par "Blood Requisition Details Modified successfully" likha hai (EV-042) — requisition ka modification history (audit log) maangna; isse koi aarop nahi banta.

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-023, EV-001–EV-005, EV-046, EV-048, EV-054, EV-056, EV-064, EV-083. Production/preservation direction ka procedure: NOT VERIFIED [provision: advocate to confirm]. Adverse inference sirf maang aur inkaar record par aane ke baad (missing_record_strategy step 5) — NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: Yeh dalil di ja sakti hai ki OP ne apne WS mein jin records par bharosa kiya (committee ke comments, ward chart, shifting assessment, counselling) unhe pesh nahi kiya, aur prayer (iv)–(v) ka jawab nahi diya; in records ki maang Commission ke saamne rakhi jaaye (POINT 12 ki list). Absence se tampering ya concealment ka aarop na lagayein. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (forecast, PLAUSIBLE) "Jo zaroori tha woh annex kar diya." → WS ke apne hawalon ki list (upar) aur OP ka evidence band (POINT 4). | **RELIEF + SEEMA:** production/preservation ki direction; yeh khud deficiency ka saboot nahi.

POINT 13: 45-din wala written statement point — CONDITIONAL

ELEMENT TAG: Procedural | **Taakat:** Weak — service report nahi; lead point nahi

1. HAMARI BAAT / AAROP: Order 22.04.2026 (EV-001): "file written statement within 30 days from the date of receipt of notice extendable for 15 days (total 45 days)"; 29.05.2026: "Vakalatnama on behalf of OP has been filed" (EV-002); WS "Dated: 26.06.2026", verified "on the 26th day of June, 2026" (WS p.12; EV-071); 01.07.2026: "Written version on behalf of OP filed" (EV-003) — SROT-SAMARTHIT TATHYA. Notice ki receipt date record mein nahi (G-03) — UNRESOLVED.

2. VIRODHI (PGIMER) KA JAWAB **[JAWAB UPLABDH NAHI — MOOL WS SE SATYAPAN AVASHYAK]**
[WS Location: WS pp.1–12 poora padha — notice ki receipt ya WS ki samay-seema par koi para nahi]

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: JAANCH AVASHYAK.
- LAGU NAHI — Express Admission, Express Denial aur No Reply: WS ne is mudde ko chhua hi nahi.
- Do tarikhen alag: WS par 26.06.2026; registry/order mein filing 01.07.2026 (READ_FIRST ke mutabiq 45 din court filing tak gine jaate hain — kanoon NOT VERIFIED).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-001, EV-002, EV-003, EV-071. Kanoon: CPA 2019 s.38 (30 + 15 din) (CIT-06) NOT VERIFIED; Supreme Court ka 45 din par sakht rukh (CIT-07, memory lead) NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: Ganana (code se, exclusion rule): filing 01.07.2026 ke aadhar par WS late tabhi jab notice 16.05.2026 ya pehle mila (inclusion: 17.05.2026); WS-tarikh 26.06.2026 ke aadhar par 11.05.2026 ya pehle (inclusion: 12.05.2026). OP ke vakil 29.05.2026 ko maujood the (EV-002). Service report ke bina yeh point sirf CONDITIONAL hai — ise lead point na banayein. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (forecast, PLAUSIBLE) "Notice der se mila; WS samay par tha." → service report (G-03). | **RELIEF + SEEMA:** WS ko record se hataane jaisi maang sirf tab, jab ganana aur kanoon dono verify hon.

POINT 14: 12/13.03.2026 ki raat — trolley par 9 pm ke baad koi vital entry nahi; 12:15 AM par P CCU mein "unresponsive & pulse was not palpable" laaye gaye; 12:51 AM par mrit ghoshit

ELEMENT TAG: Breach, Causation | **Taakat:** Strong–Moderate — OP ka apna nursing note + WS para 18 ki admission; note ke kuch shabd low-confidence; pehle pata chalne se parinaam badalta ya nahi = expert

1. HAMARI BAAT / AAROP: OP ke CCU vital chart ki aakhri regular entry: 12/3 "9 pm | 90/60 | 82 | 20" (OP Ann-5, D-08 p.17; EV-076). Usi chart par raat ka note: "pt Received in CCU at 12:15 ... Pt was unresponsive & pulse was not palpable. CPR started as per ACLS protocol ... Atropine 0.6 mg & [Adrenaline?] 1 mg IV given. At 12:51 ... Pt declared dead by on duty SR." (EV-077; mukhya shabd MODERATE confidence, baaki LOW) — SROT-SAMARTHIT TATHYA (confidence note ke saath). Interval 9 pm → 12:15 AM = 3 h 15 min (code se) — is beech Ann-5 mein koi entry nahi: RECORD GAP (ladder step 2). OP ke mutabiq is samay P CCU se bahar trolley par the (WS paras 10, 11, 14; EV-056, EV-057, EV-059). Complaint para 18 (EV-019): parivar ne ~12:10–12:15 AM P ko "unresponsive" paaya — CLIENT-REPORTED, OP ke note se mel khaata hai.

2. VIRODHI (PGIMER) KA JAWAB **[WS-MOOL-TEXT]** **[WS Location: WS p.8 para 18; pp.6–7 para 11; p.9 para 20-b — poore para padhe]**: "The nursing vital chart records that the patient was received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered placing the time of death at approximately 12:51 AM" (para 18); "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff." (para 11); "The patient continued to be monitored even on the trolley." (20-b).

3. HAMARA JAWAB:

- Record ke hisaab se virodhi ki baat: AANSHIK — 12:15 par "received", CPR, 12:51 RECORD SE SAMARTHIT (Ann-5 note); "monitoring ... even on the trolley ... evidenced by the vital chart" RECORD SE SAMARTHIT NAHI (annexed chart mein 9 pm ke baad entry nahi).
- Express Admission: P 12:15 AM par CCU mein "received" hue; 35 min CPR; 12:51 AM mrit (WS 18; EV-062).
- Express Denial: LAGU NAHI — WS trolley par nigrani ka dava karta hai, inkaar nahi.
- Omission: trolley/ward ka koi vital chart ya nursing note annex nahi (G-18); P ko kisne, kahan, kis samay unresponsive paaya — WS mein nahi.
- Hamare record ka samarthan: EV-076, EV-077 (OP ka apna Ann-5); EV-019 (parivar ka samay).
- Virodhabhas: X-24 — "conclusively evidenced by the vital chart" vs chart mein 21:00 → 00:15 koi entry nahi (Possible Inconsistency; maasoom vyakhya: alag chart jo annex nahi).

4. KANOONI AADHAR AUR SABOOT: Saboot: EV-076, EV-077, EV-062, EV-057, EV-059, EV-065, EV-019. Standard (ACS ke marij ki trolley par nigrani ki frequency; kaun zimmedar): EQ-4, EQ-13. Kanoon: CIT-02 NOT VERIFIED.

5. VAKIL KE LIYE ANTIM DALIL: "OP's own nursing record states that the patient was received in the CCU at 12:15 AM on 13.03.2026, unresponsive and without a palpable pulse, and was declared dead at 12:51 AM after CPR (EV-077; OP Ann-5; WS p.8, para 18). The last vital-sign entry before that is at 9 pm on 12.03.2026 (EV-076): for 3 h 15 min the annexed chart records no observation, while, on the OP's own case, the patient lay on a trolley outside the CCU (WS paras 10–11)." Yeh dalil di ja sakti hai ki P ka cardiac arrest CCU ke bahar, trolley par, kisi recorded nigrani ke bina hua (RECORD GAP, step 2; shifting aur trolley Express Admission, step 4). "Monitoring nahi hui" ko tathya ki tarah na kahein; OP ke trolley/ward chart ki maang karein. | **VIRODHI KA SAMBHAVIT JAWAB AUR HAMARA UTTAR:** (RECORD-SUPPORTED ho sakta hai) "Trolley par nurse dekhti rahi; arrest achanak tha (CAD, PTB)." → aisi kisi entry ka record annex nahi (Omission); achanak arrest pehle pakda ja sakta tha ya nahi = expert (EQ-13). | **RELIEF + SEEMA:** observation mein deficiency ka nishkarsh — RH-01 se juda; causation expert ke saath.

5. HAMARI KAMZORIYAN AUR UNKA ILAAJ (kuch chhupaya nahi gaya)

W-id	Kamzori	OP iska kaise istemal karega	Severity	Ilaaj (cure)	Status
W-01	Hamara rejoinder ka mauka 14.10.2026 ko band (EV-005).	WS ke tathya "unrebutted" kahega (asar NOT VERIFIED).	SERIOUS	Written Arguments mein WS ka para-wise jawab (section 6; Part A, A6a); advocate procedure check karein.	OPEN
W-02	Hamare Written Arguments 3 din (16/17.10.2026) mein file hue ya nahi — record mein nahi (DL-05).	"Complainant ne arguments diye hi nahi."	SERIOUS	Advocate se filing ki pushti; na hui ho to 22.12.2026 se pehle uchit application (procedure NOT VERIFIED).	OPEN
W-03	Koi expert medical opinion record par nahi.	"Negligence saabit nahi; sirf bura parinaam" (WS PO-I, PO-II, para 1).	SERIOUS	Cardiology + transfusion medicine expert (EQ-1–EQ-13); OP ka bhi koi evidence affidavit nahi (POINT 4).	OPEN
W-04	C-11/Ann-9 par "NOT WILLING FOR AUTOPSY" aur C1 ke hastakshar; "NOT A MEDICOLEGAL CASE" (EV-044, EV-082).	"Parivar ne khud autopsy se mana kiya."	SERIOUS	Poore cause ko na nakaarein; CAD entry apne paksh mein; aspiration ka aadhar maangein; Q-04.	OPEN
W-05	C-11 kehta hai maut CCU mein hui.	(Round 1 ka dar)	COSMETIC	SULJHA: OP khud maanta hai P CCU se bahar trolley par the aur 12:15 par CCU mein unresponsive laaye gaye (WS 10, 11, 18; Ann-5).	RESOLVED
W-06	Raat ka bayan sirf C1 ke affidavit mein, jo ghar chala gaya tha (para 16).	"Suni-sunai baat."	CURABLE	OP ke apne Ann-5 note aur WS paras 14, 18 par tikein; Q-02, Q-21.	OPEN
W-07	Maa/patni ko "Complainant No.2" kaha (X-01).	Pleading ki vishwasniyata.	CURABLE	Written Arguments mein sahi number.	OPEN
W-08	Kai purani bimariyan (POINT 8).	"Death entirely due to underlying conditions" (PO-IV).	SERIOUS	Expert; apportionment ka sawal.	OPEN
W-09	C-6 mein "Nausea vomiting x 4 days" — "anorexia isliye aspiration nahi" wala tark kamzor.	WS para 19 yahi kehta hai.	SERIOUS (us tark ke liye)	Yeh tark chhod dein; intubation record maangein (G-20).	OPEN
W-10	Complaint poore certified cause (CAD sahit) ko nakaarti hai.	"CAD to complainant bhi nahi maanta."	CURABLE	Reframe: CAD OP ka apna likha (POINT 1, 7).	OPEN
W-11	"Concealment"/"manipulation" ke aarop bina saboot.	"Frivolous, vexatious, mala fide" + exemplary costs (WS prayer).	SERIOUS	Na dohraayein (ladder step 5 nahi).	OPEN
W-12	Overstatement: "more than ten times" (asal 9.76), "critically low haemoglobin", "Shockingly".	WS PO-III "grossly exaggerates".	COSMETIC	Sahi sankhya likhein.	OPEN
W-13	C-9 par naam/CR khaali; samay 8:30/5:30.	(Round 1 ka dar)	COSMETIC	SULJHA: OP maanta hai C-9 uska Emergency chart hai, entry "8:30 AM on 11.03.2026" (WS 7).	RESOLVED
W-14	Transfusion na hona client ka bayan tha.	—	COSMETIC	SULJHA: OP ne maana "never released or issued" (WS 8).	RESOLVED
W-15	Rakam: aay ka saboot nahi; Rs. 2,50,000 ke bills nahi.	"unsupported by any documentary evidence of income" (WS 21).	SERIOUS	G-09–G-12 ke documents.	OPEN
W-16	Consumer status par aapatti ka dar.	—	COSMETIC	WS mein aapatti nahi; Rs. 2,000 advance (Ann-10).	LARGELY CURED
W-17	Drafting ki galtiyan (verification, "2025", " (Annexure P-)", "Appellant", vakalatnama ki tarikh, "& ANR").	Formal aapatti.	CURABLE / COSMETIC	Advocate ki jaanch.	OPEN
W-18	Court page 59 PDF mein nahi; Labogram pages 2 & 4 nahi.	"Adhoore documents."	CURABLE	Filed set check.	OPEN
W-19	C-7 ECG copy par kaala dabba.	Copy par sawal.	CURABLE	Original (Q-07).	OPEN
W-20	Res ipsa loquitur par zor.	WS 20-G: "no application".	CURABLE	Documentary points par tikein.	OPEN
W-21	45-din point CONDITIONAL.	"WS samay par tha."	COSMETIC	Service report.	OPEN
W-22	Prayer (iv)/(v) par koi aadesh nahi; CCTV overwrite ho chuka ho sakta hai.	"Kabhi press nahi kiya."	SERIOUS	Pehle ki requests (Round 3); application.	OPEN
W-23	Complaint para 18: "death certificate incorrectly records ... 1:53 AM" — OP ka record 12:51 AM; hamari padhai galat thi.	Vishwasniyata par chot; "mala fide".	SERIOUS (agar dohraya)	Samay ka vivaad chhodein; parivar ka 12:10–12:15 = unresponsive milne ka samay (Ann-5 se mel).	OPEN
W-24	OP ka letter: blood "kept ready for issue around 12 p.m. However, no attendant came to collect the same" (Ann-7).	"Parivar ki laaparwahi; hamne blood taiyar rakha tha."	SERIOUS	Q-26, Q-27; letter baad ka (01.06.2026); OP ke doctor ne "no urgency" kaha (WS 15); slip "from the ward at the time of issue".	OPEN
W-25	BNP 37.4 (Ann-1) aur "(N) LVEF" (Ann-2) — heart failure/"severe cardiac distress" wali framing kamzor.	"Grossly exaggerates" (PO-III).	CURABLE	ACS/Wellens + anaemia par tikein (OP ka Ann-3); BNP vs NT-proBNP expert (EQ-10).	OPEN
W-26	OP ne complainants par "exemplary costs" maange.	Unsupported aaropon ko dikhayega.	CURABLE	Bina saboot ke aarop (W-11, W-23) chhodein.	OPEN

W-id	Kamzori	OP iska kaise istemal karega	Severity	Ilaaj (cure)	Status
W-27	Complaint para 7 / ground A "no monitoring after 8:30 AM" OP ke CCU chart (Ann-5) ke viparit.	"Factually false" (WS 7).	SERIOUS (agar dohraya)	POINT 3 ka naya roop: gaps aur values.	OPEN
W-28	Complaint para 14 "no ... blood pressure monitoring" — Ann-5 mein 12/3 raat 9 pm ka BP 90/60 hai.	Hamare bayan ki sateekta par sawal.	CURABLE	Dava ko "9 pm ke baad koi entry nahi; ECG nahi" tak simit karein.	OPEN

Severity: FATAL / SERIOUS / CURABLE / COSMETIC. Abhi koi FATAL nahi. W-02 (WA filing) aur W-24 (blood collect na karna) sabse zaroori; dono par parivar/advocate ka jawab chahiye.

6. VIRODHI KE JAWAB KI JAANCH (AUDIT-OPP — OP ke apne annexures aur record se)

Har row: WS Location | OP ke shortest shabd [WS-MOOL-TEXT] | kis record se milaya | record-check label | ladder step | kya ise ek step upar le jayega | kahan istemal. OP ko jhootha maan kar nahi likha gaya; "knowingly false" (step 5) kisi row mein nahi.

WS Location	OP ke shabd	Record se milan	Label	Step	Upar kya le jayega	Use
WS p.1, PO-I	"received timely, appropriate, and guideline directed medical management"	Koi guideline annex nahi; Ann-3 plan "correct anaemia" poora nahi (EV-074, EV-055)	Unsupported Assertion	1	Expert opinion	POINT 2, 4
WS p.1, PO-II	"Para-wise Comments submitted to the Medical Grievances Committee ... conclusively demonstrated"	Comments annex nahi (D-08 pp.13–25)	Unsupported Assertion	1	Comments ki copy (G-17)	POINT 4, 12
WS p.2, PO-III	"grossly exaggerates the severity of the patient's cardiac condition"	Ann-3 "CAD – USA (Wellen's pattern)" (EV-074)	Contradiction	3	—	POINT 1
WS p.2, PO-III; p.4 para 3	NT-proBNP "recorded at GMSH-16"	C-3: WeCare Lab (private), "Sample Collected At: HR-11" (EV-029)	Possible Inconsistency	1	—	X-29
WS p.2, PO-III; p.5 para 6	"repeat BNP ... only 37.4 pg/mL" on 11.03.2026	Ann-1 print par naam/tarikh nahi; Ann-3 mein wahi values; MYO 176, D-dimer 3280 "abnormal" (EV-072)	Ambiguity / Omission	1	Lab system ki dated report	POINT 1; EQ-10
WS p.2, PO-IV	"co-existing Coronary Artery Disease (CAD)"	Ann-3, MCCD (EV-074, EV-044)	Express Admission	4	—	POINT 1, 7
WS p.2, PO-IV	"relatives were repeatedly informed of his very poor overall prognosis"	Koi counselling note annex nahi	Unsupported Assertion	1	Counselling notes (G-22); Q-28	POINT 12
WS p.2, PO-V	"only as a precautionary standby measure ... No medical indication for routine transfusion existed"	Ann-3 "correct anaemia"; Ann-6 Hb 7.5; Ann-7 "kept ready for issue" (EV-074, EV-078, EV-079)	Contradiction (indication) / Possible Inconsistency (standby)	3	Transfusion order/notes; expert	POINT 2
WS p.2, PO-V; pp.5–6 para 8	"The blood was never released from the Blood Bank"	Ann-7 register: issue khaane khaali (EV-080)	Express Admission	4	—	POINT 2
WS p.2, PO-VI	"admitted to the CCU ... at 10:10 AM on 11.03.2026 ... nearly 31 hours"	Ann-10 "11-Mar-2026 10:10:36" (EV-083); CCU chart 9:30 AM se (X-19)	Consistent Record	—	—	POINT 5 (shifting ~17:10)
WS p.2, PO-VI; p.9 20-a	"vitals ... monitored constantly"	Ann-5: 4–8 ghante ke gap (EV-076)	Possible Inconsistency	1	Monitor logs	POINT 3
WS p.2, PO-VI; p.9 20-a	"ECG was repeated ... showed NO ST elevations or any dynamic changes"	Ann-11/12 par tarikh/samay/naam nahi (EV-084); Ann-2 "T ... V2–V4 (new onset)" (EV-073)	Ambiguity	1	Dated ECGs (G-24); EQ-9	POINT 6
WS p.4, para 3	"only '?Wellens Syndrome' ... merely suspected and not confirmed"	C-8 "Imp - Wellens Syndrome"; Ann-3 " (Wellen's pattern)" (EV-037, EV-074)	Contradiction	3	—	POINT 1; X-27
WS p.4, para 3	"appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome"	C-6 (EV-032)	Express Admission	4	—	POINT 1
WS p.4, para 3	"8.2 g/dL ... did not meet the standard clinical threshold ... Hb < 7 g/dL per established guidelines"	OP ka apna Hb 7.5 (Ann-6) zikr nahi; guideline annex nahi	Omission / Unsupported Assertion	1	Guideline + expert (EQ-3)	POINT 2
WS pp.4–5, para 6	"BNP levels at PGIMER stood at 1117.48 pg/mL is denied and is a deliberate misrepresentation"	OP ke apne C-8 note mein "BNP - 1117.48 (24/2)" (EV-037)	Possible Inconsistency	1	—	POINT 1
WS p.5, para 6	"As per the existing protocol at PGIMER, all cardiac patients are shifted to CCU"	Protocol annex nahi	Unsupported Assertion	1	Protocol (G-21)	POINT 4
WS p.5, para 7	"Annexure C-9 ... is in fact the vital chart of the Emergency Department"; "8:30 AM on 11.03.2026"	C-9 (EV-040)	Express Admission	4	—	POINT 3
WS p.5, para 7	"The vital chart of the CCU as well as the Ward ... was duly maintained"	Sirf CCU chart annex (Ann-5)	Omission	1	Ward chart (G-18)	POINT 3, 14
WS pp.5–6, para 8	"admitted that a blood cross match procedure was completed ... on 12.03.2026"	C-10 12:55 (EV-041); Ann-7	Express Admission	4	—	POINT 2
WS p.6, para 8	"The six hour window ... would only commence from the time of issue which never occurred"	C-10 slip "within six hours of issue" (EV-042)	Consistent Record	—	—	POINT 2 (hamara para 8 ka tark kamzor)
WS p.6, para 10	"all patients are clinically assessed prior to shifting from CCU to the ward this was duly done"	Koi shifting note annex nahi	Unsupported Assertion	1	Shifting note (G-19)	POINT 5

WS Location	OP ke shabd	Record se milan	Label	Step	Upar kya le jayega	Use
WS p.6, para 10	"Any delay in ward bed availability is a well known systemic constraint ... cannot constitute negligence per se"	— (kanooni dalil)	Express Admission (deri) / LEGAL PROPOSITION	4	CIT-22	POINT 5
WS p.6, para 11	"It is admitted that there was a temporary delay in the availability of a ward bed"; "Even on the trolley"	C-10 "Trolly 66" (EV-041)	Express Admission	4	—	POINT 5
WS p.7, para 11	"as conclusively evidenced by the vital chart maintained by the nursing staff"	Ann-5: 21:00 → 00:15 koi entry nahi (EV-076, EV-077)	Possible Inconsistency	2	Trolley chart na ho to step 3	POINT 14
WS p.7, para 12	"pulse and oxygen saturation were checked ... no tachycardia or tachypnoea"	Ann-2 "Tachycardia", rate 123; C-8 PR 116 (EV-073, EV-037)	Possible Inconsistency	1	Us samay ka note	POINT 6; X-23
WS p.7, para 12	"Ward Doctor specifically advised the immediate re-shifting ... completely false allegation"	Client para 12 (EV-017)	Express Denial	—	Ward notes	POINT 5
WS p.7, para 14	"brought to the CCU only once at around 9pm on 12.03.26"	Ann-5 9 pm entry (EV-076)	Express Admission	4	—	POINT 5, 6, 14
WS p.8, para 15	"The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion"	Client para 16 (EV-018)	Express Admission (shabd kahe gaye)	4	—	POINT 2; W-24 ka jawab
WS p.8, para 17	"dual antiplatelets, statins, nitrates, and anticoagulants"	Ann-8: Brilinta, Ecosprin, Monit GTN, Clexane; statin nahi dikha (EV-081)	Possible Inconsistency (statin)	1	Poora drug chart	X-22
WS p.8, para 18	"time of death as 12:51 AM"; "received at approximately 12:15 AM ... 35 minutes of ... CPR"	Ann-5 note (EV-077); C-11/Ann-9	Express Admission / Consistent Record	4	—	POINT 7, 14
WS pp.8–9, para 19	"particulate matter and fluids were visually identified in the trachea"	Resuscitation/intubation record annex nahi	Unsupported Assertion	1	Resuscitation record (G-20)	POINT 7
WS p.8, para 19	"'Type I Respiratory Failure, secondary to Aspiration'"	MCCD "(b) ASPIRATION PNEUMONIA" (EV-044)	Possible Inconsistency	1	EQ-5	POINT 7; X-26
WS p.9, 20-a	"Coronary angiography was deferred ... communicated to the family"	Koi note annex nahi	Unsupported Assertion	1	Notes (G-22); Q-28; EQ-11	POINT 12
WS p.10, 20-E	"The vital chart conclusively establishes hemodynamic stability throughout the admission"	Ann-5 values (EV-076)	Possible Inconsistency	1	EQ-8	POINT 3; X-25
WS p.10, 20-G	"The Opposite Party owed a duty of care to the deceased"	—	Express Admission	4	—	POINT 1
WS pp.10–11, 20-H	three judgments "are admitted as settled law governing compensation in medical negligence matters"	CIT-12, CIT-13, CIT-14	Express Admission (legal)	4	Official text	POINT 9
WS p.11, para 21	"unsupported by any documentary evidence of income or financial dependency"	Hamare bundle mein aay ka kagaz nahi (G-09)	Consistent Record (hamari kamzori)	—	Documents	POINT 9; W-15
WS p.11, paras 22–23	limitation and territorial jurisdiction "admitted"	—	Express Admission	4	—	POINT 11
WS (poora)	Consumer status, free service, pecuniary jurisdiction; prayers (ii)–(v)	—	No Reply	—	Asar NOT VERIFIED (CIT-20)	POINT 10, 12

Self-goals (OP ke apne kagaz jo hamare haq mein hain, sandarbh ke saath): Ann-3 diagnosis "CAD – USA (Wellen's pattern)" aur plan "correct anaemia"; Ann-6 Hb 7.5; Ann-7 "kept ready for issue"; Ann-5 "Received in CCU at 12:15 ... unresponsive"; Ann-10 advance Rs. 2,000. Har ek ka sandarbh: yeh OP ke records hain; inka clinical arth (jaise "correct anaemia" ka matlab transfusion tha ya nahi) expert tay karega.

7. TIMELINE AUR RECORD GAPS

Poora Master Timeline Part C mein. Neeche 10–13 March ka plain-text graph (samay code se; ZIP_1 + ZIP_2 ke padhe gaye pages ke hisaab se):

```
RECORD TIMELINE (padhe gaye pages: ZIP_1 + ZIP_2)

10.03.2026 13:07  ●  Hb 8.2 g/dL (GMSH-16)                                  [DOCUMENTED]   EV-033
11.03.2026 00:14  ●  GMSH-16 se "PGI Emergency" refer (ECG: Wellens)        [DOCUMENTED]   EV-032
11.03.2026 02:04  ●  PGIMER EMOPD registration; "Imp - Wellens Syndrome"     [DOCUMENTED]   EV-036/037
11.03.2026 03:21  ●  Chest X-ray (sirf image)                               [AS RECORDED]  EV-075
11.03.2026 05:00, 08:30 ● Emergency BP chart (110/70; 120/88)               [AS RECORDED]  EV-040
11.03.2026 09:30  ●  CCU chart pehli entry: 113/73, pulse 128               [AS RECORDED]  EV-076
11.03.2026 10:10  ●  CCU admission; advance Rs. 2,000                       [DOCUMENTED]   EV-083
11.03.2026 (din)  ●  "CAD - USA (Wellen's pattern)", "anaemia"; plan "correct anaemia" [DOCUMENTED] EV-074
11.03.2026 (din)  ●  PGIMER Hb 7.5 g/dL                                     [DOCUMENTED]   EV-078
11.03.2026 18:00  ●  BP 90/49, pulse 106                                    [AS RECORDED]  EV-076
11.03.2026 21:00  ●  BP 130/79, pulse 110                                   [AS RECORDED]  EV-076
                  ├── RECORD GAP: 21:00 (11/3) se 01:00 (12/3) tak - 4 h 00 min (code) - vitals entries NAHI MILI
12.03.2026 01:00  ●  BP 110/74, pulse 100                                   [AS RECORDED]  EV-076
                  ├── RECORD GAP: 01:00 se 09:00 tak - 8 h 00 min (code) - vitals entries NAHI MILI
12.03.2026 09:00  ●  BP 87/46, pulse "45" [LOW]                             [AS RECORDED]  EV-076
12.03.2026 09:15  ●  Blood requisition ACC blood storage mein received      [DOCUMENTED]   EV-079
12.03.2026 ~12:00 / 12:55 ● PRBC crossmatch, "kept ready for issue"         [DOCUMENTED]   EV-079/041
12.03.2026 13:00, 15:00 ● 110/73, 82; 9?/60, 114                            [AS RECORDED]  EV-076
12.03.2026 16:47  ●  Receipt Rs. 150 (crossmatching)                        [DOCUMENTED]   EV-043
12.03.2026 ~17:10 ●  CCU se shifting (10:10 + "nearly 31 hours", code)      [INFERRED]     EV-050/056
12.03.2026 shaam  ●  Ward bed nahi; P trolley par                           [DOCUMENTED - OP admission] EV-057
12.03.2026 ~21:00 ●  "brought to the CCU only once"; BP 90/60, pulse 82     [DOCUMENTED]   EV-059/076
12.03.2026 ~21:00 ●  ACC blood bank band (parivar)                          [ALLEGED]      EV-018
                  │
                  ├─── RECORD GAP ──────────────────────────────────────┐
                  │  12.03.2026 21:00 se 13.03.2026 00:15 tak (midnight    │
                  │  ke paar) - 3 h 15 min (code se)                       │
                  │  vitals/nursing entries PADHE GAYE PAGES MEIN NAHI MILI │
                  │  (OP ke mutabiq P is samay trolley par the)            │
                  └────────────────────────────────────────────────────────┘
                  │
13.03.2026 00:10-00:15 ● Parivar ne P ko unresponsive paaya                 [ALLEGED]      EV-019
13.03.2026 00:15  ●  "Received in CCU at 12:15 ... unresponsive & pulse was not palpable"; CPR [DOCUMENTED] EV-077
13.03.2026 00:51  ●  Mrit ghoshit (CPR ~36 min, code)                       [DOCUMENTED]   EV-077/062

YEH GRAPH KYA SAABIT NAHI KARTA: yeh sirf padhe gaye pages mein vital entries ki kami dikhata
hai; yeh nahi dikhata ki P ko kisi ne dekha hi nahi. CCU monitor logs aur trolley/ward chart
(jo annex nahi hue) dekhe bina "monitoring nahi hui" nahi kaha ja sakta. Shifting ka ~17:10
samay WS ke "nearly 31 hours" se nikala gaya hai (INFERRED), kisi record mein likha nahi.
```

8. EXPERT KE LIYE SAWAL

AI faisla nahi kar sakta; real expert ki rai zaroori. Expert advocate chunenge; conflict-of-interest check zaroori. Sawal neutral hain.

ID	Sawal (neutral)	Specialty	Expert ko kaunse records dekhne honge
EQ-1	C-8 aur OP Ann-2/Ann-3 ki entries (Wellens pattern, "CAD – USA", rate 123, BP 130/100, Hb 7.5–8.2, Trop/TNI negative) ke aadhar par, March 2026 mein ek tertiary public hospital ki CCU kis tarah ki aur kitni baar monitoring karti?	Cardiology (tertiary, CCU)	C-6, C-8, C-9; OP Ann-2, Ann-3, Ann-5, Ann-6, Ann-8, Ann-10
EQ-2	C-7 ke GMSH-16 ECG tracings (10.03.2026) mein woh pattern dikhta hai jise "Wellens" likha gaya?	Cardiology	C-7 ke original ECG
EQ-3	ACS ke ilaaj par chal rahe is marij mein Hb 7.5 g/dL (11.03.2026) par PRBC transfusion indicated tha ya nahi, kis urgency se? CCU plan "correct anaemia" ka sahi arth kya hai? Transfusion na hone ka (agar) kya asar ho sakta tha?	Transfusion Medicine + Cardiology	C-7, C-8, C-10; OP Ann-3, Ann-6, Ann-7, Ann-8
EQ-4	"CAD – USA (Wellen's pattern)" wale marij ko ~31 ghante baad CCU se shift karke ward bed ke intezaar mein trolley par rakhna reasonable practice ke mutabiq tha? Shifting se pehle kaunsa assessment aur trolley par kaunsi nigrani apekshit thi?	Cardiology / Critical care	OP Ann-3, Ann-5, Ann-10; WS paras 10–14; shifting note (G-19)
EQ-5	Certified cause (I(a) Type I RF, (b) aspiration pneumonia, (c) CAD; II PTB) recorded course se mel khaata hai? Intubation par "particulate matter and fluids" (WS para 19) aspiration pneumonia ka aadhar ho sakta hai? Autopsy na hone ka asar?	Internal medicine / Forensic medicine	C-6, C-11; OP Ann-5 note, Ann-9; resuscitation record (G-20)
EQ-6	Purani bimariyon ke saath, kisi kami ka maut mein yogdaan alag kiya ja sakta hai? Samay par sahi care se kya hota (counterfactual)?	Cardiology + Internal medicine	Poora D-01 + D-08
EQ-7	Troponin negative Wellens pattern ke risk aakalan ko kitna badalta hai?	Cardiology	C-6, C-8; OP Ann-1, Ann-3
EQ-8	OP ke CCU chart (Ann-5) ke values — pulse 125–128 (11/3), BP 90/49 (6 pm 11/3), 87/46 aur pulse "45" (9 AM 12/3), 90/60 (9 pm 12/3) — is marij mein instability dikhate the jis par escalation ya doctor ki review apekshit thi?	Cardiology / Critical care	OP Ann-5 (original), Ann-8
EQ-9	OP Ann-2 ki ECG entry (rate 123, T changes V2–V4 "new onset") aur Ann-11/12 ki tracings kya dikhati hain? Ann-11/12 kab ki hain (machine time)?	Cardiology	OP Ann-2, Ann-11, Ann-12 (originals with machine time)
EQ-10	Point-of-care BNP 37.4 pg/mL (Ann-1) aur lab NT-proBNP 1117.48 pg/mL (C-3, 24.02.2026) ki aapas mein tulna ki ja sakti hai?	Cardiology / Laboratory medicine	C-3, OP Ann-1
EQ-11	Coronary angiography ko "deferred" karne ke diye gaye kaaran (WS 20-a) is setting mein reasonable the? Iske baad kaunsi nigrani apekshit thi?	Cardiology	OP Ann-2, Ann-3, Ann-6; WS 20-a
EQ-12	Drug chart ke "Inj. Lasix 40 mg stat f/b 20 mg IV BD" aur "IVF @ 30 ml/hr" orders is marij ki halat ke baare mein kya batate hain?	Cardiology	OP Ann-8, Ann-2
EQ-13	9 pm (12/3) se 12:15 AM (13/3) tak trolley par regular nigrani hoti to kya arrest pehle pakda ja sakta tha aur parinaam badal sakta tha?	Cardiology / Critical care	OP Ann-5; trolley/ward chart (G-18)

Record par expert/committee report: WS PO-II Department of Cardiology ke "Para-wise Comments submitted to the Medical Grievances Committee" ka hawala deta hai — document annex nahi (G-17). Mile to test karein: kisne likha (ilaaj karne wala vibhag — independence), kaunse records dekhe, kya poocha gaya, kaunsa standard, aur kya deviation/harm/causation alag rakhe.

9. ABHI KYA CHAHIYE (records aur documents)

Record / document	Kis POINT ke liye	Step (missing_record_strategy)	Kiske paas	Priority
Hamare Written Arguments ki filing ka saboot (14.10.2026 ke baad)	Sab	—	Advocate	IMMEDIATE
OP ke Written Arguments (14.10.2026) ki copy	Sab	Registry se copy	Commission / OP	IMMEDIATE
Notice ki service report	POINT 13	Registry se copy	Commission	HIGH
Medical Grievances Committee ke Para-wise Comments aur committee ki report (G-17)	POINT 4, 12	Request → application	OP	HIGH
Ward/trolley vital chart aur nursing notes 12.03.2026 shaam se 13.03.2026 00:51 tak (G-18)	POINT 3, 5, 14	Request → reminder → application	OP	HIGH
CCU se shifting/transfer note aur bed allotment register (G-19)	POINT 5	Request → application	OP	HIGH
Resuscitation/intubation record 13.03.2026 (G-20)	POINT 7, 14	Request → application	OP	HIGH
PGIMER protocols jinka WS mein hawala hai (CCU triage; CCU se ward shifting; transfusion) (G-21)	POINT 2, 4, 5	Request → application	OP	MEDIUM
Counselling/consent notes (prognosis; angiography deferral) (G-22)	POINT 12	Request	OP	MEDIUM
Formal echo report aur radiology (X-ray) report (G-23)	POINT 1	Request	OP	MEDIUM
Machine time ke saath saare ECG (11–13.03.2026) (G-24)	POINT 6	Request	OP	MEDIUM
Blood requisition ka modification history / system audit log ("Blood Requisition Details Modified successfully", C-10) aur doctor ka transfusion order	POINT 2	Request	OP	MEDIUM
CCTV (CCU, ward, 3rd floor, 12–13.03.2026) — preservation request/order ka record	POINT 5, 12, 14	Pehle ki request ka saboot (Round 3); application	OP / Complainants	IMMEDIATE (overwrite risk)
Expert opinion	POINT 2, 3, 5, 7, 8, 14	Advocate ka faisla	Advocate	HIGH
Aay ka saboot, bills (Rs. 2,50,000), legal heir certificate, umar ka saboot	POINT 9, 10	User se	User	HIGH
Court page 59; Labogram pages 2 & 4; C-7 ECG ka original	POINT 1, 9	Filed set / user	Advocate / User	MEDIUM

10. RAKAM (claim) — saar

RH	Head	Claimed	Documents se supported	Kami / jokhim
RH-01	Loss of life, mental tension, agony, financial loss (prayer i)	Rs. 60,00,000	SUPPORT NAHI	Aay/dependency ka document nahi (OP: WS para 21)
RH-02	Pendente lite interest @12% p.a. on RH-01 (prayer i)	12% p.a. (12% of 60,00,000 = Rs. 7,20,000 per saal — sirf ganana)	SUPPORT NAHI	Kis tarikh se — nahi likha; kanoon NOT VERIFIED
RH-03	Medical expenses (prayer ii)	Rs. 2,50,000	Rs. 2,150 (Rs. 2,000 advance EV-083 + Rs. 150 EV-043)	Baaki bills nahi; GMSH-16 ilaaj Ayushman Bharat par (double counting ka khatra); OP ka alag jawab nahi (No Reply)
RH-04	Litigation expenses (prayer iii)	Rs. 1,00,000	SUPPORT NAHI	Fee receipts nahi; Commission ka vivek
RH-05	Non-monetary: duty doctors ki details; records/CCTV preservation (prayer iv, v)	—	—	WS mein No Reply; koi aadesh nahi (POINT 12)
Kul (code se)		**Rs. 63,50,000 + byaj**	**Rs. 2,150**	Award Commission ka vivek; OP ne exemplary costs maange hain

Poora Damages Calculation Ledger Part C mein. Kam/Base/Zyada scenario abhi nahi ban sakte: aay, umar aur dependants ka koi document nahi (LAGU NAHI — inputs nahi).

11. KANOON JO VAKIL KO VERIFY KARNA HAI

Is file ki saari 23 entries (CIT-01 se CIT-23) **NOT VERIFIED** hain — official sources is session mein network policy ki wajah se nahi khule (E-01). OP ne complaint ki teen judgments (CIT-12, CIT-13, CIT-14) ko "settled law" maana (WS 20-H) — yeh OP ki admission hai, verification nahi. Poori list Part C ke Citation Register mein. Koi final argument line kisi NOT VERIFIED authority par settled law ki tarah nahi tiki hai.

12. HEARING NOTES (provisional — Round 6 mein final)

60-second pitch (English, for the advocate): "PGIMER's own CCU record diagnosed this patient with 'CAD – USA (Wellen's pattern)' and anaemia and planned to 'correct anaemia'; its own haemogram showed Hb 7.5. Blood was cross-matched and kept ready, yet the OP admits it was never issued, and its own doctor told the family there was no urgency. About 31 hours after admission

the OP shifted him out of the CCU onto a trolley because no ward bed was free — that too is admitted. Its own chart records nothing after 9 pm; at 12:15 AM he was brought into the CCU unresponsive, without a pulse, and was declared dead at 12:51 AM. The question is whether this is the care PGIMER itself said he needed. [Expert opinion: ____ to be filled.]"

5-minute outline: (1) Maintainability admitted or unchallenged (POINT 10, 11). (2) OP's own diagnosis and admitted duty (POINT 1). (3) Blood: plan, Hb 7.5, cross-match, non-issue admitted (POINT 2). (4) Shifting to a trolley admitted (POINT 5). (5) No observation 9 pm → 12:15; received unresponsive (POINT 14). (6) Monitoring gaps and values vs "stable throughout" (POINT 3). (7) WS's medical assertions unsupported; evidence closed (POINT 4). (8) Death and cause, including CAD (POINT 7, 9). (9) Relief: claimed apart from supported.

Likely bench questions (sachche jawab):

- Post-mortem hua? — Nahi; C-11 par "NOT WILLING FOR AUTOPSY" (EV-044). [Kyun — Q-04: ____ to be filled]
- Maut kahan aur kab hui? — OP ke record ke mutabiq P 12:15 AM par CCU mein unresponsive laaye gaye aur 12:51 AM par mrit ghoshit hue (EV-077); us se pehle woh trolley par the (WS 10–11).
- Blood parivar ne kyun nahi uthaya? — [____ to be filled: Q-26, Q-27]; OP ke doctor ne kaha tha urgency nahi (WS 15).
- Koi expert opinion hai? — Record par nahi (W-03); OP ka bhi koi expert affidavit nahi (evidence band).
- Complaint mein 1:53 AM kyun likha? — Certificate ki copy ki padhai ki galti; record 12:51 AM kehta hai (W-23).
- Written Arguments kab file kiye? — [____ to be filled] (DL-05).

Pehle dikhane wale teen exhibits: OP Ann-3 (D-08 p.15), OP Ann-5 (D-08 p.17), OP Ann-7 (D-08 pp.19–20). Saath mein C-8 aur C-10.

Cross-examination: LAGU NAHI — OP ka evidence band hai aur Consumer Commission matters zyada-tar affidavit/documents par chalte hain; advocate confirm karein.

13. AGLE DOCUMENT KE LIYE MATERIAL

Stage S6 (Arguments) — agla document: Written Arguments (material_for_written_arguments). **Material: PART A dekho** (issues bench ke kram mein; har issue ke POINTs; tathya aur documents; OP ke WS ke shabd aur hamara jawab (A6, A6a); relief ledger link ke saath; chronology). Rejoinder band hone ke kaaran WS ka para-wise jawab yahi section 6 aur A6a se lein.

Red flags: jo tathya ya document evidence mein nahi hai use na likhein; NOT VERIFIED authority ko settled law na kahein; complaint ki galtiyan (1:53 AM, "Complainant No.2", "no monitoring after 8:30") na dohraayein; OP ko jhootha na kahein (step 5 nahi).

14. AAPKE DRAFT KI JAANCH

LAGU NAHI — Round 2 mein user ka koi draft nahi diya gaya. Written Arguments v4.1 aur v7.0 ki jaanch (draft_review) Round 5 aur Round 6 mein.

15. AUDIT TRAIL TABLE

Category	Entries (IDs; poora text Part C mein)
Verified facts (satyapit tathya)	SROT-SAMARTHIT TATHYA, mool page padha: orders EV-001–EV-005; GMSH-16 records EV-025, EV-026, EV-030–EV-033; OP ke records EV-036–EV-038, EV-041–EV-044, EV-074, EV-078–EV-080, EV-082, EV-083; OP ke WS ke shabd EV-046–EV-071 (sirf yeh ki WS mein kya likha hai); complaint/memo/prayer ke shabd EV-008–EV-023. Kisi document ki asliyat (T4) independently VERIFIED nahi.
Unverified claims (satyapan baaki)	ALLEGED/CLIENT-REPORTED: 3rd floor staircase ke paas stretcher, ward doctor ki salah, chest pain par jaanch se inkaar, raat ~9 PM blood bank band (EV-016–EV-018); dukaan ki aay (EV-022); Rs. 2,50,000 kharcha (EV-008, EV-023). OP ke Unsupported Assertions: guidelines, protocols, committee comments, counselling, intubation finding, "no attendant came" ka kaaran (EV-046, EV-049, EV-052, EV-053, EV-056, EV-063, EV-064, EV-079). AS RECORDED (entry hai, sach/samay saabit nahi): EV-027–EV-029, EV-034, EV-035, EV-039, EV-040, EV-072, EV-073, EV-075–EV-077, EV-081, EV-084. INFERRED: shifting ~17:10 (EV-050 + code).
Missing documents (jo nahi mile / nahi padhe)	OP ke Written Arguments 14.10.2026 (G-02); service report (G-03); committee comments (G-17); ward/trolley chart (G-18); shifting note (G-19); resuscitation record (G-20); protocols (G-21); counselling notes (G-22); echo/radiology reports (G-23); dated ECGs (G-24); CCTV record (G-06); expert opinion (G-07); evidence photos (G-08); aay, bills, legal heir, umar ke documents (G-09–G-12); court page 59, Labogram pages 2 & 4 (G-13). ZIP_3–ZIP_6 NOT READ (later rounds).
Contradictions and UNRESOLVED points	RESOLVED: X-02 (maut 12:51 AM), X-06, X-08, X-13 (jagah), X-15 (8:30 AM). OPEN: X-01, X-03, X-04, X-05, X-07, X-09, X-10, X-11, X-12, X-14, X-16, X-17, X-18 – X-30 (Part C, C10). Mukhya: X-21 ("no attendant came" vs parivar), X-24 (trolley par nigrani), X-25 ("stable throughout"), X-27 ("merely suspected" vs Ann-3), X-28 ("no indication" vs "correct anaemia").
Legal authorities needing verification	CIT-01 se CIT-23 — sab NOT VERIFIED (E-01): CPA 2019 ss.2(7), 2(11), 2(42), 34, 35, 38, 69; 2021 Jurisdiction Rules; V.P. Shantha; Jacob Mathew; Kusum Sharma; Maharaja Agrasen; Sarla Verma; Charan Singh (OP ne teeno ko "settled law" maana); res ipsa; Balram Prasad; BSA 2023 s.63; evidence/rejoinder band hone ka asar; General Clauses Act; SC ka 45-din rukh; No Reply ka asar; transfusion guideline; bed ki kami aur deficiency; bina affidavit WS ka wazan.
Questions for counsel (vakil se sawal)	Q-01 (WA filing), Q-17–Q-25 (Part C, Sawal register) + Round 2 ke naye (Q-31–Q-33): kya Commission se G-17–G-24 records maange jaayein; OP ke "no attendant came" (Ann-7) ka jawab kaise rakhein; OP ke exemplary costs ke prayer ka jawab.

Completeness Check

- Banner "KUL POINTS KI SANKHYA: 14" = POINT headings ki ginti 14 (POINT 1 se POINT 14, koi gap/repeat nahi) — Part C ke Compact Integrity Result mein script se jaancha.
- Har POINT mein ELEMENT TAG aur paanchon hisse is kram mein: 1 HAMARI BAAT / AAROP, 2 VIRODHI KA JAWAB, 3 HAMARA JAWAB, 4 KANOONI AADHAR AUR SABOOT, 5 VAKIL KE LIYE ANTIM DALIL.
- Har "VIRODHI KA JAWAB" par ek hi source tag ([WS-MOOL-TEXT] 13 POINTs mein; [JAWAB UPLABDH NAHI — MOOL WS SE SATYAPAN AVASHYAK] POINT 13 mein) aur WS Location (D-08 ke padhe gaye page/para).
- Har NOT VERIFIED authority Citation Register aur is Audit Trail Table mein.
- Planned sections 1–15 sab maujood; jo lagu nahi woh "LAGU NAHI — kaaran" ke saath.

Output note

Format: HTML source se Chromium (Playwright) dwara PDF. POINT headings asli yellow highlight (<mark>, background #FFEB3B) ke saath, printBackground on — PDF render karke dekha gaya. User ke prompt aur skill v18.0 ke beech jo badla: (1) Part A court-ready Written Arguments nahi, balki "Written Arguments ke liye Advocate Material" (skill rule 11); (2) cover status line skill ki ladder se ("WORKING FILE — CHECKS NOT RUN"), "DRAFT FOR COUNSEL REVIEW" nahi; (3) Part A mein bhi EV-ids aur status rakhe gaye (traceability niyam); (4) cause title, prayer clause, verification aur signature block nahi likhe gaye; (5) 22+ ki jagah 14 POINTs (sirf record se tikne wale). Round 2 ke badlaav Change Register (Part C, C15) mein.

PART C — REGISTERS (court mein nahi jayenge)

C1. Reading Log (cumulative)

D-id	File	Pages read / total	Method	Status per range	Low-confidence / suspected missing
D-01	NOOR_AALM.pdf (complaint bundle; scan, no text layer)	62 / 62	Visual reading of page images (110 dpi; 220–600 dpi zoom on critical fields); no OCR	PDF pp.1–62 READ in 7 batches (Round 1)	Handwritten words on C-1, C-2, C-4, C-6; C-7 p.2 labels; ECG machine times; C-8 p.54 diet block; C-9 row-1 resp/temp; C-11 time digits (now settled by D-08: 12:51 AM). Missing: court page 59; Labogram "Page 2 of 5", "Page 4 of 5".
D-02–D-06	Orders 22.04, 29.05, 01.07, 07.08, 14.10.2026	7 / 7	Text layer + page image	READ (Round 1)	Words cut at right margin in D-05, D-06 (original PDFs)
D-07	eJagriti_Case_Status_22.10.2026.pdf	1 / 1	Text layer + page image	READ (Round 1)	—
D-08	reply.pdf (OP's WS + Annexures 1–12; phone scans, no text layer)	25 / 25	Visual reading; native-resolution (273–302 ppi) crops at 2–3× zoom for every handwritten chart	pp.1–12 WS READ; pp.13–25 Annexures READ (Round 2, 6 batches)	Ann-1 header; Ann-2 HTN/DM marks, echo words; Ann-3 MYO/D-dimer values, first word of plan; Ann-5 first-row date, 12 pm and 3 pm BP digits, 9 AM pulse (45/95), several words of the night note; Ann-8 administration columns; Ann-9 blurred; Ann-11/12 machine text. Ann-10 is the cover only of the inpatient file.
—	00_READ_FIRST_Round1.txt, 00_READ_FIRST_Round2.txt; user prompt	full	Text	READ (instructions, not evidence)	—
—	ZIP_3 … ZIP_6	0	Only the file listing was seen	NOT READ (Rounds 3–6)	—

Page maps: D-01 court page N = PDF page N+2 for idx pp.1–58 (PDF p.61 carries "60", p.62 carries "61"). D-08: PDF page = handwritten page number (WS printed pp.1–12; Annexures pp.13–25).

C2. Intake Register (four evidence results kept apart)

D-id	Type	Source / received	Lineage	Identity match	T1 read	T2 consistent	T3 corroborated	T4 authentic	Notes
D-01	Complaint bundle (pleading + annexures)	User, ZIP_1, 29.10.2026	L-01 (complaint), L-02 (GMSH-16), L-03 (private labs), L-04 (OP records)	MATCHED ON name, father's name "Nazir", Dhanas address, dates, CR No. …4709; C-9 now identified by the OP as its Emergency chart	PARTLY	INCONSISTENT (X-01, X-03–X-08, X-14, X-16, X-17)	Partly: L-04 (OP) and L-02 (GMSH) are different roots	NOT VERIFIED	"TRUE PHOTO COPY" advocate stamps
D-02–D-06	Commission orders	User, ZIP_1	L-05	Case no.	YES	CONSISTENT	D-07 agrees on dates	NOT VERIFIED (not certified copies)	Margin cuts in D-05, D-06
D-07	e-Jagriti status print	User, ZIP_1	L-05 (derived)	Case no.	YES	CONSISTENT	Same root as orders	NOT VERIFIED	TIER-B
D-08	OP's Written Statement + OP Annexures 1–12	User, ZIP_2, 29.10.2026	L-06 (OP's pleading); annexures = L-04 (OP's records; Ann-9 = same document as C-11; Ann-7 letter = post-complaint, derived for the defence)	MATCHED ON name, CR …4709, father's name, age 60 (Ann-2, 5, 6, 8, 10); MATCH-UNCERTAIN on their face: Ann-1 (no name/date), Ann-11, Ann-12 (no legible name/date)	PARTLY (handwritten charts)	INCONSISTENT with itself in places (X-22–X-28)	Partly: OP records corroborate our C-8/C-10 (same root L-04, so not independent); Ann-10 time = WS time	NOT VERIFIED	"DUPLICATE COPY" stamp on Ann-6; Ann-7 p.2 is a photo of a register page dated "30/05/2026, 11:45" (other patients' data — not reproduced)

C3. Case State and State Reconciliation

CASE STATE | stage: S6 Arguments (Round 2 of 6 done) | forum status: ESTABLISHED (PR-01) | mode: UPDATE + AUDIT-OPP | last completed: Round 2 (VR-03) | open questions: Q-01–Q-07, Q-26–Q-30 for the family; Q-08–Q-16 next; Q-17–Q-25, Q-31–Q-33 advocate | next action: ROUND 3 (ZIP_3, client's early papers) | urgent deadlines: DL-05 POSSIBLY EXPIRED, DL-08 DANGER, DL-06 22.12.2026 | state check: OK.

STATE RECONCILIATION (start of Round 2): AVAILABLE — Round 1 state visible (notes_round1.md, ROUND_LOG.md, VR-01 source and PDF); D-01 to D-07 on disk. NEW — D-08. CHANGED — none in the user's message (the user asked for the question list on GitHub and Round 2). CONFLICTS — none. DECISION — proceed.

C4. Forum and Proceeding Register + Maintainability

PR-id	Forum / proceeding	Parties	Stage / next date	Source	Forum status
PR-01	District Consumer Disputes Redressal Commission, Chandigarh — CC No. DC/44/CC/423/2026 (Consumer Protection Act, 2019)	C1–C6 v. PGIMER	S6 Arguments; 22.12.2026 oral arguments	EV-001–EV-006; WS EV-046	ESTABLISHED
PR-02	Possible High Court writ (a "HC Writ draft (March 2026)" is in the user's round plan for ZIP_3)	UNKNOWN	UNKNOWN	User's prompt only (not read)	UNKNOWN
PR-03	Possible police/criminal complaint	UNKNOWN	UNKNOWN	User's prompt only	UNKNOWN
PR-04	"Medical Grievances Committee" (PGIMER internal) — the WS says the Department of Cardiology submitted para-wise comments "in CC No. 423 of 2026" (PO-II)	Department of Cardiology (OP)	UNKNOWN	WS p.1, PO-II (EV-046)	UNKNOWN (internal; not a forum for this case)

Cross-effect flag: the committee's comments (PR-04) may be relied on by the OP at arguments; ask for a copy (G-17). If PR-02 or PR-03 exists, statements there may be used here (and the reverse).

Check	What the record shows (source)	Provision and version	Status	Result	What would settle it
Forum	Complaint registered and admitted by DCDRC Chandigarh (EV-001); OP contests only "a provable deficiency" (WS para 2, EV-051)	CPA 2019 (CIT-01 to CIT-08)	NOT VERIFIED (law); forum ESTABLISHED (fact)	MAINTAINABLE ON THE RECORD	—
Territorial	OP at Sector 12, Chandigarh; C1–C3 in Dhanas; admitted by the OP (WS para 23, EV-070)	s.34 (CIT-03)	NOT VERIFIED	MAINTAINABLE ON THE RECORD (admitted)	Official text of s.34(2)
Pecuniary	Consideration documented Rs. 2,150 (EV-083, EV-043); stated Rs. 2,50,000 (EV-008); claim Rs. 63,50,000 principal (EV-023); no objection in WS	s.34(1) (CIT-03); 2021 Rules (CIT-04)	NOT VERIFIED	MAINTAINABLE ON THE RECORD if the measure is consideration paid; not objected	Official text of s.34(1) and the Rules as in force on 07.04.2026
Limitation	Cause of action 13.03.2026; filed 07.04.2026 (25 days); admitted by the OP (WS para 22, EV-070)	s.69 (CIT-08)	NOT VERIFIED	MAINTAINABLE ON THE RECORD (admitted)	Official text of s.69
Consumer / service	Rs. 2,000 advance (EV-083) and Rs. 150 POS (EV-043) paid to the OP; WS raises no objection	ss.2(7), 2(42) (CIT-01, CIT-02); V.P. Shantha (CIT-09)	NOT VERIFIED	MAINTAINABLE ON THE RECORD (not objected)	Official texts; effect of no objection (CIT-20)
Proper parties	C1–C6 as legal heirs (EV-010); OP does not dispute standing	s.2(7)/s.35 (CIT-01, CIT-05)	NOT VERIFIED	CANNOT TELL (relationship of C4–C6 not stated)	Q-05; legal heir certificate (G-11)
Fresh-law check	Not looked up in this session: official sources denied by the environment's network policy (E-01).				

C5. Deadline Register (PR-01)

DL	Type	Trigger and source	Provision / rule	Period	Arithmetic (code; both rules)	Date	Risk	Verification
DL-01	REPLY (OP's WS)	Receipt of notice (date UNKNOWN; notice for 29.05.2026, EV-001; OP's counsel present 29.05.2026, EV-002); WS dated 26.06.2026 (EV-071); filing recorded 01.07.2026 (EV-003)	Order of 22.04.2026: "30 days ... extendable for 15 days (total 45 days)"; s.38 (CIT-06)	30 + 15 days	WS late only if receipt ≤ 16.05.2026 (filing basis, exclusion) / ≤ 17.05.2026 (inclusion); on the WS-date basis ≤ 11.05.2026 / ≤ 12.05.2026. If receipt = 29.05.2026: 45th day 13.07.2026.	CONDITIONAL	WATCH	NOT VERIFIED; advocate check
DL-02	EVIDENCE (OP)	Orders 29.05.2026, 01.07.2026 (EV-002, EV-003)	Orders	—	Closed by order of 07.08.2026 (EV-004)	07.08.2026	CLOSED	Order read (TIER-A)
DL-03	EVIDENCE-AND-ARGUMENTS (our rejoinder)	Order 07.08.2026 (EV-004)	Order: "within 7 days"	7 days	14.08.2026 (Fri; exclusion) / 13.08.2026 (Thu; inclusion)	14.08.2026	EXPIRED (closed 14.10.2026, EV-005)	Order read
DL-04	EVIDENCE-AND-ARGUMENTS (WA, both)	Order 07.08.2026 (EV-004)	Order: "within 2 weeks"	14 days	21.08.2026 (Fri) / 20.08.2026 (Thu); OP filed by 14.10.2026; complainants did not (EV-005)	21.08.2026	EXPIRED (superseded by DL-05)	Order read

DL	Type	Trigger and source	Provision / rule	Period	Arithmetic (code; both rules)	Date	Risk	Verification
DL-05	EVIDENCE-AND-ARGUMENTS (our WA)	Order 14.10.2026 (EV-005)	Order: "within 3 days in registry"; computation rule (CIT-19)	3 days	17.10.2026 (Sat; exclusion) / 16.10.2026 (Fri; inclusion); next-working-day rule NOT VERIFIED; today 29.10.2026	16/17.10.2026	**POSSIBLY EXPIRED** (filing not in record)	NOT VERIFIED; advocate check (Q-01)
DL-06	HEARING	Order 14.10.2026 (EV-005); e-Jagriti (EV-006)	—	—	54 days from 29.10.2026 (Tue)	22.12.2026	WATCH	Order read
DL-07	FILING (limitation)	Cause of action 13.03.2026 (EV-022; admitted EV-070)	s.69 (CIT-08)	2 years (to verify)	Filed 07.04.2026 = 25 days later; 2 years ends 13.03.2028 / 12.03.2028	—	SAFE (admitted)	NOT VERIFIED
DL-08	PRESERVATION (CCTV, EMR logs, CCU/ward records, blood bank audit log)	Events 12–13.03.2026; prayer (v) (EV-023); WS silent	Practical (not statutory)	—	No preservation order in the orders read; 7+ months have passed	IMMEDIATE	DANGER / POSSIBLY EXPIRED	—
DL-09	APPEAL	No final order yet	s.41 (not checked)	—	LAGU NAHI — no final order	—	—	—

C6. Master Timeline (with clock notes; 11–13 March hour by hour)

F-id	Date / time	Event	Tag	Source	Clock note	Flag
F-001	04.02.2026 11:37 → 15.02.2026 01:08	GMSH-16 admission; referral to PGI Pulmonology	DOCUMENTED	EV-025	HIS print	Q-10
F-002	15.02.2026 05:06 → 17.02.2026 11:35	GMSH-16 re-admission; discharge home	DOCUMENTED	EV-026	HIS print	X-06
F-003	20.02.2026 11:08	Labogram CBC/LFT/RFT (Hb 10.0)	AS RECORDED	EV-027, EV-028	private lab	pp.2, 4 missing
F-004	24.02.2026 16:46 / 18:47	NT-proBNP 1117.48 pg/mL (WeCare, private)	AS RECORDED	EV-029	private lab clock	X-08, X-29
F-005	24.02.2026 12:46 → 05.03.2026 13:39	GMSH-16 admission; "Cured"	DOCUMENTED	EV-030	HIS print	X-07
F-007	10.03.2026 12:52 → 11.03.2026 00:14	GMSH-16 admission; referral to "PGI Emergency" for ECG (Wellens)	DOCUMENTED	EV-032	HIS print	
F-008	10.03.2026 13:07	Hb 8.2 g/dL	DOCUMENTED	EV-033	analyser clock	
F-009	10.03.2026 13:1?:12 and 20:0?:34	Two GMSH-16 ECG tracings	AS RECORDED	EV-035	ECG machine clocks; LOW	W-19
F-026a	10.03.2026 ~23:00 (as recorded)	Onset of "retrosternal chest pain" ("11pm on 10/3/26")	AS RECORDED	EV-073	history, not a clock	
F-010	11.03.2026 02:04	PGIMER EMOPD registration, "SR CITIZEN (PRIORITY)"	DOCUMENTED	EV-036	registration print	
F-011	11.03.2026 (time not written)	EMOPD: BP 130/100, PR 116, SpO2 94% RA; "Imp - Wellens Syndrome"; plan shift to CCU	DOCUMENTED	EV-037, EV-038	no time	
F-024	11.03.2026 03:21	Chest X-ray acquired	AS RECORDED	EV-075	X-ray system time	no report
F-014a	11.03.2026 05:00	Emergency chart: BP 110/70, pulse 80	AS RECORDED	EV-040	nursing chart	
F-014b	11.03.2026 08:30	Emergency chart: BP 120/88, pulse 80 (time per OP's WS para 7)	AS RECORDED	EV-040, EV-054	nursing chart	X-15 RESOLVED
F-029a	11.03.2026 09:30	CCU chart: 113/73, pulse 128, resp 20, 37°C	AS RECORDED	EV-076	CCU nursing chart; date of row LOW	X-19
F-025	11.03.2026 10:10:36	CCU admission; advance Rs. 2,000	DOCUMENTED	EV-083	admission system	
F-029b	11.03.2026 10:30	CCU chart row with dashes only	AS RECORDED	EV-076	—	
F-026	11.03.2026 (time not written)	CCU proforma: ECG rate 123, T changes V2–V4 (new onset); echo tachycardia, normal LVEF	AS RECORDED	EV-073	no time	
F-027	11.03.2026 (time not written)	Diagnosis "CAD – USA (Wellen's pattern)", "anaemia"; plan "correct anaemia"	DOCUMENTED	EV-074	no time	X-27, X-28
F-028	11.03.2026 (requisition date)	PGIMER Hb 7.5 g/dL	DOCUMENTED	EV-078	no sample time printed	
F-030	11.03.2026	Drug chart (Brilinta, Ecosprin, Monit GTN, Clexane, Lasix, IVF, ATT)	AS RECORDED	EV-081	administration times LOW	X-22
F-029c	11.03.2026 12:00	CCU chart: 11?/79, 125, temp 100°F, "Tab PCM given"	AS RECORDED	EV-076	CCU chart	
F-029d	11.03.2026 15:00	CCU chart: 124/103 [LOW], 126, 24	AS RECORDED	EV-076	CCU chart	

F-id	Date / time	Event	Tag	Source	Clock note	Flag
F-029e	11.03.2026 18:00	CCU chart: 90/49, 106, 26	AS RECORDED	EV-076	CCU chart	X-25
F-029f	11.03.2026 21:00	CCU chart: 130/79, 110, 24	AS RECORDED	EV-076	CCU chart	
—	11.03.2026 21:00 → 12.03.2026 01:00	RECORD GAP: no vitals entry (4 h 00 min, code)	RECORD GAP	EV-076	computed	POINT 3
F-029g	12.03.2026 01:00	CCU chart: 110/74, 100, 20, 37°C	AS RECORDED	EV-076	CCU chart	
—	12.03.2026 01:00 → 09:00	RECORD GAP: no vitals entry (8 h 00 min, code)	RECORD GAP	EV-076	computed	POINT 3
F-029h	12.03.2026 09:00	CCU chart: 87/46, pulse "45" [LOW], 20, 99.2°F	AS RECORDED	EV-076	CCU chart; pulse LOW	X-25
F-031	12.03.2026 09:15	Blood requisition received at ACC blood storage	DOCUMENTED	EV-079, EV-080	register time	
F-032	12.03.2026 "around 12 p.m."	PRBC "crossmatched and kept ready for issue"	AS RECORDED	EV-079	letter of 01.06.2026	X-20
F-016a	12.03.2026 12:33 PM (print)	Cross Match Report printed (browser time)	AS RECORDED	EV-041	PC clock vs server clock	X-11
F-016	12.03.2026 12:55	Crossmatch; "Cardio CCU / Trolly 66"; transfusion fields blank (family copy)	DOCUMENTED	EV-041	blood bank system	POINT 2
F-029i	12.03.2026 13:00	CCU chart: 110/73, 82, 20, 37°C	AS RECORDED	EV-076	CCU chart	
F-029j	12.03.2026 15:00	CCU chart: 9?/60 [LOW], 114, 20, 37°C	AS RECORDED	EV-076	CCU chart	
F-017	12.03.2026 16:47:39	Receipt Rs. 150 POS	DOCUMENTED	EV-043	billing system	
F-033	12.03.2026 ~17:10	Shifted out of CCU (10:10 + "nearly 31 hours"); ward bed not available; trolley	INFERRED (time) / DOCUMENTED (shifting, admitted)	EV-050, EV-056, EV-057, EV-065	computed from WS	POINT 5
F-018a	12.03.2026 "evening"	Family told no CCU bed; stretcher on 3rd floor near staircase	ALLEGED	EV-016	no time	X-03
F-029k / F-034	12.03.2026 21:00	"brought to the CCU only once at around 9pm"; CCU chart: 90/60, 82, 20	DOCUMENTED (OP admission) / AS RECORDED	EV-059, EV-076	"around"	POINT 5, 6
F-018c	12.03.2026 ~21:00	ACC blood bank found closed; Nehru Hospital BB: fresh crossmatch needed	ALLEGED	EV-018	"approximately"	X-16, X-21
F-035	12.03.2026 (time not stated)	CCU duty doctor: "no immediate medical urgency for blood transfusion"	DOCUMENTED (OP admission)	EV-060	—	W-24
F-018d	12.03.2026 (later)	C1 went home; "mother" stayed	ALLEGED	EV-018	no time	X-01
—	12.03.2026 21:00 → 13.03.2026 00:15	RECORD GAP: no vitals or nursing entry in the pages read (3 h 15 min, code); P on the trolley per the OP	RECORD GAP	EV-076, EV-077	crosses midnight	POINT 14
F-019a	13.03.2026 00:10–00:15	Family found P unresponsive (complaint para 18)	ALLEGED	EV-019	family's estimate	X-02 RESOLVED
F-036	13.03.2026 00:15	"pt Received in CCU at 12:15 ... unresponsive & pulse was not palpable"; CPR	DOCUMENTED	EV-077, EV-062	nursing note	POINT 14
F-037	13.03.2026 00:51	Declared dead by on-duty SR (CPR ~36 min, code)	DOCUMENTED	EV-077, EV-062, EV-044, EV-082	nursing note; MCCD	
F-020	13.03.2026	MCCD; "NOT A MEDICOLEGAL CASE"; "NOT WILLING FOR AUTOPSY"	DOCUMENTED	EV-044, EV-082	—	W-04
F-021	07.04.2026	Complaint filed	DOCUMENTED	EV-006, EV-007, EV-024	—	
F-022	22.04.2026 → 14.10.2026	Orders (Part A, A3)	DOCUMENTED	EV-001–EV-005	—	
F-040	29.05.2026	Cardiology's letter no. Card./2026/1420 to Transfusion Medicine (referred to in Ann-7)	AS RECORDED	EV-079	—	same day as vakalatnama (EV-002)
F-039	01.06.2026	Transfusion Medicine letter (Ann-7)	DOCUMENTED	EV-079	—	post-complaint
F-038	26.06.2026; 01.07.2026	WS dated/verified; filing recorded	DOCUMENTED	EV-071, EV-003	—	DL-01
F-023	22.12.2026	Oral arguments (next date)	DOCUMENTED	EV-005, EV-006	—	DL-06

C7. Evidence Register

Columns: EV | Document and exact location | Exact content (short; quotes are verbatim) | Status | Source label | Tier | Reading | Supports | Limitation (what it does NOT prove). For D-08 rows the location cell is the WS Location (WS page and paragraph, read in full) or the OP annexure page. One item keeps one EV-id.

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-001	D-02 Order 22.04.2026, p.1	"Complaint is admitted for hearing. Notice ... for 29.5.2026 ... written statement within 30 days from the date of receipt of notice extendable for 15 days (total 45 days)."	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P11, P13	Does not show date of receipt of notice
EV-002	D-03 Order 29.05.2026, p.1	"Vakalatnama on behalf of OP has been filed. Let written version, alongwith evidence, on behalf of OP be filed within the prescribed period."	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P4, P13	Procedural only
EV-003	D-04 Order 01.07.2026, p.1	"Written version on behalf of OP filed. Copy be supplied. Evidence has not been filed. Be filed by the next date. To come up on 07.08.2026 for filing rejoinder."	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P4, P13	Registry filing date of WS not shown
EV-004	D-05 Order 07.08.2026, p.1	"... the opportunity to file eviden[ce] by OP No.1 is closed ..."; rejoinder "last chance ... within 7 days"; "Written arguments on behalf of both the parties be filed within 2 weeks"; oral arguments 14.10.2026	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P4	Legal effect NOT VERIFIED
EV-005	D-06 Order 14.10.2026, p.1	Rejoinder opportunity "closed"; "Written arguments on behalf of OP have been filed"; "Written arguments by the complainant not filed. Time sought. Allowed. Same be filed within 3 days in registry."; "22.12.2026 for oral arguments"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P4; DL-05	Does not show whether our WA were filed later
EV-006	D-07 e-Jagriti print 22.10.2026	Stage "ADMIT"; category "POOR EMERGENCY CARE"; filing 2026-04-07; hearing 2026-10-14; next 2026-12-22	DOCUMENTED	SROT-SAMARTHIT TATHYA	B	READ	P11	Portal summary, not the order
EV-007	D-01 Index, PDF pp.1–2	Index with court pages; dated 07.04.2026; C-11 at p.59, Vakalatnama p.60	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P11	Page 59 missing in PDF (G-13)
EV-008	D-01 Proforma, idx pp.1–2 / PDF pp.3–4	"Total Consideration Paid Rs. 2,50,000"; "Medical Negligence"; "Section 35(1)(a)"; "Date of Action 13.03.2026"	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P10, P11	No bill for Rs. 2,50,000
EV-009	D-01 Synopsis, idx pp.3–4 / PDF pp.5–6 (undated)	Summary of the case; "no vital parameters were recorded after 08:30 AM"; "manipulation in recording the cause and time of death"; Rs. 60,00,000	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	—	Pleading only
EV-010	D-01 Memo of Parties, idx pp.5–6 / PDF pp.7–8	C1 Noor Aalam s/o; C2 Muskan d/o; C3 Husan Bano wd/o; C4, C5, C6 "w/o" others; "All being the legal heirs"; one OP	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P9, P11	Relationship of C4–C6 not stated
EV-011	Complaint paras 1–2, idx p.7 / PDF p.9	Legal heirs; "availed medical services" → "consumer" u/s 2(7)	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P10	No payment detail
EV-012	Complaint paras 3–5, idx pp.7–9 / PDF pp.9–11	GMSH-16 history; "NT PRO-BNP ... 1117.48, which is more than ten times"; "Wellens Syndrome"; "HGB ... 8.2"; referral	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P1	X-06, X-07, X-08
EV-013	Complaint para 6, idx pp.9–10 / PDF pp.11–12	Examined by Internal Medicine doctor; "Wellens Syndrome, with BNP levels at 1117.48"; "advised to be shifted to the Coronary Care Unit"	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P1	Corroborated by C-8 (EV-037, EV-038)
EV-014	Complaint para 7, idx p.10 / PDF p.12	Vitals ~5:00 AM (110/70, 80) and 8:30 AM (120/88, 80); "no monitoring was conducted after 8:30 AM"	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P3	Date not stated in para
EV-015	Complaint paras 8–9, idx pp.10–11 / PDF pp.12–13	Crossmatch "12.03.2026 at 12:55 PM"; "six hours of issue"; "doctors neither administered the blood transfusion nor issued any written advisory"	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P2	X-12, X-16

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-016	Complaint paras 10–11, idx p.11 / PDF p.13	"no bed was available in the CCU"; shift to ward; "placed on a stretcher on the 3rd floor near the staircase"	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P5	Reconciled by OP's WS paras 10–11, 14, 18 and Ann-5: collapse outside the CCU, death declared in the CCU (X-13 RESOLVED); exact place ALLEGED
EV-017	Complaint paras 12–14, idx pp.11–12 / PDF pp.13–14	Chest pain; refusal to examine/re-admit; "solely based on pulse oximeter readings"; no BP/ECG	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P5, P6	Client-only
EV-018	Complaint paras 15–17, idx pp.12–13 / PDF pp.14–15	Transfusion refused; ACC Blood Bank "closed at approximately 9:00 PM"; Nehru Hospital BB fresh crossmatch; "no urgency"; C1 went home; mother "(Complainant No.2)" stayed	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P2	X-01, X-04
EV-019	Complaint para 18, idx p.14 / PDF p.16	Death "around 12:10–12:15 AM"; "the death certificate incorrectly records the time of death as 1:53 AM"; CPR attempted	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	X-02	Family's estimate; matches the time OP's chart records P as "Received in CCU at 12:15" unresponsive (EV-077); death declared 12:51 AM (EV-062); "1:53 AM" was a misreading (W-23)
EV-020	Complaint para 19, idx p.14 / PDF p.16	Cause of death "vehemently denied"; "Anorexia × 2 months" argument	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P7	Undercut by EV-032 (vomiting)
EV-021	Grounds A–J, idx pp.15–18 / PDF pp.17–20	A no ECG/BP after 08:30; B abandonment; C "never informed"; D "made available by 12:55 PM"; E "false assurance"; F "deliberately recorded ... to conceal", "(Annexure P-)"; G res ipsa loquitur; H–J judgments quoted	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P2–P9	Quotes of judgments NOT VERIFIED
EV-022	Complaint paras 21–24, idx p.19 / PDF p.21	"directly resulted in the death"; "Complainant No.2 has lost her husband"; "two daughters ... married"; "small clothing shop"; Rs. 60,00,000; cause of action 13.03.2026; C1–C3 in Dhanas; OP Sector 12; no similar complaint	ALLEGED	SROT-SAMARTHIT TATHYA (as pleaded)	A	READ	P8, P9, P11	X-01, X-05
EV-023	Prayer and verification, idx pp.20–21 / PDF pp.22–23	(i) Rs. 60,00,000 + 12% p.a.; (ii) Rs. 2,50,000; (iii) Rs. 1,00,000; (iv) duty doctors' details; (v) preserve records + CCTV, sealed cover; (vi) other; verification "paragraphs 1 to __"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P9, P12	Claim, not award
EV-024	Affidavit of C1, idx pp.22–35 / PDF pp.24–37	Repeats complaint paras (+1 numbering); notarised "07 APR 2026"; heading "COMPLAINT NO. ____ OF 2025"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P6	Hearsay for night events (W-06)
EV-025	C-1, idx p.36 / PDF p.38	GMSH-16 DS 04/02 11:37 – 15/02 01:08; age 55; "Ayushman Bharat"; "Chronic Smoker / Newly Dxd DM"; "PTB Sputum +ve on ATT since 7/2/26"; "Hypoalbuminemia / Hyponatremia"; "referred to PGI Pulmonology"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P8, P10	Some handwriting low-confidence
EV-026	C-2, idx p.37 / PDF p.39	GMSH-16 DS 15/02 05:06 – 17/02 11:35; age 55; "↓ appetite"; medicines	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P8	
EV-027	C-3, idx p.38 / PDF p.40	Labogram CBC 20-02-2026: "Haemoglobin 10.0 gm/dl"; age 60	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P8	"NOT FOR MEDICO LEGAL PURPOSE"
EV-028	C-3, idx pp.39–41 / PDF pp.41–43	Na 131.2 L; Cl 92.5 L; LFT (albumin 2.8 L, ALP 197.3 H); RFT (creatinine 1.7 H)	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P8	Pages 2 & 4 of 5 missing
EV-029	C-3, idx p.42 / PDF p.44	WeCare Lab, collected 24/02/2026 04:46 PM: "N-Terminal Pro-BNP ... H 1117.48 pg/mL", reference "0-114.5"	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1, P8	9.76× upper limit (computed), not "more than ten times"; OP calls it a GMSH-16 report (X-29); not comparable as such with BNP 37.4 (X-18)

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-030	C-4, idx p.43 / PDF p.45	GMSH-16 DS 24/02 12:46 – 05/03 13:39; age 60; "T2DM / PTB on ATT ... ATT induced hepatitis ... Rifampicin reintroduced on 5/3/26"; "Cured"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P8	No cardiac diagnosis written (X-07)
EV-031	C-5, idx pp.44–45 / PDF pp.46–47	USG 02.03.2026: "B/L moderate pleural effusion"; elastography 05.03.2026: 4.2 kPa	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P8	
EV-032	C-6, idx p.46 / PDF p.48	GMSH-16 DS 10/03 12:52 – 11/03 00:14; age 60; "General"; "Nausea vomiting x 4 days"; "Anorexia x 2 months"; "Trop T ... Negative"; "We are referring our patient to PGI Emergency regarding his recent ECG findings [CAD ?ACS?] (Wellens)"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P7, P8	Some handwriting low-confidence
EV-033	C-7, idx p.47 / PDF p.49	Analyser 2026/03/10 13:07:48: "HGB 8.2 [g/dL]"; "Anemia"; "B Positive"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2, P8	Value at GMSH-16, not at PGIMER
EV-034	C-7, idx p.48 / PDF p.50	Handwritten investigations: INR 1.3; Na 124; HGB 8.2; etc. (undated)	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P8	Author and date unknown
EV-035	C-7, idx pp.49–50 / PDF pp.51–52	Two ECG tracings dated 10-03-2026 (times low-confidence); black rectangle on idx p.50 copy	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1, P6	No AI interpretation; tracing to cardiologist
EV-036	C-8, idx p.51 / PDF p.53	PGIMER OPR: "SR CITIZEN (PRIORITY)"; "Date : 11-03-2026 02:04"; "Age/Sex : 60 Y/M"; "Category : GENERAL"; "Internal Medicine (E M OPD)"; history SOB, chest pain, fever, DM, PTB on ATT	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P10	Sticker print faint
EV-037	C-8, idx p.52 / PDF p.54	"BP - 130/100 mm Hg"; "PR - 116/min"; "SpO2 - 94% RA"; "Trop T Neg. (outside)"; "CBC - 8.2/13k/336k"; "BNP - 1117.48 (24/2)"; "ECG → Wellens"; "Imp - Wellens Syndrome"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P2, P7, P8	Time of examination not written
EV-038	C-8, idx p.53 / PDF p.55	"Plan ... Cardio call / Pulmo call"; "C/D/W Cardio on-call SR → Shift to CCU (ACC)"; signed "11/3/26" (JR, Internal Medicine)	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P8	Time of shift not shown
EV-039	C-8, idx p.54 / PDF p.56	Treatment sheet "Date 10/3/26", columns "11/3": Ecosprin Gold (75/75/20) OD; Sorbitrate 5 mg S/L stat; Inj LMWH 0.4 ml S/C; Inj Pantop 40 mg IV OD; "Moist O2 ... Venturi mask"; "I/O, vitals, RBS"; nurse initials	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1, P3	Administration times not visible; X-10
EV-040	C-9, idx p.55 / PDF p.57	B.P. chart: "11/3 | 5 Am | 110/70 | 80"; "11/3 | [8:30/5:30] am | 120/88 mmHg | 80 | 20 | A/F"; rest blank; name/CR/ward/bed blank	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P3, P6	Row-2 time 8:30 per OP's own WS; OP identifies C-9 as its Emergency chart (EV-054; X-15 RESOLVED); shows entries, not that monitoring stopped; identity blank
EV-041	C-10, idx p.56 / PDF p.58	"Requisition No. ACC-0073148"; "Crossmatch Date and Time 12/03/2026 12:55"; "Ward Cardio CCU"; "Bed No. Trolly 66"; bag "819538/B+/SPRBC/240 ml/18-Apr-26"; transfusion fields blank; "Transfuse within two hours of issue"; print "3/12/2026, 12:33 PM"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2, P5	Family copy; issue time not shown
EV-042	C-10, idx p.57 / PDF p.59	"Patient's Identification Slip"; "Ward/Bed No. Cardio C C U / Trolly 66"; "Requisition sent on: 12-Mar-2026"; "within six hours of issue and return them to the Blood Bank, if not transfused"; "Signature of JR/SR"; "Blood Requisition Details Modified successfully"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2, P5	Does not show issue or transfusion
EV-043	C-10, idx p.58 / PDF p.60	PGIMER Test Receipt "12/03/2026 16:47:39"; "CATEGORY : GENERAL"; "WARD : INTERNAL MEDICINE"; grouping and crossmatching PRBC Rs. 150; "POS/150 RUPEES"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2, P10, P11	Only payment proof in bundle
EV-044	C-11, PDF p.61 (handwritten "60"; index p.59)	"NOT A MEDICOLEGAL CASE"; "NOT WILLING FOR AUTOPSY" (with C1's signature); "died in the Hospital in ward No. CCU on 13-3-26 at [1[?]:51] AM"; "Address at the time of death: CCU PGIMER"; age 60; cause (a) Type I respiratory failure (b) aspiration pneumonia (c) coronary artery disease; II pulmonary tuberculosis; "Natural"; "expired on 13-3-26"	DOCUMENTED (time 12:51 AM per OP, EV-062, EV-077)	SROT-SAMARTHIT TATHYA	A	READ	P5, P7, P8, P9	Certificate, not autopsy; copy's digits "1[?]:51", settled at 12:51 AM by OP (X-02); "ward No. CCU" = where death was declared (X-13)

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-045	Vakalatnama, PDF p.62 (handwritten "61")	"NOOR ALAM & ORS ... Versus POSTGRADUATE INSTITUTE ... (PGIMER), CHANDIGARH & ANR"; complainants' names with numbers (Muskan-2, Noor Aalam-1, Husn Bano-3, Gulbahar-4, Soni-5, Jasmin-6); date blank	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P9; X-01	Date blank; "& ANR" (X-14)
EV-046	D-08 WS p.1 (title, PO-I, PO-II) — WS Location, read in full	"WRITTEN STATEMENT TO COMPLAINT ON BEHALF OF THE OPPOSITE PARTY"; PO-I "No deficiency in service ... guideline directed medical management at PGIMER from 11.03.2026 to 13.03.2026"; PO-II "deliberately suppressed and misrepresented material medical facts"; Department of Cardiology's "Para-wise Comments submitted to the Medical Grievances Committee (in CC No. 423 of 2026), conclusively demonstrated ..."	DOCUMENTED (as pleaded by OP)	SROT-SAMARTHIT TATHYA	A	READ	P4, P12	Committee comments not annexed
EV-047	D-08 WS p.2, PO-III	"the Complaint grossly exaggerates the severity of the patient's cardiac condition"; NT-proBNP "recorded at GMSH-16"; "repeat BNP was found to be only 37.4 pg/mL"; "2D Echocardiography ... normal left ventricular ejection fraction (LVEF)"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1	X-27, X-29; Ann-1 undated
EV-048	D-08 WS p.2, PO-IV	co-morbidities incl. "co-existing Coronary Artery Disease (CAD)"; death "attributable entirely to the cumulative and progressive deterioration of these underlying conditions"; "relatives were repeatedly informed of his very poor overall prognosis"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1, P7, P8	Counselling note not annexed
EV-049	D-08 WS p.2, PO-V	blood "only as a precautionary standby measure ... in the event of emergency bleeding such as haemoptysis. The blood was never released from the Blood Bank ... above 7 g/dL ... No medical indication for routine transfusion existed."	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P2	Guideline not annexed; X-28
EV-050	D-08 WS p.2, PO-VI	"admitted to the CCU at PGIMER at 10:10 AM on 11.03.2026 and was observed and monitored for nearly 31 hours"; "monitored constantly"; "ECG was repeated ... NO ST elevations or any dynamic changes"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P3, P5, P6	"31 hours" gives ~17:10 on 12.03 (computed)
EV-051	D-08 WS p.3, PO-VII and paras 1–2	Rs. 60,00,000 "emphatically denied"; para 1 "a matter of record"; negligence denied; para 2 jurisdiction resisted only because "a provable deficiency of service does not exist"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P9, P10	No consumer-status objection
EV-052	D-08 WS pp.3–4, para 3	"only '?Wellens Syndrome' ... merely suspected and not confirmed"; "8.2 g/dL ... did not meet the standard clinical threshold for blood transfusion (which is Hb < 7 g/dL per established guidelines)"; "appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1, P2	X-27; Hb 7.5 (Ann-6) not mentioned
EV-053	D-08 WS pp.4–5, para 6	examined in EMOPD incl. by Dr. A; "BNP levels at PGIMER stood at 1117.48 pg/mL is denied and is a deliberate misrepresentation"; BNP 37.4; echo normal LVEF; "As per the existing protocol at PGIMER, all cardiac patients are shifted to CCU"; "clinical falsehood"; Annexures 1–4	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1, P4	Protocol not annexed
EV-054	D-08 WS p.5, para 7	"Annexure C-9 ... is in fact the vital chart of the Emergency Department of PGIMER and not the CCU. In CCU, a new vital chart was made. The vital chart of the CCU as well as the Ward ... was duly maintained"; "no monitoring was conducted after 8:30 AM on 11.03.2026, is factually false"; Annexure 5	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P3	Ward chart not annexed
EV-055	D-08 WS pp.5–6, para 8	"admitted that a blood cross match procedure was completed by the Blood Bank on 12.03.2026"; "solely as a precautionary standby measure"; "the blood was never released or issued from the Blood Bank in the first instance"; six-hour window from issue; "management of the underlying condition, not empirical blood transfusion"; Annexures 6, 7	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P2	Express Admission of non-issue

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-056	D-08 WS p.6, paras 9–10	"constant monitoring ... in the CCU prior to being shifted. As per the routine protocol at PGIMER, all patients are clinically assessed prior to shifting from CCU to the ward this was duly done"; "Any delay in ward bed availability is a well known systemic constraint ... cannot constitute negligence per se"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P5	Shifting note not annexed
EV-057	D-08 WS pp.6–7, para 11	"It is admitted that there was a temporary delay in the availability of a ward bed due to PGIMER's extremely heavy patient load"; "denied that the patient was 'abandoned'"; "Even on the trolley, the patient continued to be under the monitoring and care ... as conclusively evidenced by the vital chart maintained by the nursing staff"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P5, P14	X-24 (no entry 21:00→00:15)
EV-058	D-08 WS p.7, para 12	ward doctor's advice to re-shift "a completely false allegation"; transfer protocol (ward SR and CCU SR); "The CCU Duty Doctor thoroughly examined the patient, pulse and oxygen saturation were checked ... no tachycardia or tachypnoea"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P5, P6	X-23; no ECG/BP mentioned
EV-059	D-08 WS p.7, paras 13–14	"to the best of our knowledge, the patient was brought to the CCU only once at around 9pm on 12.03.26"; "primary complaint ... mild breathlessness"; pulse, saturation checked; "auscultated"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P5, P6, P14	Whether re-admitted after 9 pm not stated
EV-060	D-08 WS p.8, paras 15–16	blood "not intended to be transfused unless the patient has any form of bleeding manifestation or a further decline in the hemoglobin levels, and hence was never released"; "The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P2	Own Hb 7.5 < 8.2 (Ann-6)
EV-061	D-08 WS p.8, para 17	"placed on guideline directed medical management for Acute Coronary Syndrome (ACS) ... dual antiplatelets, statins, nitrates, and anticoagulants"; "did not complain of chest pain"; Annexure 8	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1, P6	X-22 (statin)
EV-062	D-08 WS p.8, para 18	"The Medical Certificate of Cause of Death (MCCD form) clearly and correctly states the time of death as 12:51 AM. The nursing vital chart records that the patient was received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered"; Annexure 9	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P7, P14	Settles X-02
EV-063	D-08 WS pp.8–9, para 19	"'Type I Respiratory Failure, secondary to Aspiration' is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation, where particulate matter and fluids were visually identified in the trachea"; aspiration of liquids; "anorexia" = loss of appetite for solid food	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P7	No resuscitation record annexed; X-26
EV-064	D-08 WS p.9, para 20-a	CCU "10:10 AM ... nearly 31 hours"; "denied chest pain on multiple occasions"; ECG repeated; "Coronary angiography was deferred on sound clinical grounds ... this was communicated to the family"; Annexures 10–12	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P6, P12	No counselling note annexed
EV-065	D-08 WS pp.9–10, paras 20-b, 20-c	"shifted from CCU to the ward only after adequate monitoring and clinical assessment. The temporary non-availability of a bed in the ward ... does not constitute 'abandonment.' The patient continued to be monitored even on the trolley."; bed non-communication denied	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P5, P14	Assessment note not annexed
EV-066	D-08 WS p.10, paras 20-D, 20-E, 20-F	blood "precautionary standby ... never released"; "hemodynamically stable"; "The vital chart conclusively establishes hemodynamic stability throughout the admission"; "There was no manipulation of the death record."	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P2, P3, P7	X-25

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-067	D-08 WS p.10, para 20-G	"The legal ingredients of medical negligence duty of care, breach of such duty, and resulting damage are admitted as a general legal proposition"; "The Opposite Party owed a duty of care to the deceased and discharged that duty at every stage"; res ipsa "has no application"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P1	Duty admitted; breach denied
EV-068	D-08 WS pp.10–11, paras 20-H to 20-J	Maharaja Agrasen, Sarla Verma, Charan Singh "are admitted as settled law governing compensation in medical negligence matters" but "have no application"	DOCUMENTED (as pleaded)	LEGAL PROPOSITION	A	READ	P9	Not a verification of the texts
EV-069	D-08 WS p.11, para 21	"The sympathy and loss felt by the Complainants are acknowledged"; claim "wholly excessive, speculative, and unsupported by any documentary evidence of income or financial dependency"; "close monitoring in CCU for approximately 31 hours"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P9	Not an admission of liability
EV-070	D-08 WS p.11, paras 22–24	"The contents of Para 22 with regard to the date of the cause of action (13.03.2026) and the period of limitation are admitted."; "The contents of Para 23 regarding territorial jurisdiction are admitted."; para 24 "admitted to the knowledge of the Opposite Party"	DOCUMENTED (as pleaded)	SROT-SAMARTHIT TATHYA	A	READ	P11	—
EV-071	D-08 WS pp.11–12, prayer and verification	prayer: dismissal; "Award exemplary costs against the Complainants"; "Dated: 26.06.2026"; verified "on the 26th day of June, 2026" by the "Authorised Representative, PGIMER" (Professor, Cardiology); signed by OP's advocate	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P4, P9, P13	WS date ≠ filing date (EV-003)
EV-072	D-08 p.13, OP Ann-1	Point-of-care cardiac panel print: "CKMB <1.0 ng/mL", "MYO 176 ng/mL" (flagged), "TNI <0.05 ng/mL", "BNP 37.4 pg/mL", "DDIM 3280 ng/mL" (flagged), "PAT. RESULT ABNORMAL"; ID "5555555555", "PAT.ID 4"; no name, no date	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1	MATCH-UNCERTAIN on its face; values repeated in Ann-3
EV-073	D-08 p.14, OP Ann-2	CCU admission proforma: "DOA TO CCU: 11/3/26"; "retrosternal chest pain × 1 day (11pm on 10/3/26)"; "no syncope / palpitations"; vitals fields (BP, SpO2, PR) blank; "ECG: Rate: 123 bpm", "T ... V2 – V4 (new onset)"; 2D echo "Tachycardia", "EA fused", "(N) LVEF"	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1, P6	Several words LOW-CONFIDENCE; no formal echo report
EV-074	D-08 p.15, OP Ann-3	"SOB PANEL: TNI <0.05, BNP 37.4, CKMB <1.0 ..."; diagnosis "PTB on ATT", "CAD – USA", "(Wellen's pattern)", boxed "anaemia"; plan "correct anaemia", "CXR"; signed SR, Cardiology	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P2, P8	"correct anaemia" MODERATE confidence; meaning (transfusion or other) for the expert
EV-075	D-08 p.16, OP Ann-4	Chest X-ray image: "ATC, PGIMER CHANDIGARH / MANOHAR / [..4709] / 11-03-2026 / Acq Tm: 03:21 AM"	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1	Image only; no report; no AI reading
EV-076	D-08 p.17, OP Ann-5 (rows)	CCU "NURSING SERVICE" vital chart, name Manohar, CR [..4709]: 11/3 9:30 AM 113/73, 128; 10:30 dashes; 12 pm 11?/79, 125, 100°F; 3 [pm] 124/103 [LOW], 126; 6 pm 90/49, 106; 9 pm 130/79, 110; 12/3 1 am 110/74, 100; 9 AM 87/46, "45" [LOW]; 1 pm 110/73, 82; 3 p[m] 9?/60, 114; 9 pm 90/60, 82	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P3, P5, P6, P14	Entries, not continuous monitoring; some digits LOW
EV-077	D-08 p.17, OP Ann-5 (night note)	"pt Received in CCU at 12:15 ... Pt was unresponsive & pulse was not palpable. CPR started as per ACLS protocol ... Atropine 0.6 mg & [Adrenaline?] 1 mg IV given. At 12:51 ... Pt declared dead by on duty SR."	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P7, P14	Key words MODERATE confidence; does not say where or when the arrest began
EV-078	D-08 p.18, OP Ann-6	PGIMER Haematology (Emergency Lab) "HAEMOGRAM REPORT", "DUPLICATE COPY": Manohar, 60 Yrs/M, Internal Medicine-E M OPD, "Requisition date 11/03/2026"; "Haemoglobin, Hb (gm/dL) 7.5" (13.5–17.5); PCV 22.9; MCV 74.1; RDW 21.4; platelets 264; TLC 7.5	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2, P8	Sample/report time not printed

EV	Document / location	Exact content	Status	Label	Tier	Read	Supports	Limitation
EV-079	D-08 p.19, OP Ann-7 (letter)	Transfusion Medicine, "Dated 01/06/2026", reply to Cardiology's "letter no. Card./2026/1420 dated 29.05.2026": requisition "received in the Blood Storage Centre, ACC on 12th March, 2026 at 9.15 a.m. vide DTM No. ACC-0073148. Subsequently one unit of PRBC was crossmatched and kept ready for issue around 12 p.m. However, no attendant came to collect the same for the patient."	DOCUMENTED (letter) / AS RECORDED (its account)	SROT-SAMARTHIT TATHYA	A	READ	P2	Post-complaint letter, not a contemporaneous record; X-20, X-21
EV-080	D-08 p.20, OP Ann-7 (register)	"BLOOD REQUISITION, RECEIPT AND ISSUE REGISTER", "Page No. 008", "12/3/2026 MORNING": row "0073148 | 9:15 am | Manohar | [..4709] | ACC | B+" with unit, issue date/time and issuing/receiving signature columns blank; photo stamp "30/05/2026, 11:45"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P2	Other patients' rows not reproduced (privacy)
EV-081	D-08 p.21, OP Ann-8	"Drug and Treatment" chart "Date 11/3/26", columns 11/03, 12/03, 13/3: Brilinta 180 mg stat f/b 90 mg BD; Ecosprin 75 mg OD; Pantop 40; Rifampicin 600; Ethambutol 800; Levofloxacin 750; Pyridoxine 20; Monit GTN 2.6 mg BD; "I/O charts", "RBS", "vitals q[?]"; Clexane 0.6 mL S/C BD; insulin as per scale; Lasix 40 mg stat f/b 20 mg IV BD; IVF 30 ml/hr; no blood/transfusion order	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P1, P2, P3	Administration marks LOW; 13/3 column entries ambiguous
EV-082	D-08 p.22, OP Ann-9	OP's copy of the MCCD (same document as C-11): cause (a) Type I respiratory failure (b) aspiration pneumonia (c) coronary artery disease; II PTB; "Not willing for autopsy"; "Address at the time of death: CCU, PGIMER"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P7, P9	Same lineage as EV-044 (not independent)
EV-083	D-08 p.23, OP Ann-10	"INPATIENT RECORD" cover sticker: Name Manohar; 60 Y/M; Father/Spouse Nazir; Department Cardiology; "Ward: Cardio C C U"; "Adm Date/Time: 11-Mar-2026 10:10:36"; "Adv. Rs. 2000.00 Receipt No.: [..]/1 Date: 11-Mar-2026"	DOCUMENTED	SROT-SAMARTHIT TATHYA	A	READ	P1, P5, P10, P11	Cover only; file contents not annexed
EV-084	D-08 pp.24–25, OP Ann-11, Ann-12	Two 12-lead ECG photocopies; Ann-11 machine text "Vent rate 1?? BPM ..." [LOW]; Ann-12 "Site # 0 Cart # 0"; no legible date, time or patient name	AS RECORDED	SROT-SAMARTHIT TATHYA	A	READ	P6	MATCH-UNCERTAIN on their face; no AI reading

C8. Critical Value Ledger

Value	Where used	Source	First reading	Re-read (zoom)	Alternatives	Status
NT-proBNP	P1, P8	EV-029 (C-3 idx p.42)	1117.48 pg/mL, ref 0–114.5	same; C-8 "1117.48 (24/2)"	—	SOURCE-VERIFIED; CROSS-CHECKED (EV-037)
Point-of-care BNP	P1	EV-072 (OP Ann-1); EV-074 (Ann-3)	37.4 pg/mL	same in Ann-3	—	SOURCE-VERIFIED (value); identity/date of Ann-1 NOT shown
Hb at GMSH-16	P2, P8	EV-033 (C-7 idx p.47)	8.2 g/dL, 10.03.2026 13:07	same; C-8 "CBC - 8.2"	—	SOURCE-VERIFIED; CROSS-CHECKED
Hb at PGIMER	P2, P8	EV-078 (OP Ann-6)	7.5 g/dL (requisition 11/03/2026)	7.5 (native scan, 2× zoom)	—	SOURCE-VERIFIED
CCU admission time	P1, P5	EV-083 (OP Ann-10); EV-050 (WS PO-VI)	11-Mar-2026 10:10:36	same; WS "10:10 AM"	CCU chart first row 9:30 AM (X-19)	SOURCE-VERIFIED; CROSS-CHECKED (WS)
Advance paid	P10	EV-083 (OP Ann-10)	Rs. 2000.00, 11-Mar-2026	same (1.6× zoom)	—	SOURCE-VERIFIED
Requisition receipt time	P2	EV-079, EV-080 (OP Ann-7)	12.03.2026 9:15 a.m.	letter (typed) and register (handwritten "9:15 am")	—	SOURCE-VERIFIED; INTERNALLY CONSISTENT
Crossmatch time	P2	EV-041 (C-10 idx p.56); EV-079	12/03/2026 12:55	same; Ann-7 "around 12 p.m."	X-20	SOURCE-VERIFIED
Receipt time / amount	P2, P10	EV-043 (C-10 idx p.58)	12/03/2026 16:47:39; Rs. 150	same	—	SOURCE-VERIFIED
Emergency BP chart row-2 time	P3	EV-040 (C-9); EV-054 (WS p.5, para 7)	8:30 or 5:30 (VR-01)	OP's WS: "8:30 AM on 11.03.2026"	—	RESOLVED: 8:30 (TIER-A, OP)
CCU chart: 6 pm 11/3 BP	P3	EV-076 (OP Ann-5)	90/49	90/49 (3× zoom)	—	SOURCE-VERIFIED (MOD-HIGH)

Value	Where used	Source	First reading	Re-read (zoom)	Alternatives	Status
CCU chart: 9 AM 12/3 BP and pulse	P3	EV-076 (OP Ann-5)	87/46; pulse 45	87/46 (MOD); pulse "45 bpm" (MOD-LOW)	pulse 95	LOW-CONFIDENCE (pulse) — original needed
CCU chart: last entry before arrest	P14	EV-076 (OP Ann-5)	12/3 9 pm: 90/60, 82, 20	same (3× zoom)	—	SOURCE-VERIFIED
Time received in CCU (13/3)	P14	EV-077 (Ann-5 note); EV-062 (WS para 18)	12:15	"Received in CCU at 12:15" (MOD); WS "approximately 12:15 AM"	—	CROSS-CHECKED (note + WS; same root)
Time of death	P7, P14	EV-044/EV-082 (MCCD); EV-077 (Ann-5); EV-062 (WS)	1:53 AM (complaint); "1?:51" (VR-01)	WS "12:51 AM"; Ann-5 "At 12:51"; MCCD "1?:51" consistent with 12:51	—	RESOLVED: 12:51 AM (TIER-A, OP)
Age of P	P9; X-09	EV-025, EV-026 (55); C-3–C-6, C-8, C-10, C-11 and OP Ann-6, Ann-8, Ann-10 (60; Ann-9 = C-11)	55 / 60	same	—	CONFLICTED (60 in 10 documents, 55 in 2)
Claim amounts	P9	EV-023	60,00,000; 2,50,000; 1,00,000; 12%	same; total 63,50,000 (code)	—	SOURCE-VERIFIED (as pleaded)
EMOPD vitals	P1	EV-037	130/100; 116/min; SpO2 94% RA	same	—	SOURCE-VERIFIED
CCU ECG rate	P1, P6	EV-073 (OP Ann-2)	"Rate: 123 bpm"	same (2× zoom)	—	SOURCE-VERIFIED (MOD-HIGH)

C9. OP Admissions Table (WS ke apne shabd) aur OP ke records ki entries

WS Location	OP ke shabd (verbatim)	EV	Kaise kaam aayegi
WS p.10, para 20-G	"The Opposite Party owed a duty of care to the deceased"	EV-067	Duty (POINT 1)
WS p.8, para 17	"The patient was placed on guideline directed medical management for Acute Coronary Syndrome (ACS)"	EV-061	OP ne ACS maana (POINT 1)
WS p.2, PO-IV	"co-existing Coronary Artery Disease (CAD)"	EV-048	POINT 1, 7
WS p.4, para 3	"appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome"	EV-052	POINT 1
WS pp.5–6, para 8	"It is admitted that a blood cross match procedure was completed by the Blood Bank on 12.03.2026"; "the blood was never released or issued from the Blood Bank in the first instance"	EV-055	POINT 2
WS p.8, para 15	"The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion"	EV-060	POINT 2; W-24 ka jawab
WS p.6, para 10; p.9, 20-b	"prior to being shifted"; "The patient was shifted from CCU to the ward"	EV-056, EV-065	POINT 5
WS pp.6–7, para 11	"It is admitted that there was a temporary delay in the availability of a ward bed"; "Even on the trolley"	EV-057	POINT 5, 14
WS p.7, para 14	"the patient was brought to the CCU only once at around 9pm on 12.03.26"	EV-059	POINT 5, 6, 14
WS p.8, para 18	"the patient was received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered placing the time of death at approximately 12:51 AM"	EV-062	POINT 7, 14
WS p.5, para 7	"The Annexure C-9 referred to by the Complainants is in fact the vital chart of the Emergency Department of PGIMER"	EV-054	POINT 3 (C-9 ki pehchaan)
WS p.11, paras 22–23	"... the period of limitation are admitted"; "... regarding territorial jurisdiction are admitted"	EV-070	POINT 11
WS pp.10–11, 20-H	"are admitted as settled law governing compensation in medical negligence matters"	EV-068	POINT 9 (kanoon phir bhi verify)
WS p.11, para 21	"The sympathy and loss felt by the Complainants are acknowledged"	EV-069	Dhyan: liability ki admission nahi

OP ke records ki entries (admissions nahi, records): Ann-3 "CAD – USA (Wellen's pattern)", "anaemia", "correct anaemia" (EV-074); Ann-6 Hb 7.5 (EV-078); Ann-7 "kept ready for issue" (EV-079); Ann-5 night note (EV-077); Ann-10 advance Rs. 2,000 (EV-083); C-8 "Imp - Wellens Syndrome" (EV-037); C-10 "Trolly 66" (EV-041, EV-042); MCCD "CORONARY ARTERY DISEASE" (EV-044).

C10. Version A / Version B (Conflict) Register

X	Version A (source, place, tier)	Version B (source, place, tier)	Farq / maasoom aur pratikool vyakhya	Kya tay karega	Status
X-01	Memo of parties: C2 = Muskan (d/o), C3 = Husan Bano (wd/o) (EV-010; TIER-A); vakalatnama numbering same (EV-045)	Complaint paras 16, 18, 21 and affidavit paras 17, 19, 22: "mother ... (Complainant No.2)", "Complainant No.2 has lost her husband" (EV-018, EV-019, EV-022)	Numbering: drafting error in our pleading (innocent). Who stayed at night: unknown.	Memo settles numbering; Q-02 for the person present	RESOLVED (numbering) / NEEDS-USER (who stayed)

X	Version A (source, place, tier)	Version B (source, place, tier)	Farq / maasoom aur pratikool vyakhya	Kya tay karega	Status
X-02	Client: death "around 12:10–12:15 AM" (EV-019; CLIENT-REPORTED); complaint reads the certificate as "1:53 AM"	OP: "time of death as 12:51 AM" (WS p.8, para 18; EV-062); Ann-5 night note "At 12:51 ... declared dead" (EV-077); MCCD copy "1[?]:51", consistent with 12:51 (EV-044, EV-082) (TIER-A, OP)	The family's 12:10–12:15 matches the time OP's chart records P as "Received in CCU at 12:15" already unresponsive (EV-077), not the declared time. "1:53" was our misreading (W-23).	OP's own WS and chart (settled)	RESOLVED (death declared 12:51 AM; received unresponsive 12:15 AM)
X-03	Para 10: staff "informed the Complainant No.1 that no bed was available in the CCU" (EV-016)	Ground C: "the doctors never informed Complainant No.1 about the non-availability of beds or advised shifting the patient to another hospital" (EV-021)	May refer to different things (CCU bed vs ward bed/other hospital); reads as a contradiction	Client clarification (Q-11)	UNRESOLVED
X-04	Para 16: doctor said "no urgency for blood transfusion and that the same could be done later" (EV-018)	Ground E: "false assurance ... that no blood transfusion was required" (EV-021)	Paraphrase drift in our pleading. OP's WS para 15 matches para 16: the CCU duty doctor said "no immediate medical urgency for blood transfusion" (EV-060).	Use para 16 wording (OP's WS says the same)	RESOLVED (para 16 wording)
X-05	Memo: three married women C4–C6 as heirs (EV-010)	Para 21: "two daughters of the deceased are married, two children remain of marriageable age" (EV-022)	One of C4–C6 may not be a daughter, or para 21 miscounts	Q-05; legal heir certificate	NEEDS-USER
X-06	Para 4: "Later in the evening of 15.02.2026 ... re-admitted" (EV-012)	C-2: "Admission Date: 15/02/2026 05:06:10" (EV-026; TIER-A)	Pleading imprecision	C-2 (TIER-A) settles the recorded time	RESOLVED (record time 05:06)
X-07	Para 4: on NT-proBNP "appropriate treatment and medication were prescribed" (EV-012)	C-4 DS: no cardiac diagnosis or cardiac drug written (EV-030; TIER-A)	Treatment may have been elsewhere; or overstatement	Client (Q-08 next round); GMSH full record	UNRESOLVED
X-08	Para 4: "more than ten times the higher reference range" (EV-012)	C-3: 1117.48 vs ref 0–114.5 = 9.76× (EV-029; computed)	Rounding/overstatement	Arithmetic on TIER-A report	RESOLVED (9.76×)
X-09	Age 55 (C-1, C-2; EV-025, EV-026)	Age 60 (C-3, C-4, C-5, C-6, C-8, C-10, C-11; OP Ann-6, Ann-8, Ann-10; Ann-9 is the same document as C-11)	Registration entries differ	DOB/Aadhaar (Q-09)	UNRESOLVED
X-10	Treatment sheet "Date 10/3/26" (EV-039)	Registration "11-03-2026 02:04" (EV-036); columns "11/3"	Night-time slip (innocent) or form carried over	OP's original EMOPD chart (not annexed in D-08)	UNRESOLVED (minor)
X-11	Print stamp "3/12/2026, 12:33 PM" (EV-041)	"Crossmatch Date and Time 12/03/2026 12:55" (EV-041); OP letter "around 12 p.m." (EV-079; X-20)	PC clock vs server clock; print of a pending entry	Blood bank system log	UNRESOLVED (clock note)
X-12	Slip: "within six hours of issue ... return ... if not transfused" (EV-042)	Form note: "Transfuse within two hours of issue from the Blood Bank" (EV-041); complaint para 8 attributes "six hours" to the form (EV-015)	Two different instructions (time outside storage vs return); our attribution error	OP's transfusion SOP (not annexed; G-21)	UNRESOLVED
X-13	Complaint paras 10–18: P moved out of CCU, on stretcher on 3rd floor near staircase, died there (EV-016–EV-019; CLIENT-REPORTED)	C-11: "died in the Hospital in ward No. CCU"; "Address at the time of death: CCU PGIMER" (EV-044; TIER-A)	OP's own WS and chart reconcile the two: shifted from the CCU and kept on the trolley awaiting a ward bed (WS paras 10, 11, 20-b; EV-056, EV-057, EV-065), brought to the CCU once around 9 pm (para 14; EV-059), "Received in CCU at 12:15" unresponsive, pulse not palpable, CPR, declared dead at 12:51 (Ann-5, EV-077; para 18, EV-062). Collapse outside the CCU; death declared in the CCU.	OP's WS + Ann-5 (TIER-A); exact place outside the CCU: Q-03, Q-30	RESOLVED (reconciled) / exact place ALLEGED
X-14	Memo: one OP (EV-010); OP's WS title: one "Opposite Party" (EV-046)	Vakalatnama "PGIMER, CHANDIGARH & ANR" (EV-045); synopsis/paras 3, 5 "Opposite Party No.1"; D-05 "OP No.1" (EV-004)	Earlier draft with two OPs	Advocate	UNRESOLVED (cosmetic)
X-15	C-9 row-2 time "8:30 am" (complaint, EV-014)	Visual reading "5:30" possible (EV-040)	Handwriting. OP's own WS reads the entry as "8:30 AM on 11.03.2026" and identifies C-9 as its Emergency chart (WS p.5, para 7; EV-054).	OP's WS (TIER-A)	RESOLVED (8:30 AM)
X-16	Ground D / para 9: blood "made available by 12:55 PM" (EV-021, EV-015)	C-10: crossmatch at 12:55, no issue time (EV-041); para 15: blood bank closed when the complainant went to collect (EV-018)	OP's letter: PRBC "crossmatched and kept ready for issue around 12 p.m." (EV-079); register issue columns blank (EV-080); WS: "never released or issued" (EV-055). "Available" = crossmatched and kept ready, not issued.	OP's Ann-7 (settles the meaning); why not issued: X-21	RESOLVED (meaning) / reason → X-21
X-17	"NOOR AALM" (orders, e-Jagriti; EV-001–EV-006)	"Noor Aalam" (memo, EV-010; OP's WS title, EV-046); "NOOR ALAM" (vakalatnama, EV-045)	Spelling variants	Advocate (registered form)	UNRESOLVED (cosmetic)

X	Version A (source, place, tier)	Version B (source, place, tier)	Farq / maasoom aur pratikool vyakhya	Kya tay karega	Status
X-18	C-3: "N-Terminal Pro-BNP ... 1117.48 pg/mL", ref "0-114.5", collected 24/02/2026 (EV-029; TIER-A)	OP: "repeat BNP ... only 37.4 pg/mL, well within normal limits" (WS p.2, PO-III; pp.4–5, para 6; EV-047, EV-053); point-of-care panel "BNP 37.4" (Ann-1, Ann-3; EV-072, EV-074)	Two different tests (NT-proBNP and BNP) on two dates (24.02 and 11.03); the WS calls the second a "repeat". Innocent: the values may both be true for their dates. Adverse: unlike tests compared. Ann-1 print shows no name or date.	Expert (EQ-10); full lab print of Ann-1 with name and time	UNRESOLVED (expert)
X-19	CCU admission "11-Mar-2026 10:10:36" (Ann-10; EV-083); WS "10:10 AM" (PO-VI, 20-a; EV-050, EV-064) (TIER-A)	CCU vital chart first row "9:30 AM" (Ann-5; EV-076; first-row date LOW-CONFIDENCE)	40 min apart (code). Innocent: chart begun on arrival, admission entered later in the system, or different clocks.	CCU arrival / nursing admission note; system log	UNRESOLVED (minor; clock note)
X-20	Ann-7 letter: PRBC "crossmatched and kept ready for issue around 12 p.m." (EV-079)	C-10: "Crossmatch Date and Time 12/03/2026 12:55" (EV-041; TIER-A); print stamp "12:33 PM" (X-11)	"Around" is approximate; the letter was written on 01.06.2026, not from the system time.	Blood bank system log	UNRESOLVED (minor)
X-21	Ann-7 letter dated 01.06.2026 (written on Cardiology's letter of 29.05.2026): "no attendant came to collect the same for the patient" (EV-079; OP's statement, after the event)	Complaint paras 15–16 (CLIENT-REPORTED): ward doctor said transfusion "upon bringing the blood"; ACC Blood Bank "closed at approximately 9:00 PM"; Nehru Hospital blood bank asked for a fresh crossmatch (EV-018). OP's WS para 15: CCU duty doctor told the family there was "no immediate medical urgency for blood transfusion" (EV-060)	Who was to collect, when, and whether the family was told the unit was ready. The slip says "Kindly bring Blood Transport Box from the ward at the time of issue of blood" (EV-042). Innocent readings exist on both sides.	Q-26, Q-27; ward nursing notes (G-18); transfusion protocol (G-21); issue register (blank, EV-080)	UNRESOLVED (MAJOR) — DISPUTED
X-22	WS para 17: "He was prescribed dual antiplatelets, statins, nitrates, and anticoagulants" (EV-061)	Ann-8 drug chart: Brilinta, Ecosprin, Monit GTN, Clexane, Lasix, IVF; no statin as read (EV-081; TIER-A). C-8 sheet: "Ecosprin Gold (75/75/20) OD" (EV-039)	A statin may be in a combination tablet (composition NOT VERIFIED) or on a page not annexed; or the WS overstates.	Full drug chart; pharmacist or expert on the combination	UNRESOLVED (minor)
X-23	WS para 12: at the ~9 pm encounter "pulse and oxygen saturation ... within normal limits, with findings similar to those at initial admission to PGIMER, no tachycardia or tachypnoea" (EV-058)	Initial admission: Ann-2 "Rate: 123 bpm", echo "Tachycardia" (EV-073); C-8 "PR - 116/min" (EV-037); Ann-5 pulse 125–128 on 11/3; pulse 82 at 9 pm on 12/3 (EV-076)	At 9 pm the recorded pulse (82) was not fast; but "similar to ... initial admission" does not match the admission record, which shows a fast rate. Innocent: loose wording.	Note of the 9 pm encounter (G-18); dated ECG (G-24)	UNRESOLVED (Possible Inconsistency)
X-24	WS para 11: "Even on the trolley" monitoring was done, "as conclusively evidenced by the vital chart" (EV-057)	Ann-5, the only chart annexed: last entry 12/3 9 pm; next writing "Received in CCU at 12:15" (EV-076, EV-077); no trolley or ward chart annexed	3 h 15 min with no entry in the pages read (code). Innocent: a ward or trolley chart exists but was not annexed (WS para 7 says the ward chart was "duly maintained").	Ward/trolley chart (G-18); EQ-13	UNRESOLVED (Possible Inconsistency; RECORD GAP)
X-25	WS: "hemodynamically stable" throughout (paras 7, 15, 20-E; EV-054, EV-060, EV-066)	Ann-5 values: pulse 125–128 (11/3); BP 90/49 (6 pm 11/3); 87/46 with pulse "45" [LOW] (9 AM 12/3); 90/60 (9 pm 12/3) (EV-076)	Whether these values show instability is a clinical question; "stable" may be the clinicians' view of the whole picture.	Expert (EQ-8); original Ann-5	UNRESOLVED (expert)
X-26	WS para 19: "'Type I Respiratory Failure, secondary to Aspiration'" (EV-063)	MCCD: "(a) Type I respiratory failure (b) aspiration pneumonia (c) coronary artery disease" (EV-044, EV-082; TIER-A)	"Aspiration" (an event) and "aspiration pneumonia" (an infection that takes time to develop) are different things; the WS bases its account on findings at intubation during CPR.	Resuscitation/intubation record (G-20); expert (EQ-5)	UNRESOLVED (Possible Inconsistency)
X-27	WS para 3: "only '?Wellens Syndrome' ... merely suspected and not confirmed" (EV-052); PO-III "grossly exaggerates the severity" (EV-047)	OP's own Ann-3 diagnosis "CAD – USA" "(Wellen's pattern)", no "?" (EV-074; TIER-A); C-8 "Imp - Wellens Syndrome" (EV-037); WS para 17: ACS management given (EV-061)	Innocent reading: "pattern" describes the ECG, not a final diagnosis. Record check: Contradiction (OP's WS against OP's own annexure).	OP's Ann-3 (TIER-A); expert (EQ-1, EQ-7)	UNRESOLVED — label: Contradiction
X-28	WS: blood "solely as a precautionary standby measure", for "emergency bleeding such as haemoptysis"; no indication for transfusion (PO-V, paras 3, 8, 15, 20-D; EV-049, EV-052, EV-055, EV-060, EV-066)	Ann-3 plan "correct anaemia" [MODERATE] (EV-074); Ann-7 "kept ready for issue ... for the patient" (EV-079); Hb 7.5 g/dL (Ann-6; EV-078)	"No indication" against a written plan to correct anaemia: label Contradiction. "Standby" against "kept ready for issue": Possible Inconsistency (standby units are also kept ready).	Transfusion order / indication note; expert (EQ-3)	UNRESOLVED (expert)
X-29	WS PO-III and para 3: NT-proBNP "recorded at GMSH-16" on 24.02.2026 (EV-047, EV-052)	C-3: WeCare Lab (private), "Sample Collected At: HR-11" (EV-029; TIER-A)	A private laboratory's report; the sample may have been sent from GMSH-16 or given elsewhere. Little effect on the merits.	Client (where the sample was given); GMSH-16 record	UNRESOLVED (minor)

X	Version A (source, place, tier)	Version B (source, place, tier)	Farq / maasoom aur pratikool vyakhya	Kya tay karega	Status
X-30	Complaint paras 12–14 (CLIENT-REPORTED): chest pain on 12.03; refusal to examine or re-admit; check "solely based on pulse oximeter readings"; no BP/ECG (EV-017)	WS: "after initiation of cardiac management, the patient did not complain of chest pain" (para 17; EV-061); "denied chest pain on multiple occasions" (20-a; EV-064); the 9 pm complaint was "mild breathlessness" (para 14; EV-059); Ann-5 BP 90/60 at 9 pm (EV-076)	Family's account against OP's; neither side has produced a dated note of the 9 pm encounter. Our "no BP" is weakened by the 9 pm BP entry (W-28).	Note of the 9 pm encounter (G-18); dated ECG (G-24); affidavit of the person present (Q-02, Q-21)	DISPUTED (UNRESOLVED)

C11. Citation Register (every entry NOT VERIFIED — E-01)

C	Authority	Limited proposition relied on (to be checked)	Source type	What to check, where	Status
CIT-01	Consumer Protection Act, 2019 (Act 35 of 2019), s.2(7)	Who is a "consumer" (incl. beneficiary of a service)	none opened	Exact text as in force on 07.04.2026; India Code	NOT VERIFIED
CIT-02	CPA 2019, s.2(11), s.2(42)	"Deficiency"; "service" and its free-of-charge exclusion	none opened	Same; Mediation Act 2023 amendments to s.2	NOT VERIFIED
CIT-03	CPA 2019, s.34	District Commission pecuniary measure (value of consideration paid) and territorial jurisdiction	none opened	s.34(1), (2) and proviso	NOT VERIFIED
CIT-04	Consumer Protection (Jurisdiction of the District Commission, the State Commission and the National Commission) Rules, 2021	District Commission up to Rs. 50 lakh (reported); measure = consideration	skill anchor (secondary)	Gazette notification; later amendment	NOT VERIFIED
CIT-05	CPA 2019, s.35	Who may file; legal heirs (proforma cites s.35(1)(a))	none opened	s.35(1)	NOT VERIFIED
CIT-06	CPA 2019, s.38 (sub-section for services — to be identified)	Written version within 30 days, extendable by not more than 15 days	none opened	s.38(2)(a)/(3)(a): which applies to services	NOT VERIFIED
CIT-07	Supreme Court on the 45-day outer limit (memory lead: *New India Assurance Co. Ltd. v. Hilli Multipurpose Cold Storage Pvt. Ltd.*, Constitution Bench, 2020)	No extension beyond 30 + 15 days	memory (lead only)	Name, citation, paragraph, applicability to the 2019 Act	NOT VERIFIED
CIT-08	CPA 2019, s.69	Limitation: 2 years from cause of action; condonation	skill anchor	s.69(1), (2)	NOT VERIFIED
CIT-09	*Indian Medical Association v. V.P. Shantha*, (1995) 6 SCC 651	Paid medical services are "service"; free-for-all excluded	skill anchor (reports of later doubt: 2024 references)	Judgment text; later treatment	NOT VERIFIED
CIT-10	*Jacob Mathew v. State of Punjab* (2005)	Standard of care (Bolam as applied in India)	skill anchor	Citation, paragraph	NOT VERIFIED
CIT-11	*Kusum Sharma v. Batra Hospital* (2010)	Principles of medical negligence	skill anchor	Citation, paragraph	NOT VERIFIED
CIT-12	*Maharaja Agrasen Hospital v. Master Rishabh Sharma*, C.A. 6619 of 2016, 16.12.2019	Compensation on restitutio in integrum (para "11.5.4" as quoted in our complaint)	our own pleading's quotation; OP: "admitted as settled law" (EV-068)	Judgment text and para 11.5.4	NOT VERIFIED
CIT-13	*Sarla Verma v. Delhi Transport Corporation* (2009) (cited in complaint as "III (2009) ACC 708 (SC)")	"Just compensation" (para 16 as quoted)	our own pleading; OP: "admitted as settled law" (EV-068)	Correct citation, para, applicability to medical cases	NOT VERIFIED
CIT-14	*Charan Singh v. Healing Touch Hospital*, (2000) 7 SCC 68	Quantifying damages in consumer cases (as quoted)	our own pleading; OP: "admitted as settled law" (EV-068)	Text, paragraph	NOT VERIFIED
CIT-15	Res ipsa loquitur (complaint ground G); *Harish Kumar Khurana v. Joginder Singh* (2021, reported non-reportable)	Applies only where negligence is glaring (reported)	skill anchor	Text; reportability	NOT VERIFIED
CIT-16	*Balram Prasad v. Kunal Saha* (2013)	Quantum, multiplier doubts, vicarious liability, interest from complaint date (headnote topics)	skill anchor	Text, paragraphs	NOT VERIFIED
CIT-17	Bharatiya Sakshya Adhiniyam, 2023, s.63	Certificate for electronic records (blood bank system printouts, C-10)	skill anchor	Applicability to this forum and proceeding	NOT VERIFIED

C	Authority	Limited proposition relied on (to be checked)	Source type	What to check, where	Status
CIT-18	Procedure on closing a party's evidence / not filing a rejoinder (CPA 2019 and applicable Commission regulations)	Effect of D-05 and D-06 orders	none	[provision: advocate to confirm]	NOT VERIFIED
CIT-19	General Clauses Act, 1897, ss.9, 10	Computing "within 3 days"; last day a holiday	none opened	Applicability to Commission orders	NOT VERIFIED
CIT-20	Effect of no specific denial in a written version (principle of non-traverse; Code of Civil Procedure, 1908, Order VIII Rule 5 as a guide only — its application before a Consumer Commission to be confirmed)	A fact not specifically denied may be taken as admitted (our "No Reply" items, e.g. consumer status)	none opened (general principle, memory)	Whether and how the Commission applies it; CPA 2019 s.38 and the applicable regulations	NOT VERIFIED
CIT-21	Red-cell transfusion guideline in force in March 2026 for a patient with acute coronary syndrome — no specific guideline identified	When PRBC transfusion is indicated, and how urgently, at Hb 7.5 g/dL in ACS (OP relies on Hb "above 7 g/dL" as the threshold "per global guidelines", WS paras 3, 8, 15, 20-D)	none (no Benchmark Record made)	Expert to name the guideline and its version for the date of care; OP's own transfusion SOP (G-21)	NOT VERIFIED — no verified benchmark for this date
CIT-22	Authority on resource constraints (bed shortage, patient load) as a defence to deficiency in service — no authority identified	Whether a "systemic constraint" (WS para 10) excuses the care given while the patient waited for a bed	none	Advocate to research (Supreme Court / NCDRC); no authority is named here	NOT VERIFIED
CIT-23	Weight of a written version and its annexures when the OP's evidence is closed (D-05) and no evidence affidavit is on record	A written version is a pleading, not evidence; facts in it and its annexures are not proved by affidavit	none opened	[provision: advocate to confirm]; CPA 2019 and the applicable regulations; overlaps CIT-18	NOT VERIFIED

C12. Damages Calculation Ledger

RH	Head	Legal basis	Evidence	Claimed	Supported	Calculation / assumptions	Double count	Proof	Missing / risk
RH-01	Loss of life, mental agony, financial loss	CIT-12, CIT-13, CIT-14 (NOT VERIFIED; OP "admitted as settled law")	EV-022, EV-023, EV-044	Rs. 60,00,000	SUPPORT NAHI	No formula pleaded; no income figure; scenarios not possible (inputs missing)	Overlaps "financial loss" with any income head	CLIENT-REPORTED	Income documents, dependants, age (G-09, G-11, G-12); OP: "unsupported by any documentary evidence of income" (EV-069); over-claim risk HIGH
RH-02	Interest @12% p.a. pendente lite	CIT-16 (lead; NOT VERIFIED)	EV-023	12% p.a. on RH-01	SUPPORT NAHI	12% of 60,00,000 = 7,20,000 per year (arithmetic only); start date not pleaded	—	—	Start date; rate is discretionary
RH-03	Medical expenses	CIT-02 (NOT VERIFIED)	EV-023, EV-043, EV-083	Rs. 2,50,000	Rs. 2,150	Rs. 2,000 advance (11.03.2026, EV-083) + Rs. 150 POS (12.03.2026, EV-043) = Rs. 2,150 (code)	GMSH-16 care billed to Ayushman Bharat (EV-025, EV-026, EV-030); pre-existing illness costs	PART-PROVED	Other bills and receipts (G-10); OP gives no specific reply (No Reply)
RH-04	Litigation expenses	Discretion	EV-023	Rs. 1,00,000	SUPPORT NAHI	—	—	NO PROOF	Fee receipts
RH-05	Non-monetary: doctors' details; preservation and sealed deposit of records and CCTV	[provision: advocate to confirm]	EV-023	—	—	—	—	—	No order in D-02 to D-06; WS silent
Totals (computed from rows)				**Rs. 63,50,000 + interest**	**Rs. 2,150**	Claimed and supported kept apart; no figure is the sum the Commission will award. OP seeks exemplary costs against the complainants (EV-071).			

C13. NOT CONFIRMED LEADS (TIER-C; record se milana baaki)

#	Lead	Source	Status after Round 2
1	OP's WS gives the claim as Rs. 60,00,000	User's prompt	CONFIRMED by WS (PO-VII, para 21; EV-051, EV-069); WS does not address prayers (ii)–(v)
2	An MS office / Director letter of 15.03.2026 and a CCTV preservation request exist	User's round plan (ZIP_3)	Round 3
3	A High Court writ draft (March 2026) exists	User's round plan (ZIP_3)	Round 3; PR-02
4	Colour photographs of receipts and notes (Evidence_Sanlagnak_Photos) exist	User's prompt	Not in any zip (G-08)
5	OP's WS annexures include vital chart, drug chart, CCU sheet, lab slip, death/admission record	User's prompt	READ (Ann-5, Ann-8, Ann-2/3, Ann-1/6, Ann-9/10)
6	A "Medical Grievances Committee" received the Department of Cardiology's para-wise comments	WS PO-II (EV-046)	Documents not annexed (G-17); Q-29

C14. Correction Register

KOI NAHI — Round 2 mein apne pichhle (VR-01) kisi nishkarsh mein galti nahi mili. VR-01 ki jo baatein ab naye record se STALE hain (galti nahi, kyunki woh "padhe gaye pages" tak simit thi), unki list Change Register (CH-09, CH-11, CH-13) mein hai.

C15. Change Register (round-wise)

CH	Round	Change and reason	Type
CH-01	1	File created (VR-01): cover, "Filing se pehle", Part A material, Part B POINTs 1–13, Part C registers.	Structure
CH-02	2	Cover: Round 2 of 6, VR-03; OP's prayer added; payments to OP Rs. 2,150; reading status D-08. Reason: D-08 read.	Fact / Structure
CH-03	2	New CASE IMPACT page (UPDATE rule).	Structure
CH-04	2	"Filing se pehle": items on 45 days (WS date 26.06.2026), rejoinder, evidence closed, time and place of death (now settled by OP's records), "no attendant came", heart-failure wording, OP's unannexed records. Reason: D-08.	Fact
CH-05	2	Part A rewritten: synopsis, WS date in procedural history, facts F-024–F-040, issue sufficiency, OP's words with WS location and our answers, new A6a (preliminary objections), checklist, OP annexures in documents list.	Fact / Legal / Structure
CH-06	2	Banner 13 → 14 POINTs; winning-points table re-ranked (POINT 5 rank 5 → 3; new POINT 14 rank 4; POINT 3 rank 3 → 5; POINTs 10, 11 up after admissions).	Structure
CH-07	2	POINT 1: OP Ann-2/Ann-3 diagnosis, duty and ACS treatment admitted, X-27, honest counterweight (BNP 37.4, normal LVEF).	Fact
CH-08	2	POINT 2: PGIMER Hb 7.5, plan "correct anaemia", Ann-7 letter and register, non-issue admitted, X-21, X-28; W-14 resolved.	Fact
CH-09	2	POINT 3 reframed. STALE from VR-01: "no BP entry after 8:30 AM on 11.03 until death" and the RECORD GAP of 39–44 h (computed from C-9 to the death) — the OP's CCU chart (Ann-5) has entries from 9:30 AM 11/3 to 9 pm 12/3. New framing: 4–8 h gaps and the recorded values (W-27).	Fact
CH-10	2	POINT 4: WS's unannexed medical claims listed.	Legal
CH-11	2	POINT 5: shifting and trolley now Express Admission. STALE from VR-01: X-13 as a MAJOR contradiction and W-05 (SERIOUS) — reconciled by WS paras 10–18 and Ann-5.	Fact
CH-12	2	POINT 6: WS paras 12, 14, 17; 9 pm BP exists (W-28); chest pain DISPUTED (X-30).	Fact
CH-13	2	POINT 7: time of death settled at 12:51 AM. STALE from VR-01: the candidate readings (12:51 / 1:51 / 1:53) and the intervals computed to 00:10/01:51/01:53; complaint's "1:53 AM" is our misreading (W-23).	Fact
CH-14	2	POINT 8: causation chain rebuilt from OP's records.	Fact
CH-15	2	POINT 9: WS para 21 and the exemplary-costs prayer.	Fact
CH-16	2	POINT 10: advance Rs. 2,000 (Ann-10); no objection in the WS.	Fact / Legal
CH-17	2	POINT 11: limitation and territorial jurisdiction expressly admitted.	Fact
CH-18	2	POINT 12: list of records the WS relies on but does not annex.	Fact
CH-19	2	POINT 13: WS date 26.06.2026 and both calculations.	Fact
CH-20	2	POINT 14 added (no entry 9 pm → 12:15 AM; received unresponsive).	Fact / Structure
CH-21	2	Sections 5–14: W-05, W-13, W-14 resolved; W-16 largely cured; W-23–W-28 added; section 6 AUDIT-OPP filled; gap graph rebuilt; EQ-8–EQ-13; records list; RAKAM supported Rs. 2,150; hearing notes.	Fact / Structure
CH-22	2	Part C: D-08 in Reading Log and Intake; PR-04; DL-01; timeline rows; EV-046–EV-084; Critical Value Ledger; X-18–X-30 (X-02, X-13, X-15 resolved); CIT-20–CIT-23; G-17–G-24; Q-26–Q-33.	Structure

C16. Open Gaps

G	Gap	Affects
G-01	Proof of filing of complainants' Written Arguments after 14.10.2026	DL-05; everything
G-02	OP's Written Arguments (filed by 14.10.2026)	Part A answers
G-03	Notice service report (date of receipt)	POINT 13; DL-01
G-04	OP's Written Statement with Annexures 1–12 — CLOSED in Round 2 (D-08 read, 25/25 pages)	—
G-05	OP's full record: CCU admission/transfer notes, nursing notes, vital flow sheets, monitor/ECG prints, blood bank issue/return and transfusion records, death summary, MLC and bed registers (D-08 gives selected pages only; specific items at G-17–G-24)	POINT 2, 3, 5, 6, 7, 8, 14

G	Gap	Affects
G-06	Any CCTV preservation request or order (12–13.03.2026)	POINT 5, 12; DL-08
G-07	Expert medical opinion	POINT 2, 3, 7, 8
G-08	Evidence photographs (colour)	POINT 2, 3
G-09	Income proof of the clothing shop; dependants	POINT 9; RH-01
G-10	Bills/receipts for Rs. 2,50,000	POINT 9, 10; RH-03
G-11	Legal heir certificate; relationship of C4–C6	POINT 9; standing
G-12	Proof of P's age/date of birth	X-09; RH-01
G-13	Court page 59; Labogram pages "2 of 5" and "4 of 5"	D-01 completeness
G-14	Clear copy/original of C-7 ECG without the black box (C-9 row-2 time and C-11 time of death now settled by OP's WS: X-15, X-02)	W-19
G-15	Official legal texts (blocked by network policy)	All CIT-ids
G-16	Whether applications on prayers (iv) and (v) were filed or decided	POINT 12
G-17	Medical Grievances Committee: the Department of Cardiology's "Para-wise Comments" and any committee report (WS PO-II; not annexed)	POINT 4, 12; PR-04
G-18	Ward/trolley vital chart and nursing notes from the evening of 12.03.2026 to 13.03.2026 00:51 (WS para 7 says the ward chart was "duly maintained")	POINT 3, 5, 14; X-21, X-23, X-24, X-30
G-19	CCU-to-ward shifting/transfer note and bed allotment register (WS paras 9–10)	POINT 5
G-20	Resuscitation and intubation record of 13.03.2026 (WS para 19)	POINT 7, 14; X-26
G-21	PGIMER protocols the WS relies on: CCU admission, CCU-to-ward shifting, transfusion (WS paras 6, 10, 12)	POINT 2, 4, 5; X-12, X-21
G-22	Counselling and consent notes: prognosis, deferral of coronary angiography (WS PO-IV, 20-a)	POINT 12; Q-28
G-23	Formal echo report and chest X-ray report (Ann-2 and Ann-4 give only a proforma entry and an image)	POINT 1
G-24	All ECGs of 11–13.03.2026 with machine time and patient name (Ann-11, Ann-12 carry none legible)	POINT 6; X-23, X-30

C17. Sawal register (Advocate / Client ke sawal)

Q	Kisse	Sawal	Target	Status
Q-01	Advocate + Client	14.10.2026 ke baad hamare Written Arguments registry mein kis tarikh ko file hue? (diary no./copy)	DL-05, G-01	OPEN (asked)
Q-02	Client	12/13.03.2026 ki raat P ke saath kaun tha — C3 (maa) ya C2 (Muskan)? (Kyun: complaint "Complainant No.2" kehti hai, memo mein C2 beti hai.)	X-01, W-06	OPEN (asked)
Q-03	Client	Jab P ki maut hui, stretcher theek kahan tha — CCU ke andar, CCU ke corridor mein, ya 3rd floor par staircase ke paas? (Round 2: OP ke apne record se P CCU mein "unresponsive" laaye gaye aur maut wahan ghoshit hui — X-13. Sateek jagah ke liye Q-30 bhi dekhein.)	X-13, Q-30	OPEN (asked)
Q-04	Client	C-11 par "NOT WILLING FOR AUTOPSY" kisne likha, kya C1 ne wahan sign kiya, aur kya post-mortem ke baare mein samjhaya gaya tha?	W-04	OPEN (asked)
Q-05	Client	C4 Gulbahar, C5 Soni aur C6 Jasmin ka P se kya rishta hai, aur kya koi aur legal heir hai? (legal heir certificate hai?)	X-05, G-11	OPEN (asked)
Q-06	Client	PGIMER ko Rs. 150 ke alawa aur kya bhugtan kiya (registration, dawaiyan, CCU, tests) — receipts hain? Rs. 2,50,000 ka hisaab kin bills se hai?	POINT 10, RH-03	OPEN (asked)
Q-07	Client	C-9 B.P. chart aur C-7 ka ECG (jis par kaala dabba hai) aapko kahan se aur kab mile? Kya kisi ne us par kuch chhupaya?	W-13, W-19	OPEN (asked)
Q-08	Client	Dukaan ki aay ke koi kagaz (registration, GST, bank statement, ITR)? Feb 2026 mein dil ka ilaaj kahin aur hua?	G-09, X-07	NEXT ROUND
Q-09	Client	P ki janm-tithi / umar ka saboot (Aadhaar): 55 ya 60?	X-09	NEXT ROUND
Q-10	Client	15.02.2026 ke GMSH referral ke baad kya PGIMER Pulmonology gaye? Koi record?	F-001b	NEXT ROUND
Q-11	Client	12.03.2026 ko "CCU mein bed nahi" kis samay aur kisne (role) bataya? Ward bed ya doosre hospital ke baare mein kuch kaha?	X-03	NEXT ROUND
Q-12	Client	Kya aapne kabhi blood bag haath mein liya ya blood transport box diya gaya? Blood bank kis samay pahunche? (Round 2: Q-26, Q-27 dekhein.)	X-16, X-21	NEXT ROUND
Q-13	Client	Raat ko phone kis samay aaya ki saans ruk gayi (call log)? (Round 2: maut ka samay OP ke record se 12:51 AM tay; yeh sawal ab kam zaroori.)	X-02	LOW PRIORITY
Q-14	Client	PGIMER (MS/Director) ko koi likhit shikayat ya CCTV preservation request kab bheji?	G-06	NEXT ROUND
Q-15	Client	Para 21 ke "two children of marriageable age" kaun hain?	X-05	NEXT ROUND
Q-16	Client	PR-02 (High Court writ) aur PR-03 (police complaint) sach mein file hue? Kab, kahan?	PR-02, PR-03	NEXT ROUND
Q-17	Advocate	OP ka evidence band hone ka kanooni asar (CIT-18).	POINT 4	OPEN
Q-18	Advocate	Hamara rejoinder band hone ka asar; Written Arguments mein WS ka para-wise jawab kitna diya ja sakta hai.	W-01	OPEN
Q-19	Advocate	Prayer (iv)/(v) par koi application di ya order hua?	G-16	OPEN
Q-20	Advocate	Service report se notice ki receipt date; 45-din ganana (DL-01). WS par tarikh 26.06.2026; filing 01.07.2026 ke order mein darj.	POINT 13	OPEN
Q-21	Advocate	Raat maujood vyakti ka affidavit is stage par liya ja sakta hai?	W-06	OPEN
Q-22	Advocate	Registered spelling of C1 ka naam (X-17).	X-17	OPEN
Q-23	Advocate	Filed set mein court page 59 kya hai?	G-13	OPEN
Q-24	Advocate	Saari CIT-01–CIT-23 entries official text se verify.	G-15	OPEN

Q	Kisse	Sawal	Target	Status
Q-25	Advocate	Expert opinion is stage par record mein kaise laayein (agar laayein).	W-03	OPEN
Q-26	Client	12.03.2026 ko kya kisi ne (role) parivar ko bataya ki blood bank mein blood "taiyar" hai aur use lene jaana hai? Kab? Koi parchi ya blood transport box diya gaya? (Kyun: OP ka letter kehta hai "no attendant came to collect the same", Ann-7.)	X-21, W-24	OPEN (new)
Q-27	Client	Parivar ACC Blood Bank kis-kis samay gaya — kya raat ~9 pm se pehle bhi gaya? Wahan kisne (role) kya kaha? Nehru Hospital blood bank kis samay gaye?	X-21	OPEN (new)
Q-28	Client	Kya doctors ne parivar ko angiography na karne (ya baad mein karne) ke baare mein, aur "very poor prognosis" ke baare mein bataya? Kab, kisne (role)? Kisi kagaz par sign karwaya?	G-22, EQ-11, POINT 12	OPEN (new)
Q-29	Client + Advocate	Kya parivar ne PGIMER ki "Medical Grievances Committee" ko shikayat di thi, ya committee ne parivar ko bulaya? Koi notice, sunwai ya report mili? (WS PO-II mein iska hawala hai.)	PR-04, G-17	OPEN (new)
Q-30	Client	12.03 ki shaam CCU se nikalne ke baad trolley theek kahan rakhi gayi (floor, ward, corridor)? Raat ~9 pm CCU le jaane ke baad wapas kab bheje gaye? P ko unresponsive sabse pehle kisne, kahan aur kis samay dekha, aur CCU tak kaun le gaya?	X-13, X-24, POINT 14	OPEN (new)
Q-31	Advocate	Kya is stage par Commission se G-17–G-24 records (jinka WS mein hawala hai par annex nahi) mangwane ki application di jaaye?	POINT 12; G-17–G-24	OPEN (new)
Q-32	Advocate	OP ke "no attendant came to collect" (Ann-7, letter dated 01.06.2026, Cardiology ke 29.05.2026 ke letter par bana) ka jawab Written Arguments mein kaise rakhein?	X-21, W-24	OPEN (new)
Q-33	Advocate	OP ke "exemplary costs" wale prayer ka jawab kaise dein?	W-26	OPEN (new)

C18. Traceability Map (POINT → evidence)

POINT	Element	EV-ids (all READ)	Statement status
POINT 1	Duty	EV-032, EV-036, EV-037, EV-038, EV-039, EV-047, EV-048, EV-052, EV-053, EV-061, EV-067, EV-072, EV-073, EV-074, EV-081, EV-083	DOCUMENTED + ADMITTED
POINT 2	Breach	EV-018, EV-033, EV-037, EV-041, EV-042, EV-043, EV-049, EV-055, EV-060, EV-066, EV-074, EV-078, EV-079, EV-080, EV-081	DOCUMENTED + ADMITTED; DISPUTED reason (X-21)
POINT 3	Breach	EV-039, EV-040, EV-050, EV-054, EV-066, EV-076, EV-081	AS RECORDED; RECORD GAP
POINT 4	Procedural	EV-002, EV-003, EV-004, EV-005, EV-046, EV-052, EV-053, EV-056, EV-071	DOCUMENTED
POINT 5	Breach	EV-016, EV-017, EV-041, EV-042, EV-050, EV-056, EV-057, EV-058, EV-059, EV-065, EV-074, EV-076, EV-077, EV-083	ADMITTED (shifting, trolley); ALLEGED (exact place)
POINT 6	Breach	EV-017, EV-035, EV-058, EV-059, EV-061, EV-064, EV-073, EV-076, EV-084	DISPUTED (chest pain); NOT RECORDED (dated ECG)
POINT 7	Causation	EV-019, EV-020, EV-021, EV-032, EV-044, EV-062, EV-063, EV-066, EV-074, EV-077, EV-082	DOCUMENTED; UNSUPPORTED (intubation finding)
POINT 8	Causation	EV-022, EV-025–EV-034, EV-037, EV-048, EV-055, EV-057, EV-062, EV-072, EV-074, EV-076–EV-079	UNKNOWN (expert)
POINT 9	Damage	EV-010, EV-022, EV-023, EV-043, EV-044, EV-045, EV-068, EV-069, EV-071, EV-082, EV-083	DOCUMENTED (harm); ALLEGED (quantum)
POINT 10	Maintainability	EV-008, EV-025, EV-026, EV-030, EV-036, EV-043, EV-046, EV-051, EV-083	DOCUMENTED; No Reply
POINT 11	Maintainability	EV-001, EV-006, EV-007, EV-022, EV-070	ADMITTED
POINT 12	Procedural	EV-001–EV-005, EV-023, EV-042, EV-046, EV-048, EV-054, EV-056, EV-064, EV-083	DOCUMENTED; No Reply
POINT 13	Procedural	EV-001, EV-002, EV-003, EV-071	DOCUMENTED; CONDITIONAL
POINT 14	Breach, Causation	EV-019, EV-056, EV-057, EV-059, EV-062, EV-065, EV-076, EV-077	DOCUMENTED + ADMITTED; RECORD GAP

C19. Error Log

E	What was attempted	Failure type	Impact and downgrade
E-01	Open official legal texts (India Code, NCDRC, Consumer Affairs, e-Gazette, e-Jagriti, Supreme Court, DoCA) by direct download and by web fetch (two alternatives tried, then stopped)	NOT AVAILABLE (network policy: proxy 403 / host not resolved)	Every law and citation NOT VERIFIED (CIT-01–CIT-23); conditional wording used; all on the advocate's checklist. Fix: allow these domains in the environment's network settings. (Not retried in Round 2: nothing changed.)
E-02	Run the skill's own scripts (deadline_calc, interval_calc, ledger_total, ref_file_lint)	NOT AVAILABLE (blocked by the session's safety check for code from an external repository)	Dates, intervals and totals computed with separate Python date arithmetic and shown; structure checked by a separate lint (Compact Integrity Result).

E	What was attempted	Failure type	Impact and downgrade
E-03	OCR of the scanned bundles (D-01, D-08)	NOT AVAILABLE (no OCR engine installed)	Visual reading only; no second method for handwriting; low-confidence values marked (Critical Value Ledger).

C20. Version Registry

VR	Item	Version	Date	Supersedes	What changed
VR-01	This file (Written Arguments material + Vakil File)	Round 1	29.10.2026	—	First version (CH-01). Backup: _kaam/purane_round/round1/
VR-02	D-01 to D-07 (ZIP_1) received	as received	29.10.2026	—	Old names mapped per READ_FIRST; "document (5)" (duplicate) not received
VR-03	This file	Round 2	29–30.10.2026	VR-01 (SUPERSEDED, kept in backup)	CH-02 to CH-22
VR-04	D-08 (ZIP_2: reply.pdf) received	as received	29.10.2026	—	OP's WS + Annexures 1–12

C21. STATE SNAPSHOT

```
STATE SNAPSHOT (Round 2 of 6, VR-03, 30.10.2026)
Forum: PR-01 DCDRC Chandigarh, CC DC/44/CC/423/2026 - ESTABLISHED; PR-02, PR-03 UNKNOWN; PR-04 (PGIMER committee) UNKNOWN
Read: D-01 (62/62), D-02..D-06 (7/7), D-07 (1/1), D-08 (25/25); ZIP_3..ZIP_6 NOT READ
Last IDs: D-08 | EV-084 | F-040 | X-30 | CIT-23 | RH-05 | DL-09 | G-24 | Q-33 | W-28 | EQ-13 | E-03 | CH-22 | CR- none | VR-04 | PR-04
POINTs: 14 (POINT 1..14), all PROVISIONAL UNTIL PART 4; OP reply now filled from the original WS
Open questions: Q-01..Q-07 and Q-26..Q-30 (family); Q-08..Q-16 next round; Q-17..Q-25, Q-31..Q-33 (advocate)
Open gaps: G-01..G-03, G-05..G-24 (G-04 closed)
Nearest deadlines: DL-05 our WA 16/17.10.2026 POSSIBLY EXPIRED; DL-08 preservation DANGER; DL-06 22.12.2026
Pending: Round 3 (client's early papers: facts vs record, Version A/B); law verification (E-01); expert (G-07)
```

C22. Compact Integrity Result (structure and traceability checks only)

Run by a separate read-only lint (the skill's own scripts were blocked, E-02). Checks: 23; PASS 23; FLAG 0. Consistency only — it does not prove that the facts or the law are true.

Check	Result	Detail
Banner count = POINT headings	PASS	banner 14, headings 14
POINT numbers run 1..N without gap/repeat	PASS	[1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14]
Every POINT has the five parts in order	PASS	boxes 14; bad []
Every POINT has an ELEMENT TAG line	PASS	—
Every reply has exactly one source tag + WS Location	PASS	—
Winning-points table lists every POINT once	PASS	[1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14]
Every EV-id cited exists in the Evidence Register	PASS	defined 84; missing []
Every X-id cited exists in its register	PASS	defined 30; missing []
Every CIT-id cited exists in its register	PASS	defined 23; missing []
Every DL-id cited exists in its register	PASS	defined 9; missing []
Every G-id cited exists in its register	PASS	defined 24; missing []
Every Q-id cited exists in its register	PASS	defined 33; missing []
Every W-id cited exists in its register	PASS	defined 28; missing []
Every RH-id cited exists in its register	PASS	defined 5; missing []
Every E-id cited exists in its register	PASS	defined 3; missing []
Every EQ-id cited exists in its register	PASS	defined 13; missing []
Every 'POINT N dekho' resolves	PASS	—
No filing text (cause title / prayer / verification) drafted outside quotations	PASS	—
Status line from the vakil_file_status ladder	PASS	—
Every Citation Register entry shows NOT VERIFIED	PASS	rows 23
Audit Trail Table has six categories	PASS	—
Language guard: no emotive/unsupported words outside quotations	PASS	—
Privacy: no 12-digit IDs, mobile numbers or full CR No.	PASS	—